Small dental practices miss calls, rework claims and fear clawback

Problems, workflows, numbers, vocabulary and service gaps in small UK and US dental practices: independent practices of one to five dentists, NHS, private and mixed in the UK, fee-for-service, PPO and Medicaid in the US, and the agencies, vendors and freelancers that sell to them

Research report, 16 October 2026

Problems and gaps only. This document adds no solutions and no recommendations.

Scope	UK (England mainly, with Scotland, Wales and Northern Ireland noted where the rules differ) and US small dental practices: the front desk, insurance and NHS claiming, marketing and the agencies behind it, security and AI limits, the practice-software stack, the handoffs between products, and the job market for the people who could serve them.
Inputs	Research note files compiled by the research team, one per section: economics, staffing, NHS access and leakage benchmarks; practitioners' vocabulary; workflows step by step; the marketing market and its rules; security and AI limits; verification and claiming; the platform audit angle; handoffs; platform reviews; the thirteen-gap ranking; and the employed-versus-freelance market. Autocomplete, job boards and careers pages were read on 16 October 2026.
Sources blocked	Reddit on every mirror; Dentaltown's board and GDPUK's forum beyond titles; Capterra, G2, Software Advice, GetApp and TrustRadius review bodies; Fiverr; Upwork job, hire and catalog pages; Dental Economics; Becker's; hhs.gov and federalregister.gov; the ICO enforcement list; several NHSBSA and NHS England guidance pages; CQC mythbusters in part; the GDC's news pages; most job-ad bodies and practice websites (full list in section 12). The shared web-search budget ran out part-way through every workstream; facts from blocked hosts rest on search snippets, secondary reports or vendor claims and are tagged.
Currency and dates	All currency as published: GBP for UK figures, USD for US figures. Dates are given so freshness can be judged; NHS figures are financial years ending 31 March unless stated.

How to read the evidence tags

[snippet]	the fact was seen only in a search-engine summary because the page blocked the researcher
[vendor]	the vendor's, agency's or consultant's own claim or a vendor-run survey; no independent source confirmed it
[user, date]	a dated user review, forum post or verbatim survey comment
[secondary]	the figure was relayed by trade press, an aggregator or a blog rather than read from the primary report
[official]	a regulator's, public body's or association's own page
[n]	a numbered source; the URL is in the Sources appendix

Prepared for Mustafa Shoukat. Built from the research report *Small dental practice problems UK US* (12 numbered sections, 41 tables, 13 gap cards, a 25-row unverified list). 467 numbered sources.

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Read this first

The eight numbers that carry the report, then the thirteen service gaps in rank order. Each gap has a full card in section 10.

31 of 100
US callers hang up before reaching anyone; 21 book, 13 walk away unbooked, 4.8 get a call back, 0.7 return and book
Patient Prism, 11.55m calls, 8,280 locations, 2025 Patient Prism [4] vendor

42–46%
of diagnosed treatment accepted at the average US practice against 75–83% for the top 10%
Henry Schein One [5] vendor; Henry Schein One [6] vendor

$215,320
average US general dentist net income in 2025, about $60,000 below 2010 in real terms; overhead 60–65%
ADA HPI [1] snippet; Becker's [26] secondary; ADA HPI Q1 2026 [3]

£900m
clawed back from NHS dentists in England over two years for under-delivered UDAs (£479m, £457m); delivery below 96% recovered in full
Dentistry.co.uk [11] secondary; NHSBSA KA-01672 [52]

40%
of adults in England seen by an NHS dentist in 24 months; one general NHS contract in ten given up since 2017
NHSBSA 2024/25 [49]; GDPUK [9] secondary

$2.1bn
US dental spend on benefit verification in 2023; about 12 minutes by phone against 4 electronic; first-pass denials 8–15% against a 4% benchmark
ADA News [7]; DentistryIQ [8] snippet

91% / 77%
US dentists who call hygienist recruiting very or extremely hard / UK owners who had difficulty hiring a dental nurse
ADA HPI [34]; BDA DDRB 2026-27 [14]

$140,000
OCR's October 2026 settlement with a dental practice over unmet record requests; eleven dental penalties since 2019, from $5,000
Paubox [266] vendor; HIPAA Journal [249]

The thirteen gaps, ranked by problem signal and vendor weakness

#	Gap	Problem signal	Vendor coverage	Automatable	Country
1	NHS contract reconciliation: FP17 status against Compass, two-month window, 96% floor, 2026 urgent-care split, performer splits	Very high	Weak	Fully	UK
2	Unscheduled-treatment follow-up and case-acceptance exception tracking	Very high	Partial	Partly	US
3	Lead-to-seated-patient measurement across agency CRM, call tracking and the PMS	High	Weak	Fully	Both
4	Denial rework, appeal-pack assembly and timely-filing deadlines	High	Partial	Partly	US
5	Website and ad compliance checks (GDC items, ASA/CAP, FTC review rule, TCPA wording, Google health-ads policy, HIPAA in replies)	High	Thin	Fully	Both
6	Patient record requests (right of access, SAR) and retention clocks	High	Thin	Partly	Both
7	Eligibility breakdown reconciliation and difficult-payer exception handling	Very high	Strong on the check	Partly	US
8	Recall-list validation and reactivation segmentation	High	Strong on reminders	Fully	Both
9	PMS data clean-up and cross-system reconciliation, including software-stack creep	Medium-high	Unpriced	Fully	Both
10	UK exemption-evidence capture and PCN dispute handling	Medium-high	Weak	Partly	UK
11	Estimate, good-faith estimate and consent-record completeness	Medium-high	Thin	Fully	Both
12	Clinical-note quality and AI-scribe governance (mandatory items, note volume, DPIA)	Medium, new	Thin	Partly	UK lead
13	Security baseline and Security Rule or cyber-insurance readiness evidence	High	Medium	Fully	US lead

Crowded areas that are **not** gaps: AI phone receptionists; SEO, websites and Google Ads; review tools and texting; reminders; verification keystrokes and outsourced billing; imaging AI; PMS set-up gigs and VA agencies; membership-plan and credentialing services (section 10).

SECTION 1

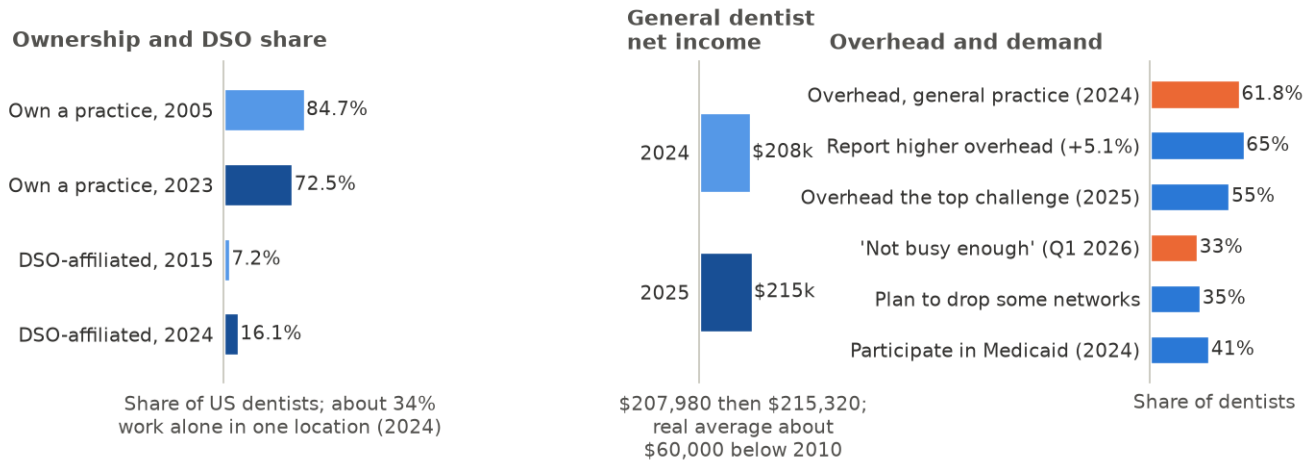
The US dentist nets $215,000 on a 62% overhead; the NHS dentist loses one pound in seven to clawback

In both countries the practice's income arrives through a payer it does not control and a desk it cannot fully staff: the US dentist nets $215,320 on a 62% overhead while a third of the schedule sits empty; the English NHS dentist delivers UDAs against a target and loses the shortfall in full below 96%. Every leakage number in between (missed calls, no-shows, unaccepted treatment) is a vendor's.

The US practice: still owner-run, thinner than it looks

- **72.5% of US dentists owned a practice in 2023** (84.7% in 2005), DSO affiliation rose from **7.2% in 2015 to 16.1% in 2024**, and about 34% worked alone in a single location, so roughly 85% of dentists sit outside a DSO and carry billing, marketing and HR in-house (Becker's on ADA HPI [24] snippet; DrBicuspid [25]).
- Average net income for a private-practice general dentist was **$207,980 in 2024 and $215,320 in 2025**; in real terms the average is down nearly $60,000 since 2010 and the median down more than $64,000 (ADA HPI [1] snippet; Becker's [26] secondary). Overhead runs **60-65% of collections** (61.8% at 100-plus general practices in 2024), 65% of practices reported higher overhead at about +5.1%, and 55% named rising overhead the biggest challenge of 2025 (Clerri [27] vendor; Dental Economics/Levin [2] snippet; Dental Economics 2025 [28]).
- Payers have not moved: dentists told the ADA in early 2026 that reimbursement is 'downright abysmal' and that 'those payers have not increased reimbursement in over 20 years but these asking salaries have now almost tripled'; reimbursement indexes stood at 131-134 against inflation at 138 (2016 = 100), and **35% planned to drop some networks** (ADA HPI Q1 2026 [3] user, 2026).
- Demand softened too: **one-third of dentists report they are 'not busy enough'** (one-quarter in Q1 2024), new-patient wait time fell to 12.4 days, one in five lack enough administrative staff, 41% participate in Medicaid, and about 64% of active dentists sit in PPO networks with write-offs of 21-30% of fees (ADA HPI Q1 2026 [3]; ADA News, Dec 2025 [32]; DenteMax [30] vendor).
- The software bill creeps: a consultant describes practices with '20 to 25 connected software tools', a $300-a-month reminder program 'barely used', $450 a month still paid to Dentrix for texting after switching to Weave, and merchant fees of $2,500-$3,000 a month (Thriving Dentist, 14 Oct 2026 [33] vendor).

US practice economics: the average general dentist netted $215,320 in 2025, about $60,000 below 2010 in real terms, on overhead of 60-65%, while DSO affiliation more than doubled and ownership fell



Sources: ADA HPI dental practice trends [1] (snippet) and Becker's [24] [26] (secondary); DrBicuspid on ADA HPI [25]; Dental Economics/Levin survey [2] (snippet); Clerri [27] (vendor, 100+ practices); ADA HPI Q1 2026 [3]; Dental Economics 2025 survey [28].

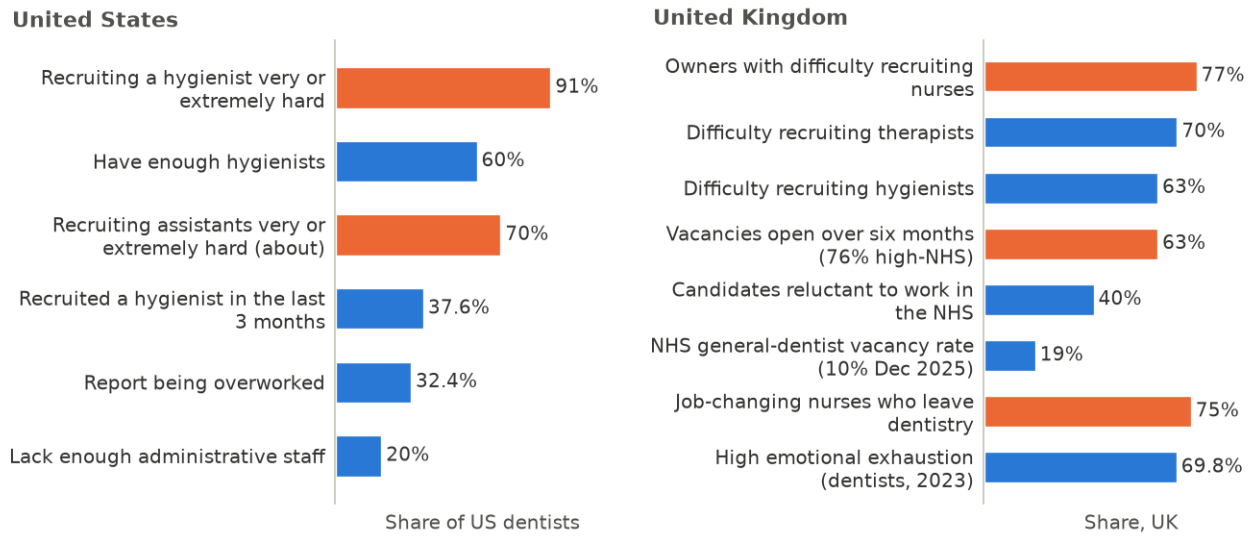
Table 1.1 US practice measures

US practice measure	Figure	Population and date	Source
Ownership and DSO share	72.5% owned a practice (2023; 84.7% in 2005); DSO-affiliated 16.1% (2024; 7.2% in 2015); 26.5% of dentists under 10 years out; ~34% solo single location	ADA HPI	Becker's Dental [24] snippet; DrBicuspid [25]
Net income, general dentist	$207,980 (2024), $215,320 (2025); specialists $346,520; owners $217,781 vs non-owners $160,891; real average down ~$60,000 since 2010	ADA HPI Survey of Dental Practice	ADA HPI [1] snippet; Becker's [26] secondary
Overhead	60-65% of collections; 61.8% general practice (2024, 100+ practices); 65% report higher overhead, +5.1%	DE/Levin survey; vendor analysis	Dental Economics [2] snippet; Clerri [27] vendor
Top 2026 concerns, non-DSO	Insurance (low reimbursement, denials) 55.2%; staffing 53.8%; overhead 42.3%	ADA HPI Q4 2025	Oral Health Group [29] secondary
Schedule fullness	One-third "not busy enough" (one-quarter in Q1 2024); new-patient wait 12.4 days	ADA HPI Q1 2026	ADA HPI [3]
New patients, collection, retention	30 new patients/month (top 10%: 81); collection rate 80% (97%); retention 70% (94%); new-patient lead time 25 days (4.5)	Henry Schein One Catalyst Index, 1-7 location practices, 22 Apr 2026	Henry Schein One [5] vendor
Supplies and tariffs	Equipment and supply prices +6% in year to Feb 2026; "The tariffs are killing our practice"	ADA HPI Q1 2026	ADA HPI [3] user, 2026
PPO participation and write-offs	~64% of active dentists in PPOs (116,978 of 181,709); ~14 carriers each (2017); write-offs 21-30% of fees	DenteMax FAQ; NADP; consultants	DenteMax [30] vendor; DrBicuspid [31]
Medicaid	41% of dentists participate (2024, flat since 2015); FFS reimbursement below 50% of charges in most states; 49% of child enrollees saw a dentist in 2024	ADA HPI	ADA News, Dec 2025 [32]
Software creep	"20 to 25 connected software tools"; $300/month reminder program "barely used"; Dentrix texting $450/month paid after switching to Weave; merchant fees $2,500-$3,000/month	Consultant, 14 Oct 2026	Thriving Dentist [33] vendor

Staffing: a retention problem, not a supply problem

- **Only 60% of US dentists have enough hygienists and 91% of those recruiting call it very or extremely challenging**, even though 2025 produced the most hygiene graduates ever; about 70% say the same of assistants, 37.6% recruited a hygienist in the last three months (ADA HPI, Apr 2026 [34]; ADA HPI Q1 2026 [3]). Owners' words: 'Not a single application in 9 months'; '3 months of sponsored advertising... and one tattoo artist applied'; 'We now have to use a recruiter and pay 17% of first year's wage' (ADA HPI Q1 2026 [3] user, 2026).
- In the UK the GDC register grew to 131,680 in 2025 with international joiners outnumbering UK graduates for the first time, yet **77% of owners who tried to recruit dental nurses had difficulty, 63% of owners with vacancies had posts open more than six months** (76% at high-NHS practices), the NHS general-dentist vacancy rate was 19% in the survey the BDA cites (10% in December 2025), and **75% of dental nurses who change jobs leave dentistry altogether** (GDC, 7 May 2026 [35] snippet; BDA DDRB 2026-27 [14]; Dentistry.co.uk, 15 Nov 2024 [36]; Dentistry.co.uk, 31 Mar 2026 [37]).
- A UK dental nurse earns **£14.73 an hour**, within about £2 of a receptionist (£13.92), and nurses call themselves 'glorified cleaners', 'hidden' and 'overworked and underpaid'; every dentist appointment needs a nurse, so an empty nurse post removes surgery sessions and UDA delivery with them (Indeed UK [38]; Dentistry.co.uk, 8 Apr 2024 [39] user, 2024).
- Morale: 32.4% of US dentists report being overworked; **69.8% of UK dentists scored high emotional exhaustion** (n=1,507, 2023), 56% of owners report low morale, 42% intend to retire or leave within five years, and 63% of GDPs expect to reduce NHS work against 1% who expect to increase it (ADA HPI, Jul 2026 [45]; PMC12334352 [46]; BDA DDRB [14]; GDPUK, 15 Sep 2026 [48]).

Staffing: 91% of US dentists recruiting a hygienist call it very or extremely hard and only 60% have enough; 77% of UK owners who tried to hire a dental nurse had difficulty and 63% of vacancies stayed open more than six months



Sources: ADA HPI hygienist shortage, Apr 2026 [34]; ADA HPI Q1 2026 [3]; ADA HPI Jul 2026 [45]; BDA DDRB 2026-27 evidence [14]; Dentistry.co.uk 15 Nov 2024 [36] and 31 Mar 2026 [37]; PMC12334352 [46] (n=1,507, 2023).

Table 1.2 Staffing measures, US against UK

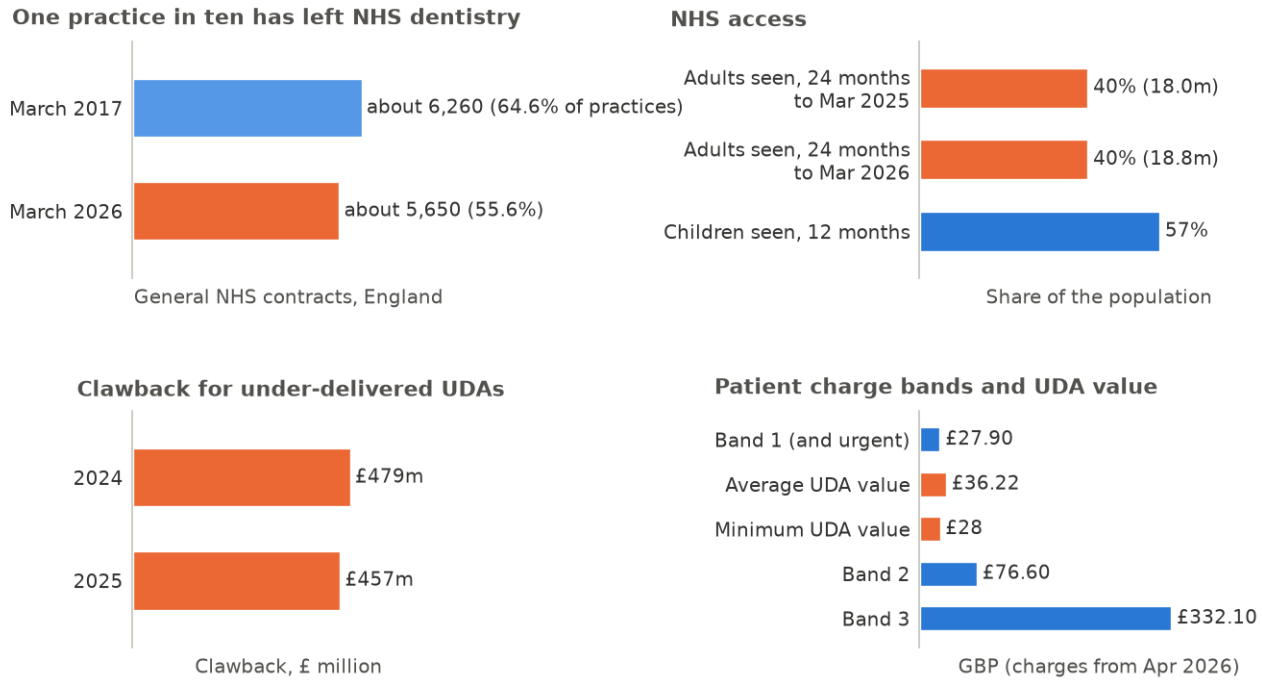
Staffing measure	US	UK	Source
Hygienist / therapist recruiting	91% very or extremely challenging; only 60% have enough; 37.6% recruited in last 3 months	63% had difficulty recruiting hygienists; 70% therapists	ADA HPI, Apr 2026 [34]; BDA DDRB [14]
Assistant / nurse recruiting	~70% very or extremely challenging; "candidate pool is large but VERY shallow"; 5% "do not show up to interviews"	77% of owners who tried had difficulty; 40% said candidates reluctant to work in NHS dentistry; 1,161 unfilled NHS nurse posts (8%, Mar 2024)	ADA HPI Q1 2026 [3]; BDA DDRB [14]; Dentistry.co.uk, 15 Nov 2024 [36]
Admin staff	One in five lack enough; more than half of those who tried say recruiting admin staff is very or extremely challenging	Receptionist £13.92/hr; nurse £14.73/hr; hygienist £38.96/hr (Indeed, Oct 2026)	ADA HPI Q1 2026 [3]; Indeed UK [40]
Vacancy duration	One practice ~18 months without a hygienist; recruiter fee 17% of first-year wage	63% of vacancies open >6 months (76% high-NHS); 2,749 FTE NHS dentist vacancies open ~180 days; 1,823 failed NHS recruitment processes Apr-Dec 2024	ADA News, 27 Feb 2025 [13]; The Probe, Jan 2025 [41]; BDA DDRB [14]
Pay	Dental assistants median $47,300 (BLS May 2024); front office $18-$30/hr in ads	Dental nurse £14-£17/hr in ads; receptionist £21,840-£24,465; practice manager £28,000-£40,000	Stealth Agents on BLS [42] secondary; DentalPost [43]; Reed [44]
Morale and burnout	32.4% of dentists report being overworked	69.8% of dentists high emotional exhaustion (n=1,507, 2023); 56% of owners low morale; 42% intend to retire or leave within 5 years	ADA HPI, Jul 2026 [45]; PMC12334352 [46]; BDA DDRB [14]
Leaving the field	"Roughly half of dental professionals are open to a job change"	75% of job-changing dental nurses leave dentistry (17% of hygienists); 63% of GDPs expect to reduce NHS work, 1% to increase	DentalPost [47] vendor; Dentistry.co.uk, 31 Mar 2026 [37]; GDPUK, 15 Sep 2026 [48]

The NHS contract: a tenth of practices gone, £900m clawed back

- NHS general dental contracts in England fell from **about 6,260 in March 2017 to about 5,650 in March 2026** while total practices rose by more than 400, so the share with a general NHS contract fell from about 65% to 56% and roughly 1,000 more practices are now private-only (GDPUK, 24 Aug 2026 [9] secondary; The Probe, 1 Oct 2026 [10] secondary).
- Activity rises, coverage does not: 76.8 million UDAs and 37.6 million courses in 2025/26, yet **18.8 million adults (40%) seen in 24 months**, the same 40% as the year before; 78% of those who tried for an NHS appointment succeeded but only 38% of those seeking a new appointment did (NHSBSA 2024/25 [49]; NHS England GP Patient Survey [50]).

- The money mechanism is the UDA: the average contract pays **£36.22 per UDA** (£28 minimum), delivery below **96%** of target is clawed back in full with a breach notice, 96-100% is carried forward, above 102% is not paid, and about £1 in every £7 paid to NHS dentists was recovered over two years (**£479m in 2024, £457m in 2025**) (GDPUK [51]; NHSBSA KA-01672 [52]; Dentistry.co.uk, 5 Mar 2026 [11] secondary). One owner 'slightly under 96%' asked for leeway and was told 'no, you'll need to pay the clawback money'; focus-group dentists call the contract a 'hamster on the wheel' (GDPUK blog [53] snippet; PMC12065166 [54] user, 2023).
- The April 2026 amendment added an urgent-care course credit of **£75 (£15 fixed plus £60 activity) with a minimum of 8.2% of contract value**, complex-care pathways at £284, £709 and £248, a £3,400 quality-improvement payment and £213 funded appraisals; Compass shows only the £15 element, so 'you cannot look at Compass and know where you are anymore' (Price Bailey [65] secondary; GDPUK, 13 Jul 2026 [12]). NHS-active dentists in England earned £83,000 taxable in 2024/25 after a fall of almost 35% over the decade to about 2022 (Dentistry.co.uk, 3 Aug 2026 [55]).

NHS England: contracts fell from about 6,260 to 5,650, only 40% of adults saw an NHS dentist in 24 months, and about £900m was clawed back over two years at £36.22 per UDA



Sources: Nuffield Trust and BDA via GDPUK, 24 Aug 2026 [9] (secondary); NHSBSA dental statistics 2024/25 [49]; Dentistry.co.uk, 5 Mar 2026 [11] (secondary); GDPUK UDA rates [51]; NHSBSA KA-02003 [61] (charges from Apr 2026); below 96% of the annual UDA target is recovered in full [52].

Table 1.3 UK NHS and access measures

UK NHS and access measure	Figure	Population and date	Source
NHS contracts	~6,260 (Mar 2017) to ~5,650 (Mar 2026); share of practices with general NHS contract 64.6% to 55.6%; 608 practices ceased, 135 since Jul 2024	Nuffield Trust, BDA	GDPUK, 24 Aug 2026 [9] secondary; BADN [57]
Activity	73m UDAs, 35m courses (2024/25); 76.8m UDAs (+5.1%), 37.6m courses (2025/26); Band 1 = 60% of courses; urgent 10%	NHSBSA	NHSBSA 2024/25 [49]; The Probe [10] secondary
Adult coverage	18m adults (40%) seen in 24 months to Mar 2025; 18.8m (40%) to Mar 2026; children 57% in 12 months	NHSBSA	NHSBSA [49]
Access attempts	78% of those who tried succeeded (South West 66%); 51.2% tried in two years; 38% of those seeking a new appointment succeeded; 52% in Sussex struggled to find an NHS dentist (Jan 2025)	GP Patient Survey Jan-Mar 2025; Healthwatch Sussex	NHS England [50]; Healthwatch Sussex [58]
Clawback	~£900m over two years; £479m (2024), £457m (2025); 32% of London NHS practices clawed back in 2022/23; BNSSG ICB £14.4m-£17.0m a year	BBC via trade press; London Assembly; ICB FOI	Dentistry.co.uk, 5 Mar 2026 [11] secondary; London.gov.uk [59]; BNSSG FOI [60]
UDA value and charges	£36.22 average per UDA; £28 minimum; Band 1 £27.90, Band 2 £76.60, Band 3 £332.10, urgent £27.90	2024-25 data; Apr 2026 charges	GDPUK [51]; NHSBSA KA-02003 [61]

UK NHS and access measure	Figure	Population and date	Source
Hand-backs	16 contracts (114,900 UDAs) in NENC 2023-24; 12 in BNSSG 2020-2026; 7 in Powys 2025-26; Bupa closing or selling 85 practices	Local FOIs	NENC FOI [62]; Powys FOI [63]; City AM [64] snippet
Dentist income	£83,000 England (2024/25, +6.1%); Wales £86,100; NI £87,600; Scotland £99,100	NHS England Dental Earnings and Expenses	Dentistry.co.uk, 3 Aug 2026 [55]
NHS intentions	63% expect to reduce NHS work; 10% think 2026 changes improved NHS dentistry; 44% of owners intend to reduce NHS commitment within five years	BDA, Sep 2026; DDRB evidence	GDPUK, 15 Sep 2026 [48]; BDA DDRB [14]
2026 contract changes	Urgent course £75 (£15 fixed + £60 activity), minimum 8.2% of contract value; complex-care pathways £284/£709/£248; £3,400 quality-improvement payment; £213 funded appraisals; fissure sealants to Band 2	From Apr-Jun 2026	Price Bailey [65] secondary; Dentistry.co.uk, 2 Feb 2026 [66]

Private UK dentistry grows, and brings a complaint load the NHS never carried

- **20% of Britons now use private dental care**; one practice went 'from around 440 patients on plan to more than 2,000 in just six months'; the Dental Complaints Service resolved **985 cases in 2024, up 111%**, access complaints rose 384%, and GDC fitness-to-practise concerns rose 26% to 1,766 in 2025 with an average assessment stage of 78 working weeks (GDPUK [67] snippet; Dentistry.co.uk, 27 Oct 2025 [68] vendor; Dentistry.co.uk, 20 Mar 2025 [69]; Dentistry.co.uk, 3 Jun 2026 [70]).
- Which? surveyed 1,009 private patients with a problem: **22% 'paid more than they had been quoted'**, 21% found prices unclear before treatment, 16% felt pressured to agree to treatment and 11% to sign up to a payment plan; six in ten did not complain (GDPUK, 6 Oct 2026 [71]). A private consultation runs a median £65 across 2,233 practices, hygiene £83, emergency £75, plans £15-£50 a month (TreatCompare, Mar-Apr 2026 [72]).
- Regulator fear is measurable: 79% of dentists name complaints or litigation as their top stressor and 72.8% are concerned about the GDC; fewer than half find fitness-to-practise fair (BDJ 2026 [78]).

Table 1.4 UK private and complaints measures

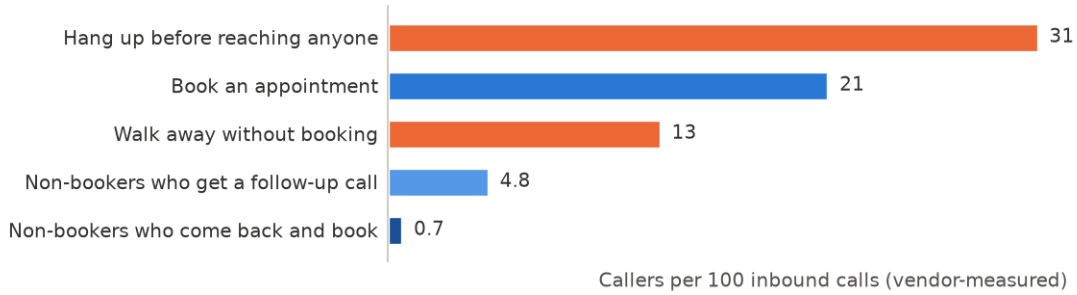
UK private and complaints measure	Figure	Date	Source
Private fees	Consultation median £65 (£23-£180; Central London £78); hygiene £83; X-ray £40; emergency £75; plans £15-£50/month	TreatCompare, 2,233 practices, Mar-Apr 2026	TreatCompare [72]
Membership plans	Denplan 6,700+ dentists, 1.4-1.5m patients; mydentist plans from £10.50/month; Rodericks from £24.14/month	2026	Denplan [73]; mydentist Hull [74]; Rodericks [75]
Goodwill	109% of gross fees overall; NHS 135%; private 95%; mixed 113%	NASDAL, quarter to 30 Apr 2026	GDPUK [76]
Practice sales	Price +2.9% in 2025; first-time buyers 33%; corporates 7% of completions; 58% considering a sale or purchase in 2026	Christie & Co, 15 Jan 2026	Christie & Co [77]
Complaints	DCS 465 (2023) to 985 (2024), +111%; new cases +137%; access complaints 26 to 126; refunds £673,500; GDC referrals 59	DCS review	Dentistry.co.uk, 20 Mar 2025 [69]
GDC fitness to practise	1,766 concerns (+26%); 761 open; 78 working weeks average assessment; erasures 18	2025	Dentistry.co.uk, 3 Jun 2026 [70]
Patient experience	39% cost excessive; 22% paid more than quoted; 21% prices unclear; 16% pressured; six in ten did not complain	Which?, 1,009 patients, 6 Oct 2026	GDPUK [71]
Regulator fear	79% name complaints or litigation as top stressor; 72.8% concerned about the GDC (2,053 dentists, 2019); fewer than half find FtP fair	BDJ; GDC survey 2026	BDJ 2026 [78]; GDPUK, 29 Sep 2026 [79]

Leakage: every number is a vendor's, and they roughly agree

- No regulator or academic body publishes a dental missed-call, lead-response or recall rate in either country. The largest dataset, **Patient Prism's 11.55 million calls across 8,280 US locations in 2025**, shows 31 of 100 callers hanging up before reaching anyone, 21 booking, 13 walking away unbooked, 4.8 getting a follow-up call and 0.7 coming back to book; reasons for not booking were timing 45.2%, money and insurance 34.4%, service fit 10.3% and connection failed 9.3% (Patient Prism, 2 Jul 2026 [4] vendor). Three other vendors put unanswered calls at 35%, 37% and 38% on different denominators (DenteMax [80] vendor; Dentaltown, 5 Aug 2026 [18] vendor; Peerlogic, Feb 2026 [81] vendor).

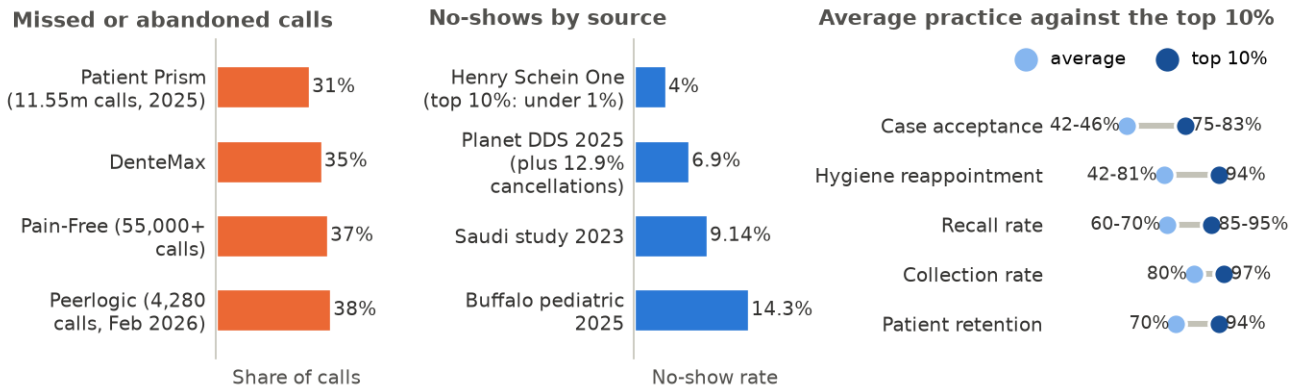
- No-shows run **4%** on Henry Schein One's data (top 10% under 1%), 6.9% on Planet DDS's 2025 data plus 12.9% cancellations, and 9.1-14.3% in peer-reviewed studies; the spread is definitions and payer mix (Henry Schein One [5] vendor; Ainora on Planet DDS [82] vendor). Once in the chair, **42% (2026) to 46% (2024) of diagnosed treatment is accepted against 75-83% for the top 10%**, leaving '$1 to $1.5 million' unscheduled per average practice; hygiene reappointment runs 42-81% against 94%, recall 60-70% against 85-95%, collection 80% against 97% (Henry Schein One [6] vendor; Ainora [94] vendor).
- The UK has no published DNA rate. The only primary enquiry data are a 2021 mystery-shop of all 74 East Sussex practices (**13.5% could not be contacted** even after a follow-up call; some 'did not log enquiries or keep waiting lists at all') and a 2017 survey of 167 principals (almost two-thirds track leads on paper diaries, Post-its or nothing; 'two to three weeks between an enquiry and first contact') (Healthwatch East Sussex, Mar 2022 [83]; Dentistry.co.uk, 22 May 2019 [84] vendor). Dormant lists run 800-2,000 patients per practice and reactivation recovers 10-25% (ACT Dental, 24 Jul 2026 [96] vendor).

Of 100 callers to a US dental practice, 31 hang up before reaching anyone, 21 book, 13 walk away unbooked, 4.8 get a follow-up call and 0.7 come back and book



Source: Patient Prism, 2 Jul 2026 [4] (vendor data: 11.55 million calls across 8,280 US locations in 2025). Reasons for not booking: timing 45.2%, money and insurance 34.4%, service fit 10.3%, connection failed 9.3%; 1,445,814 patients walked away without an appointment and 890,621 never got a callback.

Vendor benchmarks: 31-38% of calls go unanswered, no-shows run 4-14% depending on definition, and average practices sit far below the top 10% on acceptance, recall and collection



Sources: Patient Prism [4], DenteMax [80], Pain-Free Dental Marketing via Dentaltown [18], Peerlogic [81] (all vendor); Henry Schein One Catalyst Index 2026 [5] (vendor), Planet DDS via Ainora [82] (vendor), peer-reviewed studies as relayed by Patient Prism [88]; recall benchmarks Ainora [94] (vendor). No regulator or academic body publishes these rates in either country.

Table 1.5 Leakage measures, all vendor-measured

Leakage measure	Figure	Who measured it, date	Source
Missed or abandoned calls	31 of 100 hang up before an agent; 35%; 37% of 55,000+ tracked calls; 38% of 4,280 calls	Patient Prism 2025 data; DenteMax; Pain-Free Dental Marketing; Peerlogic Feb 2026	Patient Prism [4] vendor; Dentaltown [18] vendor; Peerlogic [81] vendor
After-hours share	~75% of missed calls after hours; ~30% of calls arrive outside hours; only ~14% leave a voicemail	Convirza; Peerlogic	Convirza [85] vendor
Call to booking	21 of 100 book; calls-to-booking 40%; new-patient conversion 25% vs existing 56%; ~23% of new-patient calls convert	Patient Prism; Peerlogic; VoiceStack	Patient Prism [4] vendor; Peerlogic [81] vendor

Leakage measure	Figure	Who measured it, date	Source
Follow-up of non-bookers	4.8 of 100 get a follow-up call; 38% follow-up rate; 0.7 of 100 recovered	Patient Prism 2025-2026	Patient Prism [4] vendor; Viva [86] vendor
Lead response	26 hours average without automation (second-hand); cross-industry 42 hours, 37% within an hour, 23% never	Weave via Viva; HBR audit	Viva [87] vendor, second-hand
No-shows	4% (top 10% under 1%); 6.9% (2025) and 12.9% cancellations; 9.14%; 14.3% pediatric; 3% with live call vs 39% with no answer	HSO; Planet DDS; Saudi 2023; Buffalo 2025; Teo 2017	Henry Schein One [5] vendor; Ainora [82] vendor; Patient Prism [88] vendor
Case acceptance	42% (2026) and 46% (2024) vs 75-83% top 10%; treatment-dollar 39% vs 58%; 49% of practices between 40% and 70%	HSO; Dental Intelligence 2024; 2025 Dental Industry Outlook	Henry Schein One [5] vendor; Webtonic [89] vendor, second-hand; BusinessWire, 13 Mar 2025 [90] snippet
Unscheduled treatment	"$1 to $1.5 million" a year per average practice; "$500,000 to $1 million" (AADOM/ADA-attributed)	HSO 2024; Clerri	Henry Schein One [6] vendor; Clerri [91] vendor
Hygiene reappointment	"as low as 42%"; 63% platform average; 77% vs 94%; 81% vs 94% (DI 2024)	Dental Intelligence; Becker's; Webtonic	DentistryIQ [92]; Becker's [93] secondary; Webtonic [89] vendor
Recall and reactivation	Recall 60-70% average vs 85-95% top; reactivation recovers 10-25% of lapsed lists; 800-2,000 dormant patients per practice; "40% of patients who do not make follow-up appointments never come back"	Ainora, Clerri, ACT Dental (unsourced)	Ainora [94] vendor; Clerri [95] vendor; ACT Dental, 24 Jul 2026 [96] vendor
UK enquiry handling	13.5% of 74 practices unreachable even after follow-up; 63.5% not accepting new NHS patients; 47% would offer private to NHS enquirers; two-thirds track leads on paper or nothing; 2-3 week gaps	Healthwatch East Sussex Dec 2021; DenGro 2017	Healthwatch [83]; Dentistry.co.uk [84] vendor

SECTION 2

Their own words: the US says "no-show", "breakdown of benefits" and "denied three times"; the UK says "FTA", "UDAs", "clawback" and "CQC"

The two markets describe the same desk with different nouns, and the switch matters for every keyword list: the UK employs a dental nurse and a practice manager, claims UDAs on an FP17 through Compass and fears clawback and the GDC; the US employs a dental assistant and an office manager, verifies eligibility, gets a breakdown of benefits, works denials and calls a missed visit a no-show.

- **UK**: 'dental nurse', 'practice manager' (often 'CQC registered manager'), 'diary', 'rota', 'UDAs', 'FP17', 'Compass', 'exemption', 'course of treatment', 'Band 1, 2 or 3', 'clawback', 'GDC', 'CQC inspection', 'Denplan', 'practice plan', 'FTA' ('failed to attend', 'dental fta policy', 'nhs dental fta policy') (Reed [44]; NHSBSA activity and payments [97]; Google autocomplete, UK [99]).
- **US**: 'dental assistant', 'office manager', 'schedule', 'eligibility', 'breakdown of benefits (BOB)', 'claims', 'CDT codes', 'denials', 'appeals', 'no-show' ('no show fee', 'no show policy', 'no show code'), 'in-network', 'fee-for-service', 'membership plan' (DentalPost [43]; ADA CDT glossary [98]; Google autocomplete, US [100]).
- The strongest dated voices are from association surveys, focus groups and trade press rather than forums, because Reddit, Dentaltown's board and GDPUK's forum were closed to the researcher; the five pains they cluster around are phones and no-shows, insurance phone work, money, staff, and (UK) the NHS contract and regulation.

Table 2.1 Concept by concept: UK words against US words

Concept	UK words	US words
Chairside staff	Dental nurse, trainee dental nurse, DCP, therapist	Dental assistant, RDA, EFDA, hygienist
Desk and admin lead	Dental receptionist; practice manager; CQC registered manager	Front desk, patient coordinator, treatment coordinator, insurance/financial coordinator; office manager, practice administrator
Appointment book	Diary, rota	Schedule, huddle, route slip
Payer	NHS contract, UDA, FP17, Compass, eDEN, band charge, exemption, PCN; Denplan, practice plan	PPO, in-network, fee-for-service, Medicaid/CHIP, eligibility, breakdown of benefits, CDT code, claim, EOB/ERA, denial, appeal, predetermination, credentialing, CAQH; membership plan
Missed visit	FTA, failed to attend, DNA	No-show, broken appointment, same-day cancellation
Follow-up visits	Recall, recall letter, 24-month recall, NICE interval	Recall, recare, reactivation, unscheduled treatment
Regulators and rules	GDC, CQC, HTM 01-05, IR(ME)R, DSPT, UK GDPR, ASA/CAP, ICO	State dental board, HIPAA, OCR, OSHA, DEA, TCPA, FTC review rule, No Surprises Act
Software	Exact (Software of Excellence), Dentally, R4/Sensei, Aerona, iSmile, Systems for Dentists, DenGro, Kiroku	Dentrix, Dentrix Ascend, Eaglesoft, Open Dental, Curve, Denticon, Fuse, Oryx, tab32; Weave, NexHealth, RevenueWell, Solutionreach, Dental Intelligence, Adit

Verbatim pains, dated

- US insurance work: 'denied three times, and she didn't know why'; 'over an hour and a half' on the phone; 'Claim denied,' 'Resubmit,' 'Appeal,' 'Sit on hold'; 'The insurance company doesn't pay this employee. You do.'; calls are 'soul-crushing' (Yahoo/Daily Dot, 2026 [103] user, 2026; Dentaltown, 13 Aug 2026 [104] vendor).
- UK contract and records: 'hamster on the wheel'; 'Universally Detested Acronym'; 'unknowingly driving a car with a faulty speedometer'; 'The volume of AI notes is getting incredibly scary'; 'computer files that are the length of textbooks'; a missed note could be 'career-ending' (PMC12065166 [54] user, 2023; GDPUK, 13 Jul 2026 [12]; GDPUK, 16 Oct 2026 [109]).
- AI tools: 'the latest grift' (AI receptionists); 'confidently made things up'; 'Patients hung up on robot voice'; 'They are using AI to try to upsell you on dental procedures.' (Dentaltown, 7 Mar 2025 [115] vendor). Marketing: 'no idea' of ROI; 'You need to give it more time'; 'your marketing isn't broken, your front desk is' (Dentistry.co.uk, 29 Dec 2021 [17]; Dentaltown, 5 Aug 2026 [18] vendor).

Table 2.2 Practitioners' words by pain, with who said them and when

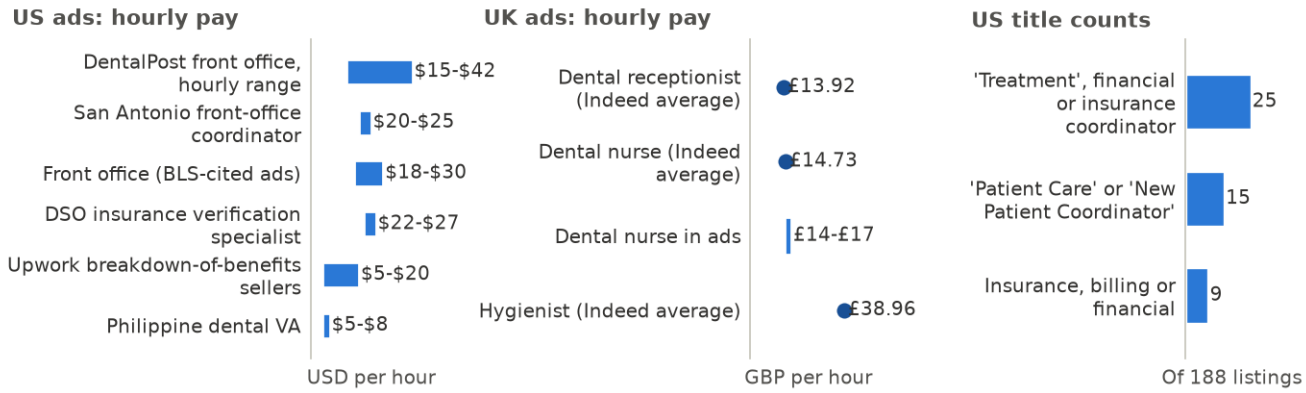
Pain	Practitioners' words (verbatim)	Who said it, when	Source
Phones and no-shows (US)	"the biggest pain in the butt"; "whatever you do you lose"; patients "still no-show or cancel the day before"; repeat offenders on a "call-in basis"; "no no-show or short notice cancellation policy in place and no follow-up on missed appointments"	Dentaltown owners (old thread); AAPD newsletter	Dentaltown [101] user, 2002; AAPD PDF [102]
Insurance phone work (US)	"denied three times, and she didn't know why"; "over an hour and a half" on the phone; "Claim denied," "Resubmit," "Appeal," "Sit on hold"; "The insurance company doesn't pay this employee. You do."; "the PPO staffing tax"; "on hold with the payer while the schedule backs up behind us"; calls are "soul-crushing"; "a deep sense of dread"; "When a Post-Payment Audit Turns Into a Recoupment Battle"	Dr Katherine Sislow, Jan 2026; Jordon Comstock, 13 Aug 2026; office manager via vendor; Dentaltown thread title Oct 2026	Yahoo/Daily Dot [103] user, 2026; Dentaltown [104] vendor; Staffingly [105] vendor; Dentaltown thread [106]
Money (US)	"Managed care/dental insurance reimbursement rates are downright abysmal."; "Patients will NOT pay for teeth if they can't buy gas and groceries."; "People tend to consider dentistry elective"; "sometimes the same feature is in four places, and you're paying for it four times"; "It's outta sight, outta mind."	ADA HPI verbatim, Q1 2026; Thriving Dentist, 14 Oct 2026	ADA HPI [3] user, 2026; Thriving Dentist [33] vendor
Staff (US)	"Not a single application in 9 months."; "one tattoo artist applied"; "The candidate pool is large but VERY shallow."; "stealing assistants from other practices"; "take on other positions' duties... because the office is understaffed"; "lack of a raise for years at a time"	ADA HPI Q1 2026; PayScale front-desk reviews	ADA HPI [3] user, 2026; PayScale [107] user, undated
NHS contract (UK)	"hamster on the wheel"; "stick after, stick after stick after stick"; "a lot of duplicate work and nobody's sharing the information with each other"; "we're not charity workers"; "Financially, it can't do it"; "run out of UDAs"; "Universally Detested Acronym"; "chasing UDA targets"; "You cannot look at Compass and know where you are anymore"; "unknowingly driving a car with a faulty speedometer"	North East focus groups, Jul 2023; GDPUK thread titles and poll; Peter Ingle, 13 Jul 2026	PMC12065166 [54] user, 2023; GDPUK [108]; GDPUK, 13 Jul 2026 [12]
Records and regulation (UK)	"The volume of AI notes is getting incredibly scary."; "computer files that are the length of textbooks"; "working through their lunch hours and logging in the evenings"; a missed note could be "career-ending"; "the wait is the hardest part" (GDC); "fear of regulation shapes how dental professionals practise"; inspection pressure "gets worse with last-minute preparation"; "about two weeks' warning"	CDO Jason Wong, 9 Oct 2026; GDC's Theresa Thorp, Sep 2026; practice managers	GDPUK, 16 Oct 2026 [109]; GDPUK, 29 Sep 2026 [79]; Dentistry.co.uk, 20 Aug 2024 [110]; Dental Nurse Network [111]
Nurses and pay (UK)	"a cleaner earns more money than most dental nurses"; "We feel hidden!"; "glorified cleaners"; "overworked and underpaid"; "worthless, angry and disrespected"; "second jobs to afford to live"; "why are dental nurses paid so little" (autocomplete)	Dentistry.co.uk readers, 8 Apr 2024; Dentistry Census 2021	Dentistry.co.uk [39] user, 2024; Dentistry.co.uk [112] user, 2021
Marketing and agencies (both)	"Most dental social media marketing turns out a big bust"; "generic menu-style packages"; "canned, white-labeled daily posts"; "no idea" of ROI; "You need to give it more time"; "locked into a long-term SEO and website agreement, that wasn't showing any real ROI"; leads "turned up not even knowing the cost or what they wanted"; "your marketing isn't broken, your front desk is"	Dentaltown; Chris Barrow, 29 Dec 2021; Trustpilot on Dentology; agency, 5 Aug 2026	Dentaltown [113]; Dentistry.co.uk [17]; Trustpilot [114] user; Dentaltown [18] vendor
AI tools (both)	"the latest grift" (AI receptionists); "confidently made things up"; patients "got frustrated having to repeat their names and birthdates over and over"; "Patients hung up on robot voice—lost 3 new patients in one week"; "AI is definitely not the answer"; "They are using AI to try to upsell you on dental procedures."; "Why didn't you drill it?"	Dentaltown post, 7 Mar 2025; Reddit users via vendor, Mar 2026; Joanna Stern via Futurism, 16 May 2026	Dentaltown [115] vendor; Swiss Monkey [116] vendor, unverified quotes; Futurism [117]
Patients (UK)	"paid more than they had been quoted"; "pressured to sign up to a dental payment plan"; "their expectations are here, and what you can realistically provide is usually somewhere else"; "Just because they've Googled it does not mean that they are an expert"	Which?, 6 Oct 2026; BDA dento-legal advisers, 9 Oct 2026	GDPUK [71]; GDPUK [118]

The duty vocabulary, from job ads and VA listings

- A San Antonio front-office post at $20-$25 an hour wants the coordinator 'verifying patient dental insurance benefits before appointments and confirming eligibility, deductibles, annual maximums, frequencies, waiting periods, and limitations', 'submitting claims, tracking their status, and appealing denials' and 'submitting pre-treatment estimates and pre-authorizations as needed'; others require 'dental insurance verification experience (required)' and 'OpenDental experience required' (DentalPost 557606 [119]; DentalPost 557359 [120]). Of 188 DentalPost front-office listings, about 25 carry 'Treatment' in the title, 15 'Patient Care' or 'New Patient Coordinator', 9 insurance, billing or financial; pay runs **$15-$42 an hour and $30,000-$80,000 salaried** (DentalPost [43]).

- UK receptionist ads list 'completing NHS and private scheme forms', 'chasing outstanding patient debt', a 'monthly debtors list' and 'liaising with dental labs and suppliers' at **£21,840-£24,465**; practice-manager ads lead with 'Ensure compliance with CQC, GDC, HTM 01-05 and NHS standards', 'Manage diaries, rotas, and daily operations', 'banking reconciliation' and profit and loss at **£28,000-£40,000**; Indeed UK keeps a landing page called 'Cqc Registered Manager Dental jobs' (Rodericks [121] snippet; findajob 17683574 [122] snippet; BDJ Jobs 535611 [123] snippet; Indeed UK [124]).
- Offshore VA ads list what the in-office desk cannot keep up with: 'Make recall and reminder calls for upcoming or overdue appointments'; 'Contact patients with pending treatment plans or unscheduled visits'; 'call new potential patients from the practice's lead system to get them scheduled'; 'dental insurance verification: confirming eligibility, getting benefit breakdowns, and updating patient records', at **$5 an hour plus $3 per new patient lead booked**, $2.50 an hour plus $2 per qualifying appointment, or $400-$700 a month (OnlineJobs.ph 1491618 [125]; OnlineJobs.ph 1505586 [126]; OnlineJobs.ph search [127]).

Job ads: US front-office pay runs $15-$42 an hour, UK receptionists and nurses earn £13.92-£17 an hour, and 49 of 188 DentalPost front-office titles name treatment, patient-care, insurance or billing roles



Sources: DentalPost front-office listings [43] [119] [120]; MB2 Dental job [326]; Upwork job [131] (snippet); OnlineJobs.ph [127]; Indeed UK salaries [38] [40]; Reed [44]; Rodericks [121] (snippet); BDJ Jobs [123] (snippet). Salaried US front-office roles $30,000-$80,000; UK receptionist £21,840-£24,465 and practice manager £28,000-£40,000 a year. Title counts are from the 188 DentalPost listings read, not board-wide statistics.

What autocompletes and what does not

- The US has a full 'ai receptionist for dental office' cluster with cost, reviews, Reddit and country modifiers (a crowded, late-stage category), an insurance cluster in job-seeker and vendor language ('dental insurance verification jobs remote', 'outsource dental billing services', 'dental claim denied due to frequency', 'dental claim denial codes') and a recall cluster ('dental recall text message sample', 'dental reactivation letter') (Google autocomplete [100]; Google autocomplete, marketing [128]).
- UK practice-side phrases are logins and deadlines ('compass nhs dental login', 'nhs dental compass cut off dates', 'nhs dental contract uplift 2026/27'), compliance ('cqc dental inspection checklist', 'gdc social media guidance', 'gdc advertising guidance') and staffing ('dental nurse shortage occupation list', 'why are dental nurses paid so little'); patient phrases dominate NHS roots ('can i get an nhs dentist', 'nhs dental exemption checker').
- Phrases with **no suggestion at all**, and so living in forums and guidance rather than search: 'how to reduce no shows dental', 'can dentists use chatgpt', 'hipaa compliant ai for dentists', 'fp17 claim rejected', 'nhs dental udas explained', 'gdc advertising rules', 'dental billing company cost', 'dental membership plan uk', 'dentist google ads cost'. One UK vendor's Keyword Planner pull (Q1 2026): 'dentist near me' 165,000+ a month, 'emergency dentist' 90,000+, 'NHS dentist accepting patients' 74,000+, 'Invisalign [city]' 27,000+ (Whito [129] vendor).

Table 2.3 Autocomplete by topic: US, UK, and phrases with no suggestions

Topic	US phrases that autocomplete	UK phrases that autocomplete	Phrases with no suggestions
Missed visits	dental office no show policy; dental no show fee; dental no show code; dentist no show fee reddit; dental office end of day checklist	dental fta policy; nhs dental fta policy; failed to attend dental appointment letter; can dentists legally charge for missed appointments	how to reduce no shows dental

Topic	US phrases that autocomplete	UK phrases that autocomplete	Phrases with no suggestions
Insurance and claims	dental insurance verification jobs remote; dental insurance verification companies; dental billing company near me; outsource dental billing services; how much do dental billing companies charge; dental claim denied due to frequency; dental claim denial codes; dental credentialing services; dental membership plan software	nhs dental claims dates; how to claim back nhs dental charges; compass nhs dental login; nhs dental compass cut off dates; pcse dental application; dental band 2 charge; nhs dentist exemption checker	dental billing company cost; fp17 rejected; fp17 claim rejected; nhs dental udas explained; dental membership plan uk
AI	ai receptionist for dental office; ai front desk for dental office; ai agent for dental office; claude ai for dental office; ai scribe for dentists; ai notes for dentists; ai automation for dentists; hipaa compliant ai note taker; hipaa compliant chatgpt cost; dental chatbot ai; dental ai answering service	ai dental receptionist uk; ai in dentistry uk; dental ai uk	can dentists use chatgpt; hipaa compliant ai for dentists; chatgpt for dentists (self only)
Software	dentrix vs open dental; dentrix vs dentrix ascend cost; open dental vs dentrix reddit; eaglesoft vs dentrix; open dental vs curve; dental practice management software price	dentally vs exact; dentally vs carestack; software of excellence support; dental practice management software uk	(none tested)
Recall	dental recall text message sample; dental recall script; dental recall reminder template; dental reactivation letter; dental appointment reminder software	dental recall guidelines uk; dental recall letter example; dental recall cards	dental recare (self only)
Marketing	dental marketing agency; dental marketing jobs remote; dental seo cost; dental google ads agency; new patient dental specials; dental office phone scripts	dental marketing agency uk; best dental marketing agency uk; dental seo agency uk; seo cost per month uk; dental practice google reviews; dental practice complaints procedure	dentist google ads cost; gdc advertising rules
Compliance	(not tested)	cqc dental inspection checklist; cqc dental inspection questions; how often do cqc inspect dental practices; cqc dental cpd requirements; gdc social media guidance; gdc advertising guidance	gdc advertising rules
Staffing	dental assistant shortage 2025; why don't dental assistants get paid more; dental office staffing agency; dental front desk salary; dental treatment coordinator jobs remote; dental virtual assistant hiring philippines	dental nurse shortage uk; dental nurse shortage occupation list; why are dental nurses paid so little; dental nurse recruitment agency; dental practice manager course level 4; treatment coordinator dental salary uk per hour	dental office overwhelmed

Marketplace selling phrases

- The admin side sells on 'breakdown of benefits (BOB)', 'error-free', 'exact copay', 'before appointments', 'via portals (Availity, Skygen, Delta Dental portals and many more) or direct calls to payers', 'Dental Xchange experience', 'OpenDental experience (preferred)', 'Curve Dental experience required' and 'persistent follow-up on denied and aged claims' (Upwork catalog [130] snippet; Upwork job [131] snippet).
- The marketing side sells 'Dental SEO and AI Visibility Services', 'Modern Dental Website with Online Booking Integration', 'Give database of 4K+ UK Dentists & Dental Clinics' and 'I will setup dentrix eaglesoft open dental curve denticon carestack tab32 identalsoft' covering 'account creation, provider profiles, patient data migration, insurance setup, and recall systems' (PeoplePerHour [132]; Fiverr prosalesfinders [133] snippet).
- Fiverr's dental pages are 'mostly dominated by website design, Google Ads, and SEO gigs, with few VA-style listings'; the admin and insurance work lives on Upwork's Project Catalog and OnlineJobs.ph; the only UK-specific offers found were dentist contact databases for cold outreach at $65, which says UK practices are an agency target rather than active freelance buyers (Fiverr /gigs/dental [134] snippet).

SECTION 3

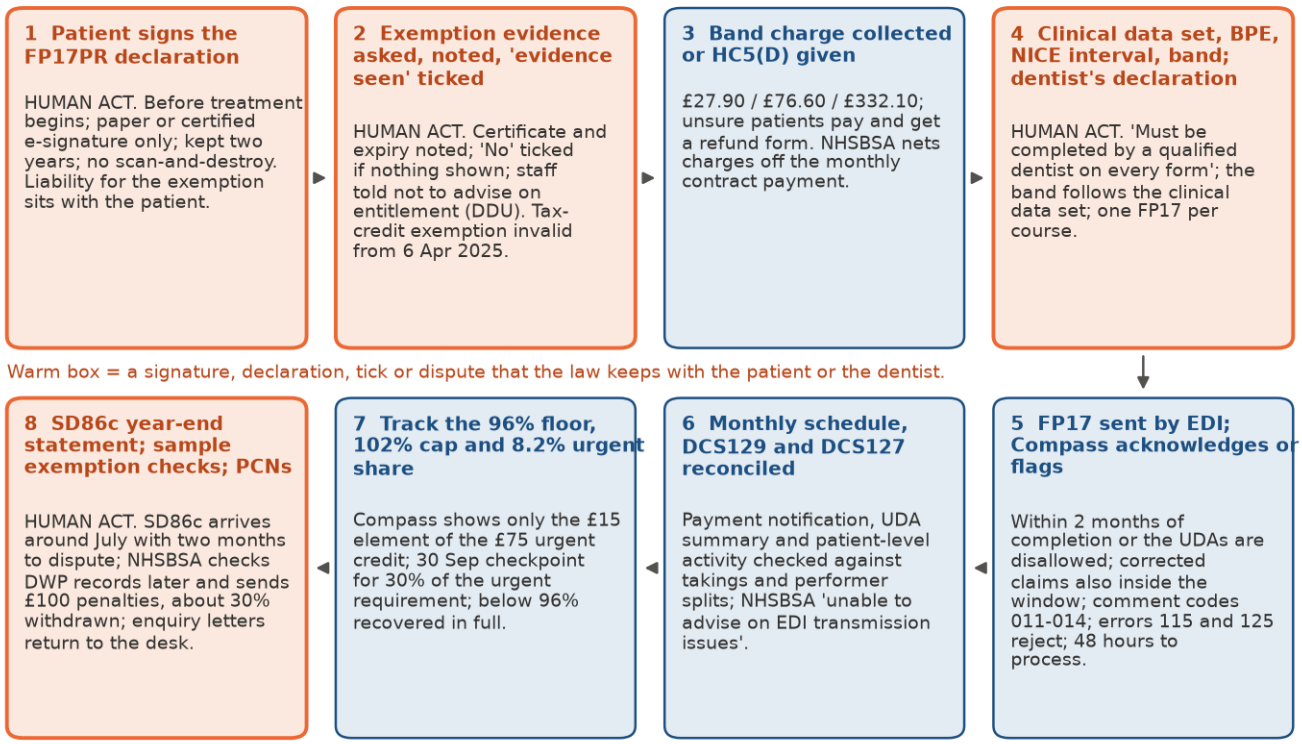
Every workflow runs on a form, a phone and someone else's deadline

The UK mixed practice runs an NHS flow and a private flow through one receptionist, with the NHS side governed by a signed declaration, a two-month claim window and a year-end statement; the US front desk runs a six-stage loop from eligibility check to denial rework, with every per-patient minute measured by a firm selling verification. No independent time-and-motion study of a dental desk exists in either country.

The UK NHS flow: declaration, evidence, band, claim, reconcile, dispute

- For an NHS patient the desk must have the **FP17PR declaration completed and signed before treatment begins**, ask for exemption evidence, note a valid certificate and its expiry, and if nothing is shown mark 'No' in the 'evidence seen' box and tell the patient checks are routine; an unsure patient pays and is given an HC5(D) refund form (NHSBSA exemption guide [135]). The signed form 'must be retained by the practice as part of the patient record for a period of two years', only a certified e-signature system replaces paper, and 'a practice cannot scan an original paper PR form and destroy the original' (NHSBSA FAQ KA-01916 [136]).
- The desk collects the band charge (£27.90, £76.60 or £332.10) and NHSBSA nets patient charges off the monthly contract payment, so any mismatch between what reception took and what the FP17 declared shows up on the monthly schedule and has to be chased (NHS.uk charges [137]; UDA Pulse [138] secondary).
- The clinician completes the clinical data set and signs the dentist's declaration, one FP17 per course; the claim must reach Compass **within two months of completion** or its UDAs are disallowed and marked 'Late Submitted Claim', corrected claims must also land inside the window, and electronic claims need 48 hours to process (NHSBSA FAQ KA-01820 [139]). The practice then works from Compass outputs (monthly payment notification, DCS129 UDA summary, DCS127 patient-level activity, a mid-year review) and the **SD86c year-end statement around July with two months to dispute it** (UDA Pulse [138] secondary; ICLR, Balachandra 2026 [142]).
- Three more calendars sit on top: a 30 September checkpoint for 30% of the urgent-care requirement, the DSPT self-assessment by 30 June, and the CQC's expectation of staff files, decontamination logbooks under HTM 01-05, fire and IR(ME)R records and audit cycles with 'about two weeks' warning' of an inspection (DSPT news 169 [143]; Dental Nurse Network [111]).
- The private flow runs beside it: a **written treatment plan and cost estimate before treatment** (GDC standard 1.6) and a fresh estimate if the plan changes (1.7), a deposit of 50-100% at booking at one Plymouth practice, an up-to-date medical history each time, and consent that after Montgomery must cover material risks and alternatives, since a signature 'does not indicate valid consent' (GDC Standards [146]; SpaDental Plymouth [147]; Dentistry.co.uk, Feb 2021 [149]). The 2026 contract adds funded appraisals (£213 each), a quality-improvement audit (£3,400), an urgent-care volume to track (8.2% of contract value) and complex-care claims, summed up by one consultant as 'increased admin, monitoring, benchmarking' for a practice that gains no administrative staff (Dentistry.co.uk, 2 Feb 2026 [66]; Price Bailey [65] secondary).

The UK exemption-to-clean-FP17 cycle: the software sends and flags, the practice reconciles, and four steps stay with a named person by law



Sources: NHSBSA exemption checking guide [135]; NHSBSA FAQ KA-01916 [136], KA-01820 [139], KA-01672 [52]; FP17 completion guidance V11.4 [151]; NHSBSA common errors [152]; UDA Pulse [138] (secondary); GDPUK, 13 Jul 2026 [12]; ICLR, Balachandra 2026 [142]; gov.uk algorithmic transparency record [345]; NAO 2019 [349]. US parallel: eligibility 270/271 pulls are electronic but the interpretation, the clinical narrative on an appeal and the signature stay human (section 6).

Table 3.1 UK steps, part 1: who, manual, repetitive

UK step	Who	Manual	Repetitive
Answer enquiry; say whether the NHS list is open; re-confirm the NHS.uk profile ("last confirmed" date)	Reception	Yes	Yes
FP17PR declaration signed; exemption evidence asked, noted with expiry; "evidence seen" box	Reception, patient	Yes	Every NHS course
Collect band charge or record exemption; HC5(D) for unsure patients	Reception	Yes	Every NHS course
Medical history updated and signed	Nurse, clinician, patient	Yes	Every visit
Clinical data set, BPE, NICE recall interval, band, dentist's declaration	Clinician	Yes	Every course
FP17 transmitted by EDI; receipt checked in Compass; rejections (comment codes 011-014, error 115, 125) corrected	Practice manager	Partly (PMS sends; checking and fixing manual)	Monthly schedule
Reconcile monthly payment notification, DCS129, DCS127 against takings and performer splits	Owner, manager, accountant	Yes	Monthly
Track UDA delivery against 96% floor and urgent-care share (Compass shows only the £15 element)	Owner	Yes	Continuous
Private estimate and written plan; deposit; finance; plan membership	Reception, TCO, clinician	Yes	Every private course
Lab moulds organised by patient; X-rays made available; supplier invoices checked	Reception, nurse	Yes	Daily
Recall letters or texts at NICE interval (NHS direction 24 months for healthy adults)	Reception	Partly	Continuous
CQC evidence: staff files, logbooks, audits, risk assessments, fire, IR(ME)R, registered manager	Practice manager	Yes	Daily to annual
DSPT self-assessment	Manager	Yes	Annual

UK step	Who	Manual	Repetitive
GDC website items: names, qualifications, GDC numbers, complaints route, "date last updated"	Manager, web agency	Yes	Every staff change
Funded appraisals (£213 each), QI audit (£3,400), complex-care pathway claims	Owner, manager	Yes	Annual, per claim

Table 3.2 UK steps, part 2: documents, deadlines, sources

UK step	Document-heavy	Deadline-driven	Source
Answer enquiry; say whether the NHS list is open; re-confirm the NHS.uk profile ("last confirmed" date)	No	Profile goes stale	NHS.uk profile [150]; Healthwatch East Sussex [83]
FP17PR declaration signed; exemption evidence asked, noted with expiry; "evidence seen" box	Yes (paper or certified e-signature, kept 2 years)	Before treatment starts	NHSBSA guide [135]; KA-01916 [136]
Collect band charge or record exemption; HC5(D) for unsure patients	Yes	Same visit	NHS.uk [137]
Medical history updated and signed	Yes	Each treatment	Dental Update [148]
Clinical data set, BPE, NICE recall interval, band, dentist's declaration	Yes	Before claim	NHSBSA FP17 guidance V11.4 [151]
FP17 transmitted by EDI; receipt checked in Compass; rejections (comment codes 011-014, error 115, 125) corrected	Yes	2 months from completion	KA-01820 [139]; NHSBSA common errors [152]
Reconcile monthly payment notification, DCS129, DCS127 against takings and performer splits	Yes	Mid-year review; 31 Mar year end; SD86c July + 2 months	UDA Pulse [138] secondary
Track UDA delivery against 96% floor and urgent-care share (Compass shows only the £15 element)	Yes	30 Sep urgent checkpoint; 31 Mar	GDPUK, 13 Jul 2026 [12]
Private estimate and written plan; deposit; finance; plan membership	Yes	Before treatment	GDC Standards [146]; SpaDental [147]
Lab moulds organised by patient; X-rays made available; supplier invoices checked	Yes	Lab return date	Indeed UK duties [153]
Recall letters or texts at NICE interval (NHS direction 24 months for healthy adults)	No	Interval	The Probe, 1 Oct 2026 [10] secondary
CQC evidence: staff files, logbooks, audits, risk assessments, fire, IR(ME)R, registered manager	Very	Inspection (~2 weeks' notice); registered-manager checks from 1 Oct 2026	Dentistry.co.uk, 20 Aug 2024 [110]; GDPUK, CQC [154]
DSPT self-assessment	Yes	30 June	DSPT news 169 [143]
GDC website items: names, qualifications, GDC numbers, complaints route, "date last updated"	Yes	Continuous	GDC advertising guidance [155]
Funded appraisals (£213 each), QI audit (£3,400), complex-care pathway claims	Yes	Registration 8 May 2026 and year end	Price Bailey [65] secondary

The US front desk: a six-stage loop

- Before the day the desk checks the answering service, email and portals, arrives at the huddle with open slots, overdue accounts and lab cases, and **verifies eligibility and benefits one to three days ahead at 5 to 30 minutes per patient**, with holds 'sometimes up to 30 minutes or more' at peak and at the start of the year (Burkhart Dental [156]; Curve Dental, 8 Sep 2025 [158] vendor). The CAQH Index puts the average at **about 12 minutes per manual verification, 7 per portal check and 4 fully electronic**, with dental spending on verification up 15% to $2.1 billion and a $580 million savings opportunity unrealised because 270/271 responses 'often don't return data robust enough to be reliable' (ADA News, 24 Mar 2025 [7]).
- A 2017 outsourcer put it at 'an average of two hours a day' and noted staff put eligibility last 'because it is the most time-consuming task', so easy payers get checked and hard ones slip, and the slip 'surfaces later as a denial or patient balance' (Dentaltown, 28 Jun 2017 [160] vendor).
- Check-in uses a print-ahead or online form bundling demographics, financial agreement, cancellation policy ('$25 charge for broken appointments without 24-hour notice') and medical history; check-out collects the balance identified that morning; claims go out next day through a clearinghouse at about **$0.50 per claim** with X-rays, perio charts and narratives attached; the EOB or ERA is posted (Open Dental 'attempts to automatically match claims' and detaches the ones it cannot); denials are worked within **90-180 days**, a simple one taking about 30 minutes and one example 'three hours chasing a $28 reimbursement' (XLDent, 27 Mar 2024 [162]; Open Dental ERA manual [163]; Curve Dental, 1 Jul 2026 [164] vendor).

- The day closes on paper: day sheet, transaction report, card settlement report and deposit slip, often not closed until the next morning because card transactions are still pending (DentistryIQ [166]). Around the loop sit credentialing packets with **CAQH re-attestation every 120 days** and Medicaid enrolment that 'can take months', the CDT update every 1 January (60 changes for 2026), the OSHA plan, DEA registration every three years at $888, state X-ray registrations, HIPAA notices and risk analysis, TCPA consent records and a **good-faith estimate within three business days** of a self-pay request (ADA News, 6 Jul 2026 [167]; ADA News, 29 Sep 2025 [168]; ADA News, Mar 2022 [171]).

Table 3.3 US steps, part 1: who, manual, repetitive

US step	Who	Manual	Repetitive
New-patient call or web "request" form, then staff confirmation by phone	Front desk	Yes	Every lead
Collect insurance details; verify eligibility, deductible, maximum, frequencies, waiting periods, limitations; record breakdown in PMS	Insurance coordinator	Mostly (phone, portal); 270/271 partly	1-3 days before every visit
Predetermination or pre-authorisation for larger cases	Coordinator	Yes	Per case
Intake forms: demographics, financial agreement, cancellation policy, medical history, HIPAA acknowledgement	Patient, front desk	Yes (print-ahead or Jotform)	Every new patient
Treatment plan presented; estimate; financing (CareCredit, in-house plan)	Treatment coordinator	Yes	Per plan
Check-out: collect balance identified that morning; book next visit (60% prefer to book in office)	Front desk	Yes	Every visit
Code (CDT, updated each January); submit claim via clearinghouse with attachments; change "planned" to "completed"	Biller	Partly	Daily
Post EOB/ERA; detach unmatched claims; bill patient balance; secondary claim with primary EOB	Biller	Partly (auto-match)	Daily
Work denials: find reason, call payer (~20 min), gather EOB, notes, X-ray, narrative, cover letter; resubmit or appeal	Biller, dentist	Yes	~1 in 6 claims
End of day: day sheet, card settlement, deposit slip; close next morning	Office manager	Yes	Daily
Recall and reactivation calls; unscheduled-treatment follow-up (next morning after a plan, or within 3 days)	Front desk or VA	Yes	Continuous; "easiest work to move to tomorrow"
Credentialing: licence, DEA, NPI, CAQH, CV, diplomas, malpractice, references; re-attest every 120 days; reset on tax-ID change	Office manager	Yes	Per dentist, per payer, 30-120 days each
Compliance documents: HIPAA NPP, policies, training, risk analysis, BAAs; OSHA plan, HBV, sharps log; DEA; X-ray registration; TCPA consents; record retention 5-10 years by state	Office manager	Yes	Annual to three-yearly

Table 3.4 US steps, part 2: documents, deadlines, sources

US step	Document-heavy	Deadline-driven	Source
New-patient call or web "request" form, then staff confirmation by phone	No	Speed-to-lead	Mose Family Dentistry [172]; Patient Prism [4] vendor
Collect insurance details; verify eligibility, deductible, maximum, frequencies, waiting periods, limitations; record breakdown in PMS	Yes	Before the visit	ADA News, 24 Mar 2025 [7]; Open Dental ebenefits [173]
Predetermination or pre-authorisation for larger cases	Yes (X-rays, narrative)	Payer turnaround (not found)	Becker's, Mar 2024 [174] secondary
Intake forms: demographics, financial agreement, cancellation policy, medical history, HIPAA acknowledgement	Yes	At first visit	Jotform example [161]; TLC Dentistry [175] snippet
Treatment plan presented; estimate; financing (CareCredit, in-house plan)	Yes	GFE within 3 business days for self-pay	ADA News, Mar 2022 [171]
Check-out: collect balance identified that morning; book next visit (60% prefer to book in office)	No	Same visit	GoAudits [157]; DentistryIQ survey [176]
Code (CDT, updated each January); submit claim via clearinghouse with attachments; change "planned" to "completed"	Yes	Timely filing 90 days to 12 months	Curve Dental, 1 Jul 2026 [164] vendor; Gerald guide [177]

US step	Document-heavy	Deadline-driven	Source
Post EOB/ERA; detach unmatched claims; bill patient balance; secondary claim with primary EOB	Yes	30-day aging targets	Open Dental ERA [163]; Medi-Cal billing tips [178]
Work denials: find reason, call payer (~20 min), gather EOB, notes, X-ray, narrative, cover letter; resubmit or appeal	Very	90-180 days from denial	Curve Dental [164] vendor; Henry Schein One, 10 Sep 2024 [179] vendor
End of day: day sheet, card settlement, deposit slip; close next morning	Yes	Next morning	Carestream help [165]; DentistryIQ [166]
Recall and reactivation calls; unscheduled-treatment follow-up (next morning after a plan, or within 3 days)	No	Interval	Hello Patient [180] vendor; DentistryIQ, Levin [181]
Credentialing: licence, DEA, NPI, CAQH, CV, diplomas, malpractice, references; re-attest every 120 days; reset on tax-ID change	Very	120-day CAQH; payer timelines	ADA News, 6 Jul 2026 [167]
Compliance documents: HIPAA NPP, policies, training, risk analysis, BAAs; OSHA plan, HBV, sharps log; DEA; X-ray registration; TCPA consents; record retention 5-10 years by state	Very	1 March (KS); 3 years (DEA); annual (OSHA)	RDH Magazine [182]; Ohio Dental Association [169]; Hawaii board compilation [183]

The marketing and new-patient flow is phone-first and brochure-shaped

- Of five UK independent sites read directly (Hull, Plymouth, Carlisle, Wolverhampton), every one leads with a phone number, three add third-party online booking, none has a chat widget, **none offers online registration or a medical-history form, none takes online payment**, all present plans and finance as links that withhold provider and price, **no home page shows a GDC number** and no site shows the 'date last updated' the GDC requires (Lund Dental [184]). NHS.uk status lines are self-reported snapshots that disagree for the same practice ('when availability allows' in one, 'children only' in another; hours 'last updated 2019'), so the first contact is a call to ask whether the list is open (NHS.uk Doncaster results [188]).
- Of 13 US practices profiled across eleven cities, every independent site gives phone first, three use a web 'request' form that staff confirm, two state phone-only booking, insurance pages say 'most major insurance plans' or 'call us today to verify your benefits' while the in-network lists live only in third-party directories, financing is CareCredit or an unnamed in-house plan, and the only live scheduling, portal and texting set-up was on the one DSO-affiliated site (Mose Family Dentistry [172]).
- The lead is handled by hand: fewer than half of UK practices check for new leads daily and reception handles them between calls; in the US the front desk is 'splitting attention between phones and in-office patients' and only 4.8 of 100 non-bookers get a call back (Dentistry.co.uk, 22 May 2019 [84] vendor; Patient Prism blog, 1 Sep 2026 [88] vendor).

Table 3.5 Marketing and new-patient steps, UK against US

Marketing and new-patient step	UK	US	Manual / repetitive / document-heavy / deadline	Source
Enquiry arrives (website, Google Business Profile, phone, Facebook form, directory)	Website then email inbox; phone; "request a callback"; WhatsApp on one site	Phone first; web request form; finder listings "free to claim, phone verification required"	Manual and repetitive; speed-to-lead deadline	Dentistry.co.uk, DenGro [84] vendor; New Patients Inc, Tulsa [194]
Log the lead and call back	Paper diaries, Post-its, spreadsheets or nothing (two-thirds); 2-3 week gaps	4.8 of 100 non-bookers called back; VA at $3 per lead booked	Manual; no system of record	Dentistry.co.uk [84] vendor; Patient Prism [4] vendor; OnlineJobs.ph 1505586 [126]
Answer "do you take NHS / my plan?"	NHS list status from NHS.uk profile; 47% offer private to NHS enquirers	"Most major plans"; verify by phone; DSO sites list carriers and Medicaid	Manual; profile re-confirmation task	Healthwatch East Sussex [83]; Rodeo Dental Lubbock [195]
Quote a price	Consultation median £65; plan price withheld on site; "shoppers are not necessarily looking for the cheapest option"	"$175 for Exam, X-rays, Cleaning, and Fluoride" with gingivitis-only fine print; $119/month braces "No Down - No Interest"	Manual; GDC and ASA pricing and claim rules	TreatCompare [72]; Smiles of Spokane [191]; Dentaltown, fee shoppers [196]
Book and confirm; deposit	Deposit 50-100% at booking (SpaDental); third-party booking link	Request form confirmed by staff; live booking only at the DSO site	Manual; reconciliation of deposits	SpaDental [147]; O'Donnell [192]
Registration and medical history	No online form on any of five sites	Print-ahead or Jotform	Document-heavy	Plumfield [185]; Jotform [161]

Marketing and new-patient step	UK	US	Manual / repetitive / document-heavy / deadline	Source
Ask for a review; reply to reviews	Reply must not confirm the person is a patient (GDC 4.2.3; MDDUS)	FTC rule on incentives and insider reviews; HIPAA on replies; Google Business Profile suspensions after acquisitions	Manual; legal exposure	MDDUS, Apr 2022 [197]; CDA, 17 Apr 2025 [198]; RevenueWell on GBP [199] vendor
Keep the website compliant	GDC: names, qualifications, GDC numbers, complaints route, "date last updated"; 1.8% fully compliant in one audit	State specialty-advertising rules; FTC; TCPA consent wording	Document-heavy; every staff change	GDC [155]; PubMed, BDJ 2021 [19]
Measure what the spend returned	"no idea" of ROI; agency says "give it more time"	Cost per lead reported; cost per seated patient not reported by any source	Manual; no shared data between agency CRM and PMS	Dentistry.co.uk, 29 Dec 2021 [17]; Delmain, Jun 2026 [200] vendor

SECTION 4

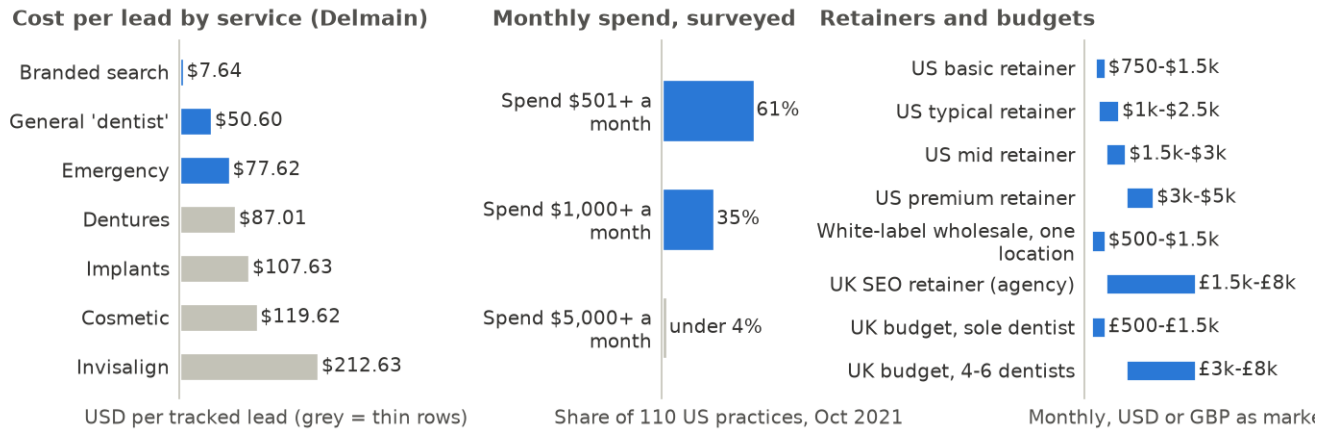
The marketing market: $1,000-$2,500 a month bought on trust, with the front desk and the regulators in between

Spend is small, vendor-measured and badly defined; agencies charge $1,000-$2,500 a month in the US and £1,500-£8,000 in the UK; owners say they get 'no idea' of return; agencies answer that the front desk drops the lead; and the pages that bring in private patients are the pages the GDC, the ASA, the FTC and Google check.

How much is spent, and what a lead costs

- The only survey-based US figure is ProSites' 2022 report (110 practices, October 2021): **61% spent $501 or more a month, 35% more than $1,000, under 4% more than $5,000**; word of mouth (80%), online reviews (57%), website and SEO (46-51%) were rated important, TV, radio and mailers unimportant (DrBicuspid, 21 Mar 2022 [201] vendor survey). Agencies recommend 3-6% of gross revenue (5-8% in growth mode), which for a $1M practice is $2,500-$5,000 a month, well above the median, so the agencies' addressable market is the top third by spend (HIP Agency, 16 Jul 2026 [202] vendor).
- Cost per new general-dentistry patient is quoted at **$150-$300** (implant and cosmetic $400-$800; SEO-sourced $50-$150 against Google Ads $150-$350) and lifetime value at $5,500-$10,000, all by agencies (Ainora [204] vendor; RankAI, 7 Dec 2025 [15] vendor). The one treatment-level dataset is Delmain's own 187 search campaigns (June 2026): cost per tracked lead, a call or a form and not a booked patient, of **$7.64 branded, $50.60 general, $77.62 emergency, $87.01 dentures, $107.63 implants, $119.62 cosmetic and $212.63 Invisalign**, the last three rows 'thin and directional' (Delmain, Jun 2026 [200] vendor).
- UK figures come from one self-described research publisher ('directional benchmarks'): spend 5-10% of revenue for private-focused practices, under 2% NHS-heavy; monthly budgets **£500-£1,500 for a sole dentist, £3,000-£8,000 for four to six**; cost to acquire £30-£150 from Google search; annual new-patient value £300-£500 NHS-heavy, £1,000-£2,500 private (Whito, May-Oct 2026 [129] vendor). UK agencies quote cost per lead by treatment that does not reconcile between them (implants £100-£350, Invisalign £80-£200, emergency £25-£40) and £295 per new patient (Dominate Dental [205] vendor).

Marketing: cost per tracked lead runs from $7.64 (branded search) to $212.63 (Invisalign) on one agency's 187 campaigns; 61% of surveyed US practices spend $501+ a month and retainers cluster at $1,000-$2,500 (UK £1,500-£8,000+)



Sources: Delmain, Jun 2026 [200] (vendor; calls or forms, not booked patients; dentures, cosmetic and Invisalign rows 'thin and directional'); ProSites survey via DrBicuspid, 21 Mar 2022 [201] (110 practices); RankAI [15], HIP Agency [202], ClicksGeek [216], Dominate Dental [16] (all vendor; the Dominate page showed a 'Hacked by' header when read); Whito [129] (vendor).

Table 4.1 Marketing money measures, US against UK

Marketing money measure	US	UK	Source
Monthly spend, surveyed	61% $501+; 35% $1,000+; <4% $5,000+ (110 practices, Oct 2021); 33% spent under 1% of revenue (2020)	None independent; £500-£15,000 by practice size (vendor estimate)	DrBicuspid [201] vendor survey; Whito [129] vendor
Recommended share of revenue	3-6% established; 5-8% growth; 8-12%+ start-up; 2-3% maintain, 4-5% grow	5-10% private; 3-5% mixed; <2% NHS-heavy; 8-12% for £500k+ practices (agency)	HIP Agency [202] vendor; RankAI [15] vendor; Dominate Dental [16] vendor
Cost per new patient	$150-$300 general; $400-$800 implant/cosmetic; $50-$150 SEO vs $150-$350 Google Ads; $74 at group level ($14,800 median monthly spend)	£30-£150 Google; £295 average; SEO ~£170; Ads ~£270	Ainora [204] vendor; Emulent [209] vendor; Dominate Dental [206] vendor
Cost per lead by treatment	Branded $7.64; general $50.60; emergency $77.62; dentures $87.01; implants $107.63; cosmetic $119.62; Invisalign $212.63 (calls or forms, not bookings)	Emergency £25-£40; cosmetic consult £60-£120; implants £100-£350; Invisalign £80-£200; £62.01 average at 9.08% conversion	Delmain [200] vendor; Dominate Dental [205] vendor
Click costs	High-intent dental CPC $80-$150 (unsourced); Meta CPL $8-$15 general, $25-$60 implants	General CPC £1.50-£4.50; "dental implants London" £6-£15+; "Invisalign London" £11-£18	Viva [86] vendor; Whitehat SEO [210] vendor
Lifetime value	$5,500-$7,500; $7,000-$10,000; gross production ~$4,200 per patient; implant patient $8,000-$40,000	£300-£500 NHS-heavy; £600-£1,200 mixed; £1,000-£2,500 private; £2,000-£5,000+ cosmetic; "£5,000+ LTV can afford £100-£200 per new patient"	RankAI [15] vendor; Webtonic [211] vendor; Whito [129] vendor
Membership plans as marketing	38% of practices offer one (2025; 12% in 2019); average practice enrols 8-12% of uninsured patients	Denplan 1.4-1.5m patients; plans £15-£50/month; corporates "Switch to Denplan"	Clerri [212] vendor; Denplan [73]; Rodericks [75]

What agencies charge, and what owners say about them

- US dental SEO and marketing retainers cluster at **$1,000-$2,500 a month** (basic $750-$1,500, premium $3,000-$5,000+; small DSO $5,000-$15,000; ad spend billed separately), Clutch-listed dental agencies run $100-$149 an hour with $1,000+ minimums, and pricing guides say a 'full service' quote under $750-$800 'suggests subcontracting overseas or skipping the reporting layer' (RankAI [15] vendor; Clutch [214]). White-label local SEO for 'a dentist' costs an agency **$500-$1,500 a month**, which implies a 2-5x markup on subcontracted work (ClicksGeek [216] vendor).
- UK dental SEO runs **£1,500-£8,000+ a month** (consultations £75-£200 an hour; technical audit about £3,000; 'AI-focused agencies charge 20-30% premiums') on an agency page that showed a 'Hacked by Panataran' header when read (Dominate Dental [16] vendor). Named-agency data are thin and conflicting: Firegang lists a $2,000+ minimum with 5.0 from 8 reviews and claims '$10-15 back for every $1'; Roadside is listed at both under $500 and $5,000-$14,000 a month by different aggregators; Dental Focus (UK) claims 'over 5,000 dentists and practices' and shows no prices (UpCity, Firegang [219]).
- Owners' side: a UK principal paying £18,000 a year had 'no idea' of return and heard 'incomprehensible excuses'; a Trustpilot reviewer of a UK lead-gen agency said prospects 'turned up not even knowing the cost or what they wanted'; one US owner says that after firing a $4,200-a-month agency 'the week after, our lead flow went up'; an older case shows an agency threatening a cancelling dentist over a non-disparagement clause, now barred by the Consumer Review Fairness Act (Dentistry.co.uk, 29 Dec 2021 [17]; Trustpilot, Dentology [114] user; socialperks [224] user, unverified; ABA Journal [225]).
- The agencies' answer is that the practice loses the lead: **37% of 55,000+ tracked client calls went unanswered**, 'your marketing isn't broken, your front desk is', and answering phones 'requires a special set of skills that has to be taught'; Dentaltown's marketing channel itself is agency-written sponsored posts ('$2,500-$7,500 a month and 6-12 months' to reach page 1) (Dentaltown, 5 Aug 2026 [18] vendor; Dentaltown [228] vendor).

Table 4.2 Agencies and vendors: price, reviews and criticism

Agency or vendor	Price	Reviews and criticism	Source
US retainers (generic)	$1,000-$2,500/month typical; $750-$5,000+; $100-$149/hr on Clutch; $1,000+ minimums; management fee ~$400-$700/month at the $1,500-$5,000 tiers	Sub-$800 "full service" flagged as offshore subcontracting	RankAI [15] vendor; HIP Agency [202] vendor; Clutch [214]
White-label wholesale	$500-$1,500/month single-location SEO; $300-$750 entry; UK £500-£1,000/month	Implies 2-5x markup	ClicksGeek [216] vendor; seo.co [229] vendor

Agency or vendor	Price	Reviews and criticism	Source
UK retainers	£1,500-£8,000+/month SEO; £995-£5,000+; pay-per-lead ~£150 per consultation (Head45)	Page hacked at time of reading; no review text reached for any UK agency	Dominate Dental [16] vendor; Webtonic London list [230] vendor
Firegang	$2,000+/month minimum; "from $1,800"	5.0 from 8 verified reviews; "$10-15 back for every $1" (own claim)	UpCity [219]
Wonderist	No rate card; "from $1,500", "$2,500/mo PPC", "$6K-15K" (conflicting)	4.9 from 215 ratings, no text	DesignRush [218] snippet
ProSites	Pricing page JavaScript-only	Capterra 4.2 from 10	Capterra [221]
My Social Practice	$50-$450/month, month-to-month; $59 reseller tier	Claims 375,000 reviews generated; no independent reviews	My Social Practice [222] vendor
Golden Proportions	$2,500+ project; $2,000+/month	5.0 from 6 reviews dated 2019	UpCity [220]
Roadside Dental Marketing	Under $500/month (UpCity) vs $5k-$14k/month (aggregator); $125/hr	5 public reviews; conflicting listings	UpCity [231]; Texta.ai [232] snippet
Dental Focus (UK)	No prices or terms shown	"over 5,000 dentists and practices"; five testimonials	Dental Focus [223] vendor
Dentology (UK lead-gen)	Not shown	2-star Trustpilot: leads "not even knowing the cost or what they wanted"	Trustpilot [114] user
Freelance floor	Fiverr dental websites $80-$425; SEO articles from $10; Google Ads management $450 and websites $235-$1,000 on PeoplePerHour; Upwork SEO/PPC for a practice $10-$35/hr	One backlink gig promises "Google #1 ranking within 4 weeks" at 3.8 stars	Fiverr [233] snippet; PeoplePerHour [132]; Upwork [234] snippet

Lead to seated patient: the number nobody reports

- Using Patient Prism's 2025 funnel, about a fifth of inbound calls become bookings and under 1% of lost opportunities are recovered by callbacks; **1,445,814 patients walked away without an appointment and 890,621 never got a callback** across 8,280 locations (Patient Prism, 2 Jul 2026 [4] vendor). One AI-lead-response vendor claims, without a source, that 60-70% of paid dental leads are never successfully contacted and that 53% of prospects give up after one missed callback (Viva [86] vendor, unsourced).
- **No source in either country reports cost per seated patient or cost per registered patient**, the metric one agency commentator calls the only one that matters, and no public data exist on agency contract terms, churn or client-reported ROI (LinkedIn commentary [235]). The UK leak is older and less measured: a 2011 test found 44% of 3,557 calls failed to reach the team (Dentistry.co.uk, 1 Jul 2011 [236] snippet).

The rules that sit on the marketing pages

- **US.** OCR's tracking-pixel bulletin was partly vacated on 20 June 2024 but guidance on authenticated pages stands and private wiretap suits continue: Aspen Dental settled a Meta Pixel and booking-data class action for **$18.7 million** (HIPAA Journal, 21 Jun 2024 [237]; DentistryIQ [239]). The FTC's consumer-review rule took effect 21 October 2024: fake or AI reviews, sentiment-conditioned incentives and suppression draw penalties of about **$51,744-$53,088 per violation**; first warning letters 22 December 2025; no dental case yet (FTC [240]; CDA, 17 Apr 2025 [198]; Crowell [241]). Under the TCPA, reminders need prior express consent and marketing texts prior express *written* consent, at **$500-$1,500 per message with no cap**; the FCC's one-to-one consent rule was vacated on 24 January 2025 (Appointment Reminder [243] vendor; Morrison Foerster [244]). Google bars remarketing on invasive and cosmetic procedure pages (HawkSEM [245] vendor). Replying to a review with any patient detail is a HIPAA breach: three dental OCR cases ($10,000, $23,000, $50,000) were for review responses (HIPAA Journal [249]).
- **UK.** The GDC's 2013 advertising guidance is still current: a practice website must show the practice name and address, the GDC's contact details, the complaints procedure and the **date the website was last updated**; every named professional must show qualification, country and GDC number; only dentists on a GDC specialist list may use 'specialist'; social posts must say the treatment 'may not be appropriate for every patient'; and the registrant 'is responsible for checking that it is correct', with no mention of agencies (GDC advertising guidance [155]). A peer-reviewed audit of practice websites in North East England found **only 1.8% fully compliant** (PubMed, BDJ 2021 [19]). CAP requires before-and-after images 'genuine and representative' with 'signed and dated proof', says 'Dr' must not imply a medical qualification and that Botox cannot be advertised; no 2024-2025 ASA ruling against a UK dental practice was located (ASA/CAP, Dental: General [251]; ASA ruling A22-1157178 [254]). The ICO finalised cookie and pixel guidance on 29 April 2026, and the GDC's social-media guidance forbids any patient-identifying post or reply (ICO [255]; GDC social media guidance [256]).

Table 4.3 Rules on marketing pages: what they require and what happened

Rule	Country	What it requires on marketing pages	Penalty or case	Source
HIPAA tracking technologies	US	Pixels on authenticated pages (portals, booking behind login) still covered; public pages freed by the Jun 2024 ruling; private wiretap suits continue	Aspen Dental $18.7m settlement (DSO); no small-practice case found	HIPAA Journal [237]; DentistryIQ [239]
FTC consumer review rule (effective 21 Oct 2024)	US	No fake or AI reviews, no sentiment-conditioned incentives, no undisclosed staff or relative reviews, no suppression	Up to ~$51,744-$53,088 per violation; warning letters 22 Dec 2025; no dental case	FTC [240]; CDA [198]
TCPA	US	Reminders need prior express consent; marketing texts need signed written consent; stop on STOP; one-to-one rule vacated 24 Jan 2025	$500-$1,500 per message, no cap	Appointment Reminder [243] vendor; MoFo [244]
HIPAA on review replies	US	Do not acknowledge anyone as a patient	Elite Dental $10,000; Igbinadolor $50,000; New Vision $23,000; Iowa board fine	HIPAA Journal [249]
Google Ads personalized advertising	Both	No remarketing on invasive or cosmetic procedure pages; general dentistry allowed	Ad disapproval	HawkSEM [245] vendor
State specialty-advertising rules	US	Varies; Texas replaced rule Mar 2021; South Dakota struck down	Board discipline	DrBicuspid [247]
GDC advertising guidance (2013, current)	UK	Name, address, GDC link, complaints route, "date last updated", GDC numbers and qualifications for every named clinician, NHS/mixed/private statement, specialist-title rules, social-post caveat	Fitness-to-practise investigation; "can be a criminal offence"; 1.8% of audited sites compliant	GDC [155]; PubMed [19]
ASA/CAP Code	UK	Before/after images genuine with signed proof; "Dr" caveat; whitening and brace claims need evidence; Botox not advertisable	Rulings against Perfect Smile Surgery, 15 Dental, OrthoAccel, Dental Centre Turkey (2022), HiSmile	ASA/CAP [251]; ASA ruling [254]
GDC social media; UK GDPR	UK	No patient-identifying posts or replies; cookie and pixel consent per ICO guidance (Apr 2026)	GDC standards 4.2.3 and 9.1.3; ICO reprimands	GDC social media [256]; ICO [255]

Crowded and thin. Crowded: AI phone receptionists and call intelligence (Peerlogic, Patient Prism, VoiceStack, Viva, Convirza, Weave and Adit all publishing near-identical missed-call statistics; 'ai dental receptionist' already carrying cost, reviews, Reddit and five country modifiers in autocomplete), dental SEO and websites (dozens of agencies; Fiverr gigs from $10), review tools (including a $500 Upwork post asking for 'genuine 5-star' reviews that the FTC rule prohibits), social content packages and agency-written 'the front desk is the problem' content. Thin: any independent measurement of leakage; UK enquiry-handling data newer than 2017-2021; agency contract terms, churn or ROI; cost per seated patient by treatment; enforcement against small practices (the only US pixel case and the only UK ASA dental ruling are against a DSO and a facilitator); and compliance checking of practice websites and social posts against GDC, ASA and FTC rules, which no agency in the sample advertises and which the 1.8% figure says is not being done (Google autocomplete [258]; Upwork review job [259] snippet).

SECTION 5

Security and AI limits: fines land on missing records, ChatGPT has no BAA, and the GDC has no AI rule yet

OCR's record against dental practices is a record of paper and policy failures, not of hacking; the biggest breaches arrive through vendors and stop the money; consumer ChatGPT offers no business associate agreement; the UK has a device line from the MHRA, a DPIA expectation from NHS England and no GDC rule; and a UK IT provider's 192 on-site audits show what 'everyday security' looks like at three surgeries.

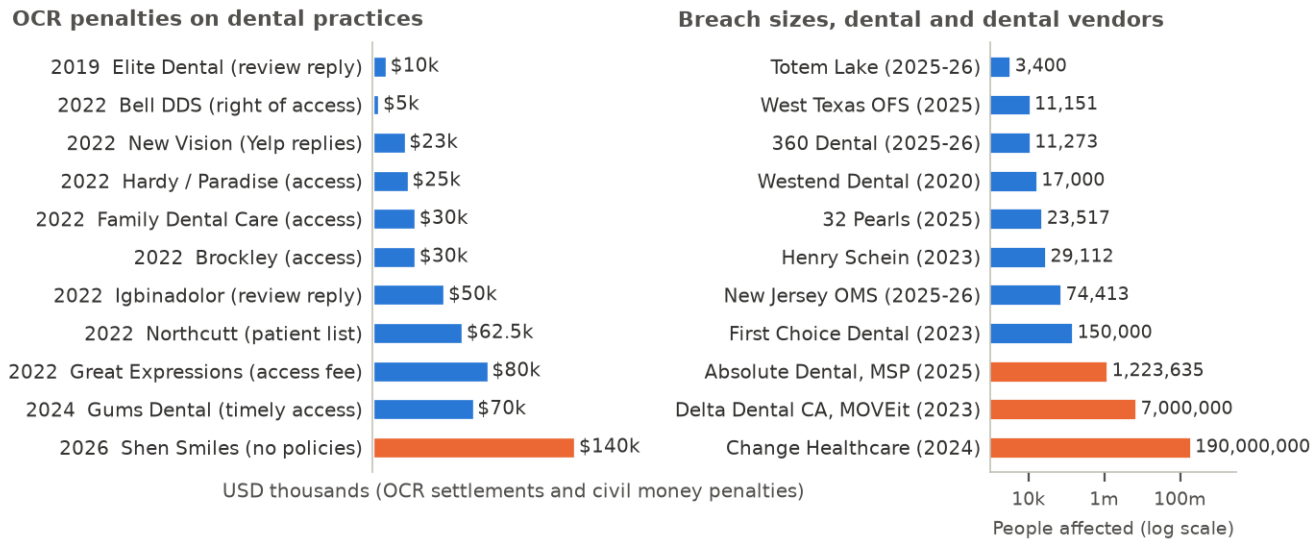
HIPAA, business associates and the consumer AI app

- A US practice is a covered entity, so any AI vendor touching PHI is a business associate and must sign a BAA; the CDA's endorsed compliance partner tells dentists to 'never use the free, public version of ChatGPT with patient data' and to 'avoid vendors that refuse to sign a BAA' (CDA/Abyde, 27 Aug 2025 [260] vendor). OpenAI's help centre, quoted second-hand, says 'we don't offer a BAA for ChatGPT Business'; civil penalties run **$145 to $2,190,294 per violation** by culpability tier (IntuitionLabs [261] secondary).
- Three separate disclosure paths: an AI receptionist that answers, transcribes or books is a business associate, not a conduit; a website chat widget's free-text box 'should be assumed to collect PHI'; and staff 'pasting claim narratives or chart details into unapproved AI tools' (My Social Practice, Sep 2025 [262] vendor). The endodontists' association adds that general AI tools 'lack knowledge of carrier claims histories, state rules, and policy exclusions' and give confident but unreliable coverage answers (AAE [265]).

OCR's dental record: records, review replies and missing policies

- HIPAA Journal's penalty list carries **eleven dental entries since 2019, from $5,000 to $140,000**: refusing or delaying record requests (six cases), disclosing patient details in review replies (three), and having no privacy officer, policies or training. The latest, Shen Smiles (Pennsylvania), announced 8 October 2026 at **$140,000**, began with a 2020 complaint about unfulfilled record requests and ended with records found 'in plastic boxes, the floor, and a private bathroom' (HIPAA Journal [249]; Paubox, 12 Oct 2026 [266] vendor).
- The largest dental sum came from a state attorney general: Westend Dental (Indianapolis, about 17,000 patients) paid **$350,000** after a 2020 ransomware attack it reported two years late and described as 'not a ransomware attack'; investigators found plain-text passwords, no risk analysis, no training before November 2023, no BAA with the non-employee running the practices and servers 'in employee break rooms and bathrooms' (HIPAA Journal, 2 Jan 2025 [267]). First Choice Dental (Wisconsin) paid **$1.2 million** to settle a class action over an October 2023 hack affecting 150,000-plus patients (LexBlog, 18 Nov 2025 [268]).

OCR's dental penalties run from $5,000 to $140,000 (2019-2026), mostly for unanswered record requests and review replies, while breaches reach the practice through vendors: Change Healthcare 190 million, Delta Dental 7 million



Sources: HIPAA Journal penalty list [249]; HealthITSecurity [269] (snippet); National Law Review [270] (snippet); Paubox, 12 Oct 2026 [266] (vendor); HIPAA Journal on Westend [267] (Indiana AG, not OCR); LexBlog [268] (class action); breach scale from KrebsOnSecurity [298], Cybersecurity Dive [299], Orthodontic Products [300], Healthcare Finance News [305], HIPAA Journal [307] [308], TEISS [310]. Not shown: Westend $350,000 (Indiana AG, 2024) and First Choice $1.225m (class action, 2025); the 2019 DDS Safe ransomware hit about 400 practices.

Table 5.1 OCR and state penalties on dental practices, 2019-2026

Dental practice	Year	Amount	Cause	Source
Elite Dental Associates	2019	$10,000	Social media disclosure; notice of privacy practices	HIPAA Journal [249]
New Vision Dental	2022	$23,000	PHI in Yelp review replies	HIPAA Journal [249]
Great Expressions Dental Center of Georgia	2022	$80,000	Right of access; $170 copying fee	HealthITSecurity [269] snippet
Family Dental Care, PC (Chicago)	2022	$30,000	Right of access	HealthITSecurity [269] snippet
B. Steven L. Hardy DDS / Paradise Family Dental	2022	$25,000	Right of access	HIPAA Journal [249]
Lawrence Bell Jr DDS	2022	$5,000	Right of access	HIPAA Journal [249]
Dr U. Phillip Igbinadolor DMD & Associates	2022	$50,000 civil money penalty	Google review reply disclosed PHI	HIPAA Journal [249]
Northcutt Dental-Fairhope	2022	$62,500	Patient list shared with a political campaign; no privacy officer	National Law Review [270] snippet
Dr Brockley	2022	$30,000	Right of access	HIPAA Journal [249]
Gums Dental Care	2024	$70,000 civil money penalty	Failure to provide timely access	HIPAA Journal [249]
Shen Smiles	2026 (announced 8 Oct)	$140,000	No Privacy Rule policies, training or safeguards; records in boxes and a bathroom	Paubox [266] vendor
Westend Dental (Indiana AG, not OCR)	Dec 2024	$350,000	Concealed 2020 ransomware; plain-text passwords; no risk analysis, training or BAA	HIPAA Journal [267]
First Choice Dental (class action)	Nov 2025	$1,225,000 cap	Oct 2023 ransomware; 150,000+ patients	LexBlog [268]

The proposed Security Rule and US AI governance

- Published in the Federal Register on 6 January 2025, the Security Rule rewrite would make nearly all specifications mandatory: **encryption at rest and in transit, MFA for all access to ePHI systems 'with no size exceptions'**, a written asset inventory, network segmentation, vulnerability scans every six months, an annual penetration test, tested recovery plans with 72-hour restoration, and direct obligations on business associates (WSDA, 29 Apr 2026 [271]; Georgia Dental Association, 26 May 2026 [272]). The ADA joined a coalition letter asking OCR to rescind it; status reports conflict (final action May 2026, July 2027, or 'final' per one vendor) and hhs.gov blocked reading. Not in dispute: OCR's line that 'the size of a practice is irrelevant', that in 2022 more than half of OCR penalties targeted small practices, and a Senate committee's 22-1 vote in February 2026 to advance a bill codifying MFA and encryption (Physicians Practice, 10 Mar 2026 [274]).
- US AI governance is standards and insurer caution: the ADA's white paper (1106:2022), a technical report on evaluating image-analysis systems (1109:2025) and the first ANSI-approved dental AI standard (1110-1:2025); the FDA list shows a steady run of dental radiograph 510(k) clearances (Overjet, Pearl, VideaHealth, Diagnocat), so imaging is the one dental AI category with a clear regulatory status while scribes, chatbots and phone AI sit under HIPAA contract rules only (ADA News, 3 Feb 2025 [277]; FDA list [278]). The main dental malpractice insurer says liability for AI decisions is 'a legal gray area' and tells dentists to 'obtain informed consents that explicitly cover AI use' (CDA/TDIC, 27 Jan 2025 [279]); California's AB 3030 requires a disclaimer on generative-AI clinical messages unless a licensed provider reviews them, exempting scheduling and billing (ArentFox Schiff [280]).
- Adoption is real and uneven: **43.3% of US dentists use AI for at least one task** (imaging 22.8%, insurance verification 13.6%, reception 10.1%), 34.8% plan charting and note-taking, 32.6% plan verification, 82.6% never plan to use it for treatment recommendations; a three-country survey found 12% who tried and stopped (cost 28%, poor PMS integration 22%, accuracy 18%) (ADA HPI, Jul 2026 [45]; GDPUK, 1 Jun 2026 [281]). The reputational limit is public: a columnist's AI-read X-ray led to a four-session periodontal plan 'costing thousands of dollars' that three other dentists said home care would handle; staff said owners asked 'Why didn't you drill it?' (Futurism, 16 May 2026 [117]).

The UK: no GDC rule yet, a device line, and record clocks

- As of 16 October 2026 the GDC has published a literature review ('No UK-based studies were included') and, on 17 September 2026, a joint statement of intent with the medical, nursing and social-work regulators to develop 'high-level principles' with no timeline; Dental Protection asked the GDC to clarify 'professional accountability' (Dentistry.co.uk, 17 Sep 2026 [285]; GDPUK, 9 Sep 2026 [286]).
- The MHRA, with NHS England, drew the device line on 2 August 2026: tools used only for 'transcription, consultation summaries, draft referral letters or suggested clinical codes' for clinician review 'are not medical devices', while products supporting 'diagnosis, treatment or prevention' or taking 'automated action without clinician review' are, and 'clinicians remain responsible for checking AI-generated transcripts' (Dentistry.co.uk, 2 Aug 2026 [287]). NHS England's ambient-scribe guidance (v3, 29 July 2026) with a template DPIA expects DCB0160 documentation, explicit consent, and 'CQC inspection teams will consider if: Technologies are safely deployed' (GPCE summary [289]). The Chief Dental Officer's 9 October 2026 warning that 'the volume of AI notes is getting incredibly scary' is the first regulator-level signal that note volume itself is a problem (GDPUK, 16 Oct 2026 [109]).
- Under UK GDPR a dental law firm warns that US AI vendors' user terms 'may put the dentist in breach of their own data protection policies' and that pseudonymised data 'is still personal data' (JFH Law [290]); the DSPT is due every 30 June and NHS contracts 'may not be renewed or commenced' without it (DSPT news 169 [143]). The ICO issues reprimands rather than fines to health bodies; none against a dental practice was found, the nearest being a GP surgery reprimanded in February 2026 for sending 23 years of records to an insurer that asked for five (Freevacy, 3 Feb 2026 [294]). Dental records are kept **11 years minimum**, a subject access request is answered **within one month, free**, and at least one practice site still advertises £10 and £50 fees (NHSBSA KA-01913 [295]; MDDUS [296]).

Hit through vendors: when the clearinghouse stops, the money stops

- The 2019 ransomware attack on the backup firm DDS Safe encrypted files at about **400 dental practices**; Henry Schein's 2023 attack notified 29,112 people and cost an estimated 10-15% of customers who 'went elsewhere'; Delta Dental's MOVEit breach exposed about **7 million** people's claims data; and the 21 February 2024 Change Healthcare attack (**190 million** people) left dental services 'not fully restored' nearly ten weeks later, with dentists mailing paper claims and paying for postage and call-centre time (KrebsOnSecurity, 29 Aug 2019 [298]; Cybersecurity Dive [299]; Orthodontic Products, 18 Dec 2023 [300]; ADA News, 16 Apr 2024 [301]).
- In the physician proxy survey after Change Healthcare, **36% suspended claim payments, 32% could not submit claims, 22% could not verify eligibility** and 90% were still losing revenue two months later (AMA follow-up survey [303]). Absolute Dental's 1,223,635-patient breach came through its managed-service

provider's account (HIPAA Journal [249]).

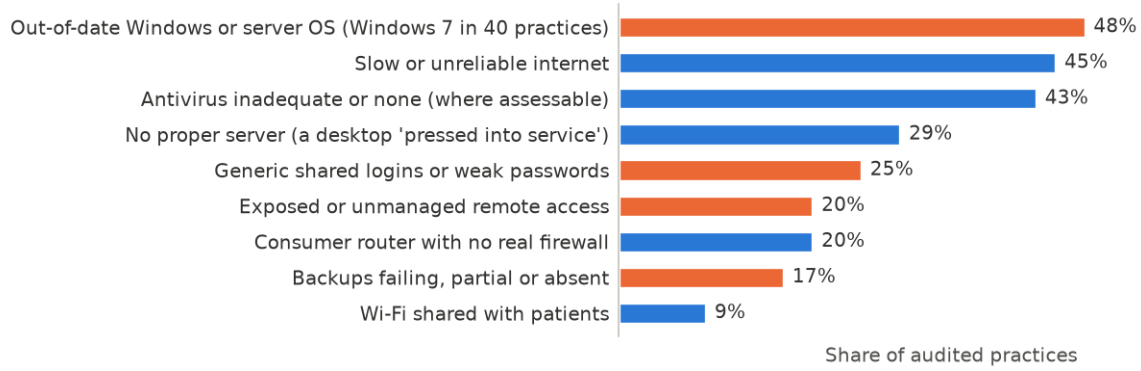
Table 5.2 Incidents that reached dental practices, 2019-2026

Incident	Date	Scale	What it stopped or exposed	Source
DDS Safe / PerCSoft ransomware (backup vendor)	26 Aug 2019	~400 practices	Backups encrypted; decryptor incomplete; payroll fears	KrebsOnSecurity [298]
Westend Dental ransomware	Oct 2020, reported Oct 2022	~17,000 patients	Records unavailable; $350,000 AG settlement	HIPAA Journal [267]
BDA cyberattack	30 Jul 2020	Member names, bank details, case notes	Legal action by members; no ICO outcome found	SD Magazine [304]
Henry Schein (AlphV/BlackCat)	Sep-Nov 2023	29,112 notified; 35TB claimed	E-commerce down twice; 10-15% of customers "went elsewhere"	Cybersecurity Dive [299]
Delta Dental of California (MOVEit)	May-Nov 2023	~7 million	Claims and procedure data	Orthodontic Products [300]
First Choice Dental ransomware	Oct 2023	150,000+	$1.2m class settlement	LexBlog [268]
Change Healthcare	21 Feb 2024	190 million	Claims, eligibility, ERAs and e-prescribing down for weeks	Healthcare Finance News [305]; ADA News [301]
Cosmetic Dental Group, Jersey	Aug 2024	"hundreds of islanders"	Referral letters and treatment reports taken	Bailiwick Express [306]
Absolute Dental (Nevada, 50+ sites) via MSP account	Feb-Mar 2025	1,223,635	SSNs, IDs, diagnoses, insurance, card data	HIPAA Journal [307]
32 Pearls; West Texas Oral Facial Surgery; Mid America Health	2025	23,517; 11,151; n/a	Ransomware; leak-site listing of imaging files	HIPAA Journal, 11 Aug 2025 [308]
Totem Lake Family Dentistry; 360 Dental PC; New Jersey Oral & Maxillofacial Surgery	2025-2026	3,400+; 11,273; 74,413	Email account access; intrusion; SSNs and treatment data	Becker's [309] snippet; TEISS [310]

Everyday security at three surgeries

- A UK IT provider's **192 on-site audits at 184 dental and orthodontic practices** (median three surgeries): 48% ran out-of-date Windows or server software (Windows 7 in 40 practices, a couple on XP), 45% slow or unreliable internet, 29% no proper server, **25% generic shared logins or weak passwords, 20% exposed or unmanaged remote access, 17% backups failing, partial or absent**, 9% Wi-Fi shared with patients, antivirus 'inadequate / none' at 43% of sites where assessable; anonymised cases include RDP 'left open to the entire internet', 'Every member of staff logged in with the same password: Password1!... no backup system at all', and nightly backup logs reading 'backed up files: 0' (Dental IT, 30 Jul 2026 [311] vendor). A UK practice adviser says 'a huge number of dental practices still aren't using Multi-factor Authentication' although most software offers it free (Practice Plan, 27 Mar 2026 [312] vendor).
- Insurance now enforces what the rule proposes: carriers probe MFA on email, remote access and the PMS, EDR, full-disk encryption and immutable offline backups, 'most carriers will not even issue a policy unless a practice proves it has implemented' them, misattesting lets the carrier deny a claim, and premiums run about **$1,000-$2,500 a year for a solo US practice** (Techvera [313] vendor; Florida Dental Association, 17 Jul 2025 [314]). Across all UK businesses, 43% identified a breach or attack in the year to April 2025 and phishing hit 35% of micro businesses (DSIT Cyber Security Breaches Survey 2025 [316]).

Everyday security at UK dental practices: 48% run out-of-date operating systems, 25% share logins, 20% expose remote access and 17% have failing or absent backups



Source: Dental IT, 30 Jul 2026 [311] (vendor; 192 on-site audits at 184 dental and orthodontic practices, median three surgeries; antivirus assessed at a subset). Cases: RDP 'left open to the entire internet'; 'Every member of staff logged in with the same password: Password1!'; nightly backup logs reading 'backed up files: 0'.

Table 5.3 Everyday security findings and the clocks around records

Everyday security finding	Figure	Source
Out-of-date operating system	48% of 137 detailed audits; Windows 7 in 40 practices; XP in "a couple"; Server 2008 (16) and 2012 (17)	Dental IT [311] vendor
Shared or weak logins	25%; "Password1!" for all staff, all administrators	Dental IT [311] vendor
Remote access exposed	20%; RDP open to the internet; home Macs via TeamViewer	Dental IT [311] vendor
Backups failing or absent	17%; logs "backed up files: 0"; a drawer of USB drives	Dental IT [311] vendor
Consumer router, no firewall; patient Wi-Fi shared	20%; 9%	Dental IT [311] vendor
US enforcement findings	Plain-text passwords reused across servers; servers in break rooms and bathrooms; no risk analysis; no BAA with IT helper; paper records on the floor	HIPAA Journal, Westend [267]; Paubox, Shen Smiles [266] vendor
Cyber insurance	$1,000-$2,500/yr solo; $2,500-$8,000+ groups; MFA, encryption and offline backups required to bind	Techvera [313] vendor; Florida Dental Association [314]
Records and SARs (UK)	11-year minimum retention; SAR within one month, free; £10/£50 fees still advertised	NHSBSA KA-01913 [295]; MDDUS [296]
Records retention (US)	5-10 years by state (Texas 5, California 7, Illinois and Michigan 10; minors longer)	Hawaii board compilation [183]

SECTION 6

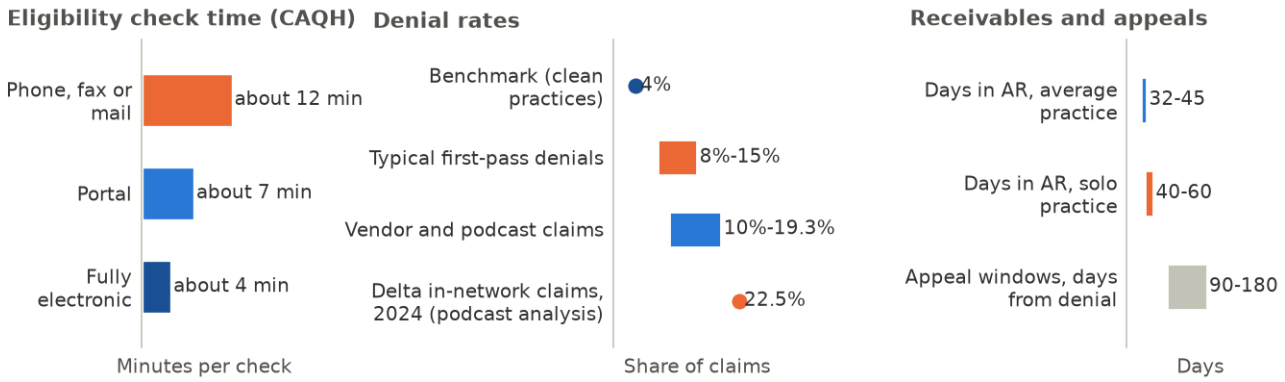
Verification and claiming: the data pull takes minutes, the phone call takes an hour, and the signature stays human

The US cycle has six stages and the automation sits unevenly across them: the eligibility pull is electronic for easy payers and a phone call for hard ones, the claim is cheap, the posting is semi-automatic and the denial rework is where the hours go. The UK cycle is one claim per course against a contract target, and its risks are deadlines and reconciliation rather than per-procedure adjudication. On both sides a signature, a declaration or a narrative stays human by law.

The US cycle: the pull takes minutes, the call takes an hour

- Collecting insurance details and verifying eligibility is where the money goes: **$2.1 billion** of dental administrative spend in 2023 (+15%) across **1.2 billion verifications**, about 12 minutes each by phone, fax or mail, 7 by portal and 4 fully electronic, with **82% electronic adoption for dental against 96% for medical and 15% still fully manual** (ADA News, 24 Mar 2025 [7]; Deepgram on CAQH CORE [317] secondary).
- The 270/271 transaction is inside the PMS, but carriers 'frequently' send 'nothing more than a single yes or no response', so a person 'must still interpret the data before importing'; '30-40% of plans still require phone calls', complex cases (Medicaid, HMO, dual coverage) take 30-45 minutes, and one practice described checks taking '30 minutes to 4 hours per patient' (Open Dental manual [173]; Ainora [319] vendor).
- The products that automate it differentiate on portal scraping and breakdown compilation, not on the EDI transaction: Zuub (Capterra 4.7 from 38; cannot search by insurer, name or date of birth), Pearl Precheck, Curve Eligibility+, NexHealth Verification (insurance details 'not written to the EHR'), Weave, Overjet and Verrific; none publishes a price (Capterra, Zuub [321] user, 2026; NexHealth help [323] vendor). The human alternative is priced: eAssist at **$250 a month per location for under 30 checks, $650 for 31-75, plus $299 set-up**; others at $2.50-$3.00 per check; a DSO employs verification specialists at $22-$27 an hour; Upwork sellers offer 'Breakdown of Benefits' at $5-$20 an hour with 50-plus proposals per post (eAssist [324] vendor, snippet; MB2 Dental job [326]; Upwork job [131] snippet).
- The claim is cheap (about **$0.51 per claim** through DentalXChange; Vyne Trellis on one flat fee; Change Healthcare 'not recommended' by Open Dental since the 2024 attack) and posting is semi-automatic (ERA auto-match with detached exceptions); the rework lives in stage six (XLDent, 27 Mar 2024 [162]).
- Denial rates have no regulator source: the defensible statement is a **4% benchmark against 8-15% at most practices**, with vendor and podcast figures of 10-19.3%, a podcast's reading of Delta Dental transparency files at **22.5% of 932,795 in-network claims denied in 2024**, and a 2026 survey in which nearly 80% of practices reported more denials (DentistryIQ [8] snippet; Henry Schein One, 10 Sep 2024 [179] vendor; The Dental Billing Podcast [329] secondary; Becker's on Zentist [330] snippet). A study of one large insurer found **72.88% of denials administrative**: bundling, thin narratives, frequency limits, coordination-of-benefits errors and wrong identifiers (DOAJ record [331]; Curve Dental, 1 Jul 2026 [164] vendor).
- Only about **5% of claims reach appeal and about 50% of those are reversed**; 54% of denied claims are eventually paid after an average of three resubmission cycles of 45-60 days; an appeal needs the original claim, the EOB with the reason highlighted, clinical notes, radiographs and a cover letter within 90-180 days, with 'a new, more detailed narrative' each round (dentalbilling.com [332] snippet). Days in AR run **32-45 on average, 40-60 at solo practices**, AR over 90 days is 18% against a 5-10% target, outsourced billing costs 4-10% of collections, and credentialing runs 30-120 days per carrier with CAQH re-attestation every 120 days (Dental Billing Assist [334] vendor; MedsDental [337] vendor; ADA News, 6 Jul 2026 [167]).

US verification and denials: $2.1bn a year on benefit checks at about 12 minutes by phone against 4 electronic; first-pass denials of 8-15% against a 4% benchmark; 32-60 days in AR



Sources: ADA News, 24 Mar 2025 [7] (CAQH Index: 1.2bn checks, $580m savings opportunity); StealthAgents on CAQH [159] (secondary); DentistryIQ [8] (snippet); The Dental Billing Podcast [329] (secondary; Delta transparency files); Becker's on Zentist [330] (snippet); Dental Billing Assist [334] (vendor); Arini [335] (vendor); dentalbilling.com [332] (snippet). 72.88% of denials are administrative; about 80% of practices report more denials; AR over 90 days runs 18% against a 5-10% target; 54% of denied claims are paid after about three cycles; about 5% are appealed and about half of appeals succeed.

Table 6.1 US verification and claims measures

US verification and claims measure	Figure	Source
Verification spend and volume	$2.1bn (2023, +15%); 1.2bn checks (+24%); $580m savings opportunity	ADA News [7]; StealthAgents [159] secondary
Minutes per check	~12 manual, ~7 portal, ~4 electronic (CAQH); 5-30 per patient; 15-30 by phone; 20+ per payer call; "soul-crushing"	StealthAgents [159] secondary; Curve [158] vendor; Henry Schein One [179] vendor
Hours per practice	~2 hours a day; 15-20 hours a week before automation; 25 appointments = 6-12 hours	Dentaltown, 2017 [160] vendor; Overjet case study [340] vendor
Channel mix	82% electronic adoption (dental) vs 96% (medical); 15% fully manual; "30-40% of plans still require phone calls"	Deepgram [317] secondary; Ainora [319] vendor
Outsourced verification price	$250-$650/location/month + $299 set-up (eAssist); $2.50-$3.00 per check; $5-$20/hr Upwork; $5-$8/hr Philippines	eAssist [324] vendor, snippet; Upwork [131] snippet; OnlineJobs.ph [127]
Claim cost	~$0.50-$0.51 per e-claim; Vyne flat monthly; Dentrix bundle	XLDent [162]; WVDA [341]
Denial rate	4% benchmark; 8-15% most practices; 10-19.3% vendor claims; 22.5% Delta in-network (podcast analysis); 72.88% administrative; ~80% report more denials	DentistryIQ [8] snippet; Dental Billing Podcast [329] secondary; Becker's [330] snippet
Appeals	~5% appealed; ~50% reversed; 54% of denials paid after ~3 cycles of 45-60 days; 90-180 day windows; 30 min simple rework; "three hours chasing a $28 reimbursement"	dentalbilling.com [332] snippet; DentistryIQ [8] snippet; Curve [164] vendor
AR	32-45 days average; 40-60 solo; 18% of AR over 90 days vs 5-10% target; collection rate 80% vs 97% top	Dental Billing Assist [334] vendor; Henry Schein One [5] vendor
Timely filing	12 months usual; 90 days to 15-18 months by plan; MassHealth 345 days temporary	Gerald guide [177]; MassHealth Bulletin 56 [342]
Outsourced billing	4-10% of collections (5-7% typical); $4-$12 per claim; $1,350/month under $40k	MedsDental [337] vendor; Dental Claim Support [343] vendor
Credentialing	30-120 days per carrier; CAQH every 120 days; Medicaid "months"; 85% unaware of ADA service	ADA News, 6 Jul 2026 [167]
CDT update	60 changes for 2026 (31 new, 6 deleted incl. D1352, D9248)	ADA News, 29 Sep 2025 [168]

The UK cycle: one claim per course against a target

- At the desk the patient signs the PR form; an exemption needs the 'evidence seen' box ticked; the tax-credit exemption is no longer accepted for courses accepted on or after 6 April 2025 (NHSBSA FP17 guidance V14 [344]). NHSBSA then sample-checks after the event through DWP records (about **1.2 million checks a month** across prescriptions and dental), posts an enquiry letter where it cannot confirm, and issues a **penalty charge notice of five times the charge up to £100 plus the charge**, with a 50% surcharge after 28 days (gov.uk transparency record [345]).
- The volumes: **115,463 dental PCNs to Universal Credit claimants alone in the year to 16 May 2025**; the NAO found about **30% of PCNs (1.7 million, £188m) later withdrawn** because a valid exemption was confirmed after challenge; NHS England pays NHSBSA £7.59 million for six months of checking (NHSBSA

FOI-02776 [347]; NAO 2019 [349]; Find a Tender [350]). Liability sits with the patient 'regardless of whether the FP17PR form is completed by the patient, someone on the patient's behalf, or dental practice staff', the DDU advises staff not to advise on entitlement, and the BDA objects that eligible people 'shouldn't face £100 penalties by default just for ticking the wrong box' (DDU Journal [351]; GDPUK [352]).

- The FP17 is validated against band rules: since 1 April 2022 the clinical data set must match the band claimed, failures first produce **comment codes 011-014** on the schedule, error 115 and 125 reject outright, BPE and NICE recall interval are mandatory on adult banded claims, and the dentist's declaration 'must be completed by a qualified dentist on every form' (NHSBSA FP17 guidance V11.4 [151]; NHSBSA common errors [152]).
- Year-end reconciliation 'is completed by September each year': **below 96% delivery, clawback up to 100% of the shortfall plus a breach notice; 96-100% carried forward; above 102% not paid** (NHSBSA KA-01672 [52]). The April 2026 reforms converted urgent care to a £75 credit per course (£15 fixed, £60 activity) with a minimum of 8.2% of contract value and converted all cash items back to UDAs at each practice's own rate, 'a redistribution of the current contract base' (Price Bailey [65] secondary; Dentistry.co.uk, 21 Jul 2026 [354]).
- The reporting gap followed: Compass's 'Unscheduled' figure shows only the £15 element, the £60 element 'cannot be identified on Compass', associates cannot see the split because eDEN is limited to commissioners and providers, and an LDC motion called it 'a fundamental contract management failure' (GDPUK, 13 Jul 2026 [12]). The practice software sends and flags but does not reconcile: Dentally marks a rejected claim 'Invalid' with a notification found through the NHS Claims report filter and fixed by hand, cannot invalidate a claim once an exemption is linked, and for Scotland requires each dentist to 'manually count teeth present, filled, and extracted for every check-up claim'; NHSBSA 'are unable to advise on EDI transmission issues' (Dentally help [356]; Dentally community, Feb-Apr 2026 [357] user, 2026; NHSBSA KA-01797 [141]). No NHSBSA statistic for the share of FP17s rejected or late was found, and no minutes-per-claim figure exists for UK desks.

Table 6.2 UK claiming measures

UK claiming measure	Figure	Source
FP17 window	2 months from completion; late claims lose UDAs; corrected claims also inside 2 months; 48 hours to process; FP17O exempt	NHSBSA KA-01820 [139]; NHSBSA schedule programme [140]
UDA values	Band 1 = 1; Band 2a 3, 2b 5, 2c 7; Band 3 = 12; denture repair 2; urgent £75 credit; £36.22 average per UDA	UDA Pulse [138] secondary; Price Bailey [65] secondary
Clawback rule	<96% recovered in full + breach notice; 96-100% carried forward; >102% lost; reconciled by September; SD86c with 2 months to dispute	NHSBSA KA-01672 [52]; ICLR [142]
Clawback totals	£479m (2024), £457m (2025); 32% of London practices (2022/23); BNSSG £14.4-£17.0m a year	Dentistry.co.uk [11] secondary; BNSSG FOI [60]
Exemption checks	~1.2m checks/month (all NHS); DWP API <5s; enquiry letter; PCN 5x charge up to £100 + charge; 50% surcharge after 28 days	gov.uk transparency record [345]
PCN volumes	115,463 dental PCNs to UC claimants (year to May 2025); 96,489 (Feb 2024-Jan 2025); 8,232 JSA; ~30% withdrawn after challenge; 47% of treatments claimed exempt; £212m lost (2017-18)	FOI-02776 [347]; FOI-02518 [348]; NAO 2019 [349]
Exemption checking contract	£7,589,216 for 1 Oct 2025-31 Mar 2026 (DECS)	Find a Tender [350]
PR form	Signed paper or certified e-signature; kept 2 years; no scan-and-destroy	NHSBSA KA-01916 [136]
Validation	Band-matching from 1 Apr 2022; comment codes 011-014; errors 115, 125; BPE and NICE interval mandatory	NHSBSA [353]; NHSBSA common errors [152]
2026 urgent-care split	£75 per course = £15 fixed + £60 activity; 8.2% of contract value (11 courses per £10,000); Compass shows only the £15 element	Price Bailey [65] secondary; GDPUK, 13 Jul 2026 [12]
Recovery Plan outcomes	£28 UDA floor produced no identifiable delivery increase; new patients per working day -5% in 2024/25; under 20% of 240 golden-hello posts filled by Feb 2025	NHS England analysis [358] snippet; Dentistry.co.uk, 4 Apr 2025 [359]

What a machine may not do

What stays human is fixed by law, consent or clinical judgement on both sides of the Atlantic, and it is the boundary every service has to be designed around rather than a gap to fill.

Table 6.3 Steps that stay human, and why

Step that stays human	Country	Why	Source
Interpreting and importing eligibility data; deciding what to tell the patient and collect	US	Carriers often return a bare yes/no; "a person must still interpret the data"; an eligibility response "is not the same as a guarantee of payment"	Open Dental [173]; EDI study guide [360]
The clinical narrative and the dentist's marking of decay on an appeal image	US	Reviewers "decide only from what was submitted"; each appeal needs a more detailed clinical narrative; the dentist is the clinician of record	Periospectives [333] snippet; dentalbilling.com [332] snippet
Deciding to treat, code and bill; informed consent covering AI use	US	Malpractice insurer: AI "a tool to assist, not replace" judgement; obtain consents that cover AI; liability "a legal gray area"	CDA/TDIC [279]
Any use of PHI by a tool without a BAA; replying to reviews	US	Covered-entity rules; consumer ChatGPT has no BAA; review-reply penalties	CDA/Abyde [260] vendor; HIPAA Journal [249]
Good-faith estimate to self-pay patients; credentialing attestations	US	Statutory 3-business-day duty; CAQH attestation is the provider's	ADA News [171]; ADA News, Jul 2026 [167]
The dentist's declaration on every FP17; the clinical band	UK	"must be completed by a qualified dentist on every form"; band follows the clinical data set	NHSBSA FP17 guidance [151]
The patient's signed PR declaration and the truthful "evidence seen" tick	UK	Paper or certified e-signature only; kept 2 years; liability for the exemption sits with the patient; DDU says staff should not advise on entitlement	NHSBSA KA-01916 [136]; DDU Journal [351]
Written plan and estimate; Montgomery-standard consent; medical history each visit	UK	GDC 1.6, 1.7, principle 3; a signature "does not indicate valid consent"; CQC requires a current medical history	GDC Standards [146]; Dentistry.co.uk, Feb 2021 [149]
Checking any AI transcript, summary or output; anything that supports diagnosis or acts without review	UK	MHRA: clinicians remain responsible; diagnostic or autonomous tools are medical devices; DPIA and DCB0160 required	Dentistry.co.uk, 2 Aug 2026 [287]; GPCE summary [289]
Fixing EDI transmission failures and disputing the SD86c	UK	NHSBSA will not advise on EDI; the dispute is the contractor's within 2 months	NHSBSA KA-01797 [141]; ICLR [142]

The pattern: the automatable layer is the data pull, the comparison and the exception list (eligibility responses against coverage tables, FP17 status against the two-month clock and the 96% floor, denial reason codes against appeal deadlines); the non-delegable layer is a named person accountable for a declaration or a decision (the dentist's FP17 declaration and clinical band, the patient's signed exemption form, the clinician's narrative and signature on an appeal, Montgomery-standard consent, the review of any AI transcript) and a contract the consumer AI app does not offer (the BAA). Reviews put the pain in the hand-back: bare yes/no responses that a person must interpret, breakdowns that live outside the ledger, and rejections flagged 'Invalid' that a person must find and fix.

SECTION 7

The platform audit angle: Open Dental publishes its prices, everyone else quotes, and nobody sells the reconciliation

Every US practice management system in scope draws the same four complaint clusters in 2024-2026 reviews: support hold times and inconsistent answers, reporting that will not export or disagrees with the analytics add-on, opaque and rising prices with add-ons, and painful migration. Open Dental is the only vendor with fully published pricing; UK pricing is almost entirely behind a demo; and nobody sells the reconciliation between the systems.

US practice management systems: prices and limits

- **Open Dental publishes everything**: $199 a month per location for the first year then $149 (up to three providers), eReminders, portal, web forms and payment portal free, eConfirmations $25-$40 a month plus $0.04 a text, Web Sched Recall $75, AI image analysis $149, custom SQL queries **$110 an hour**, online training $80 an hour, on-site training $4,325 a day, conversions about $1,450 per database plus $850 for documents and $900 for X-rays, non-refundable once complete (Open Dental fees [361] vendor; Open Dental conversions [362] vendor).
- Dentrix restructured its plans on 4 February 2026; third parties estimate $400-$1,200 a month on an annual contract plus $5,000-$15,000 of server hardware, support $100-$300 a month, modules $50-$300 each, and third-party API access at **$1,750 registration plus $25 per site per month for read access and $5,000 plus $38 for write** (user-reported) (CostBench changelog [363] secondary; CostBench hidden costs [364] secondary). Eaglesoft is estimated at $400-$600 a month with payment processing tied to Global Payments 'at above-market rates' and imaging in proprietary formats; Curve, Denticon, Sensei, Fuse, Oryx and tab32 are quote-only with aggregator guesses of $299-$750 (Dentaltown/RevUp, 4 Jun 2026 [366] vendor; CostBench pricing 2026 [368] secondary).
- The functional limits that matter to an auditor are specific: Dentrix Ascend cannot print route slips more than three days ahead, sends preauthorisations only to the primary insurer, and its Power Reporting is 'unnecessarily clumsy' so users buy Jarvis 'at additional cost'; Dentrix reports 'are generic and most don't allow importing them to Excel'; Fuse reporting 'falls short for when adjustments were made or refunds were applied'; Sensei Cloud has no online booking; Denticon required 're-charting the entire chart' to move a patient between accounts; Open Dental has 'allocated/unallocated/hidden payments in the ledger' that users find hard to read (Capterra, Dentrix [370] snippet; Dentaltown/RevUp [366] vendor).

Table 7.1 US PMS, part 1: published price and rating

US PMS	Published price	Rating (reviews)
Dentrix (on-premise)	None; est. $400-$1,200/month + server; API $1,750-$5,000 + per-site fees	Capterra 4.3 (376)
Dentrix Ascend (cloud)	None; est. ~$500/location	Capterra 4.1 (235)
Eaglesoft	None; est. $350-$1,000/month; per-user; needs a physical server	Software Advice 4.1
Open Dental	$199 then $149/month; add-ons $5-$165; queries $110/hr; conversion ~$1,450 + $850 + $900	Capterra 4.6 (88)
Curve (cloud)	None; "startup pricing"; est. $299-$599 tiers	Capterra 4.4 (285, Mar 2026)
Denticon (Planet DDS)	None; est. $199-$750 (conflicting)	Capterra 4.5 (118)
Sensei Cloud (Carestream)	None (US); UK G-Cloud £135-£185/licence	GetApp 3.7 (31); value 3.4
Fuse (Patterson)	None	G2 3.2 (4)
Oryx; tab32	None	G2 4.8-4.9 (22-29); none
Migration (any)	Conversion $500-$2,500; implementation $1,000-$5,000; 2-4 weeks disruption, 4-8 weeks migration	n/a

Table 7.2 US PMS, part 2: what it does not do, per users and vendor

US PMS	What it does not do, per users and vendor	Source
Dentrix (on-premise)	Reports not exportable to Excel; support holds "no less than 20 minutes"; billing continued after cancellation; updates "a headache"; "Too many clicking steps"	Capterra [370] snippet; CostBench [364] secondary
Dentrix Ascend (cloud)	Route slips only 3 days ahead; preauth to primary only; no online medical-history intake; Power Reporting "clumsy"; X-ray features "did not work alot of the time"; support 6am-5pm Mountain	Capterra [374] snippet; TechRadar [369] snippet
Eaglesoft	"Really developing and fixing Eaglesoft stopped 15 years ago"; "Imaging resolution is stuck in the 80s"; "so many ways to get to the same point"; updates "can take hours"; proprietary imaging	Software Advice [375] snippet; Dentaltown/RevUp [366] vendor
Open Dental	Chart and schedule not open together; "billing and accounting are very hard"; support reps "gave a different answer"; ledger allocations confusing; "many new hires require" training	Open Dental fees [361] vendor; Capterra [376] snippet
Curve (cloud)	"transition with billing and accounts receivable were difficult... lag in insurance reimbursement. Prepare financially"; image import "tedious"; no denture or implant-abutment charting; cloud "stopped working a few times"; suggestions "don't seem to be taken seriously"	Capterra [377] snippet; Curve pricing [378] vendor
Denticon (Planet DDS)	"expensive with frequent cost increases, poor xray integration and worthless support"; a 2026 request "over six months without resolution"; "way too many clicks"; frequent downtime	Capterra [373] snippet
Sensei Cloud (Carestream)	"price almost doubled within our first month once add-ons for essential features were included"; no online booking; reporting incomplete; support "abysmal" (May 2025); UK exit: CSV-only paid export, 60 days	GetApp [372] snippet; G-Cloud 14 [379] vendor
Fuse (Patterson)	Reporting misses adjustments and refunds; provider numbers "altered too much"	G2 [371] snippet
Oryx; tab32	Reviewers found Ascend "easier to use, set up, and administer"; tab32 appears only in Fiverr setup gigs	G2 [380] snippet
Migration (any)	"Insurance claims in progress and proprietary-format imaging often don't transfer cleanly"	Dentaltown/RevUp [366] vendor; DentiMax [381] vendor

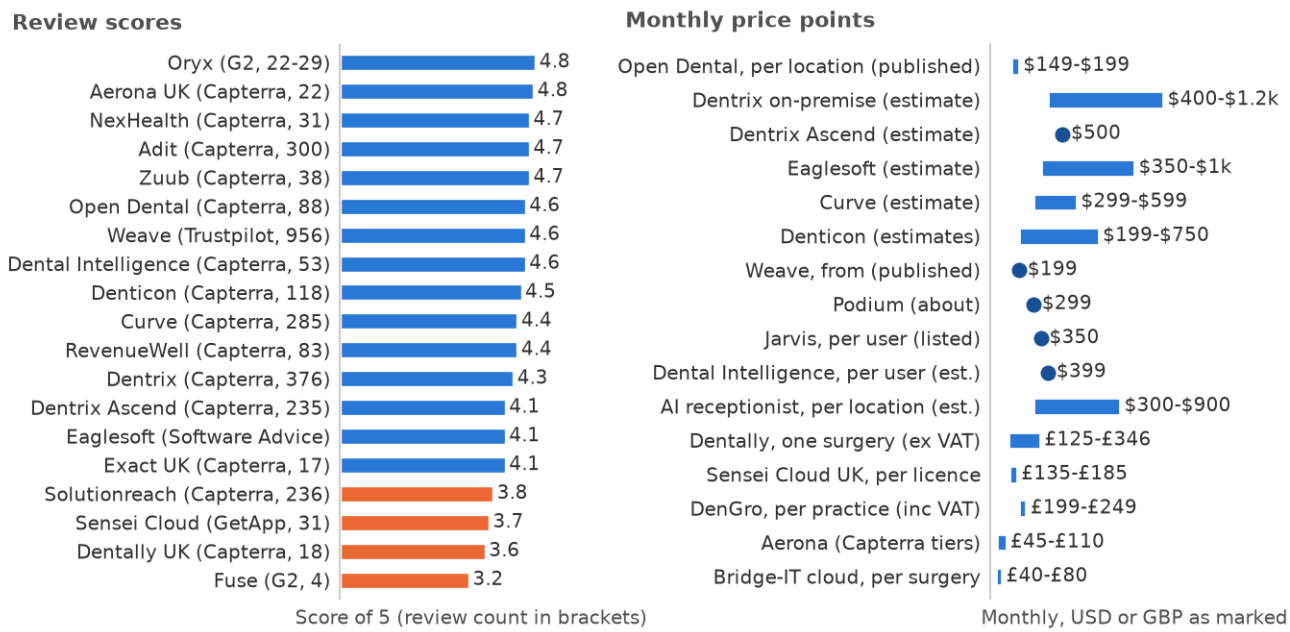
The communication, analytics and AI-receptionist layer

- Weave starts 'from $199 per month' with tiers priced on request and holds a Trustpilot 4.6 from 956 reviews with 9% one-star: 'no way to cancel on the website', 'left on hold', a promised $1,600 buy-out credit never issued and a number 'ported incorrectly' (January 2026), charged after giving 60 days' notice (Weave pricing [382] vendor; Trustpilot, Weave [383] user, 2026). NexHealth (Capterra 4.7 from 31) has no public prices, annual plans paid up front with 90 days' non-renewal notice, and complaints that with Curve it 'didn't integrate as nicely' and support is 'hard to get in touch with... once we were an established customer' (NexHealth pricing [385] vendor; Capterra, NexHealth [386] snippet).
- RevenueWell's pricing page returns 404 and reviews say 'Very hard to get away from contarct' and set-up 'took over three months'; Solutionreach scores 3.8 from 236 with a five-day outage 'without a callback' and auto-renewal (Capterra, RevenueWell [387] snippet; Capterra, Solutionreach [388] snippet). The analytics layer sells the number but not the follow-up: Dental Intelligence (4.6 from 53; start-up fee, no refunds, auto-renewal; SelectHub guesses $399 a user a month) documents an 'Unscheduled Patients' board as a paid layer, one reviewer says 'the numbers were always inaccurate and DI wouldn't/couldn't fix it', and DI's own help centre explains its claims-aging figure will not match Dentrix's Insurance Aging Report, both 'valid', with no reconciliation method given (G2, DI [391] snippet; DI vs Dentrix aging [392] vendor).
- AI receptionists have **no public pricing and no independent reviews**: Arini's, Viva's and Peerlogic's pricing pages are 404, dental-specific platforms are estimated at **$300-$900 per location per month plus $0-$2,000 set-up and $50-$300 a month for PMS integration**, and vendors concede 'limitations exist in handling complex nuances or accents' (AI Frontdesk on Arini [396] vendor; Sagnik Bhattacharya, 27 Sep 2026 [397] secondary). Integration depth is thin: Zapier app pages for Open Dental and Dentally return 404, Sensei's API needs Carestream approval, and Dentally is the only in-scope PMS with its own Zapier page (US Tech Automations [399] vendor).

UK pricing behind a demo, and the first-hand pain on Dentally's own community

- Dentally publishes tiers excluding VAT, **Starter about £125-£134 for one surgery, Essentials £220-£238, Pro £320-£346**; Starter has chat-only support and no portal (no digital forms, online booking or kiosk); AI clinical notes are capped at 45 or 90 transcriptions per surgery per month; recalls, SMS costs, migration fees and contract terms are not on the page (Dentally pricing [401] vendor; Whito [402] secondary). Scottish users wrote (February to April 2026) that the software is 'clunkier than older softwares especially for use within the Scottish NHS SDR', that the GP17 DMF count is manual 'for every check-up claim', that 'phone support is a paid add-on' and that sister practices were trained 'with wrong information' that delayed claims 'by at least a month'; the product director replied that 'Auto-add fees and GP17 DMF are known gaps' (Dentally community [357] user, 2026).
- Exact (Software of Excellence) publishes no price and scores Capterra 4.1 from 17 with value for money at 3.3; Aerona lists £45-£110 tiers on Capterra (4.8 from 22); Bridge-IT cloud is £40-£80 per surgery; DenGro (lead CRM) **£199 or £249 per practice per month including VAT**, rolling 30 days; both Exact's and Dentally's Trustpilot pages have zero reviews (Capterra, EXACT [403] snippet; DenGro pricing [405] vendor). Sensei Cloud's UK G-Cloud terms are the clearest statement of exit cost in the sample: 12-month rolling contract with 90 days' notice, data export on exit **CSV only, on written request and advance payment, available for 60 days**, API access requiring Carestream approval (G-Cloud 14 [379] vendor).
- UK AI receptionists are priced by a consultant at **£150-£800 a month plus £500-£3,000 set-up**, with the caveats that they 'occasionally say something inappropriate', fail on complex multi-surgery scheduling, and below about 50 convertible missed calls a month may cost more than they recover (Lilach Bullock [406] vendor). Nothing in the UK stack reconciles FP17 submissions to Compass or eDEN for the 2026 unscheduled target or per-performer pay, no Denplan reconciliation feature was found in Exact or Dentally, and lab tracking in CareStack 'can be used only to keep track of orders internally. There is no communication to the labs' (GDPUK, 13 Jul 2026 [12]; CareStack [408] vendor).

Platforms: review scores run from Fuse 3.2 to Oryx 4.9 and Open Dental is the only US PMS with fully published prices ($149-$199 a month); UK Dentally lists £125-£346 a surgery and everyone else quotes



Sources: Capterra, G2, GetApp, Software Advice and Trustpilot pages as seen in search snippets [370] [374] [375] [376] [377] [373] [372] [371] [383] [386] [388] [432]; vendor price pages [361] [382] [401] [405]; CostBench estimates [368] (secondary); Whito [402] (secondary).

Table 7.3 The stack for a two-dentist practice, 2026

Stack for a 2-dentist practice, 2026	Monthly cost	Basis	Source
Open Dental + web scheduling + eConfirmations + eClipboard	~$420-$435 before hosting; eServices bundle $165 alternative; conversion ~$3,200 one-off	Published fees	Open Dental fees [361] vendor
Dentrix on-premise	$400-$1,200 + $5,000-$15,000 server every few years; support $100-$300; modules $50-$300 each	Vendor blog and aggregator estimates	Dentaltown/RevUp [366] vendor; CostBench [364] secondary
Eaglesoft; Ascend; Curve	$400-$600; ~$500; $299-$599	Estimates	CostBench pricing 2026 [368] secondary
Comms layer	Weave from $199; Jarvis $350/user; DI ~$399/user (unconfirmed); Podium ~$299	Vendor and aggregator	Weave pricing [382] vendor; SelectHub [410] secondary
AI receptionist	$300-$900/location + $0-$2,000 set-up + $50-$300 integration (US); £150-£800 + £500-£3,000 set-up (UK)	Consultant estimates	Sagnik Bhattacharya [397] secondary; Lilach Bullock [406] vendor
Cyber insurance	~$80-$200/month	Broker and association	Insureon [315]; FDA blog [314]
Whole US stack	$800-$2,000/month before phones and payments; 66% of practices budget $401-$500/month for workflow software	Inference; Software Advice survey	Becker's on Software Advice [411] snippet
Dentally, 2 surgeries (excl. VAT)	Starter £195-£210; Essentials £300-£324; Pro £415-£448	Published, slider labels ambiguous	Dentally pricing [401] vendor
Sensei Cloud UK; DenGro; Bridge-IT	£135-£185/licence; £199-£249; £40-£80/surgery	Published	G-Cloud 14 [379]; DenGro [405]; Whito [402] secondary

Misconfiguration, underuse, and the audit nobody prices

- Dentrix Enterprise's own clean-up checklist (2020) is the de facto audit scope: duplicate insurance plans and employers, departed providers still active, patients never inactivated, **multiple continuing-care types that 'cause recall dates to reset incorrectly and patients to be missed on lists'**, procedures not billed, claims not sent, outdated fee schedules (Dentrix Enterprise blog [412] vendor). Other evidence of underuse: only 26% of practices offered online booking in NexHealth's 2022 survey; eligibility is 'almost always going to be last'; two-thirds of UK practices track leads on paper; a two-doctor practice paid $300 a month for a reminder program 'it had barely used for months'; two analytics products reading the same Dentrix database show different claims-aging totals (SchedulingKit [413] secondary; Thriving Dentist, 14 Oct 2026 [33] vendor).
- The audit and consultant market is unpriced because it is unbundled: vendors give the checklist away, analytics vendors sell the dashboard, offshore labour sells the keystrokes, and nobody sells the validation step. **No published price for a 'Dentrix audit', 'Exact health check', 'recall audit' or 'database cleanup' was found.** The anchors that exist are Open Dental's $110 an hour for custom queries and $4,325 a day on site; an Upwork Open Dental posting at **$4-$7 an hour** (30+ hours a week); Philippine dental VAs at $600-$700 a month; managed IT at $1,200-$1,500 a month per office; coder-forum anchors of consulting 'from $75 hourly' and audits 'from $500 and my biggest one was $15,000'; and a UK trainer day at **£600-£1,500** (Open Dental fees [361] vendor; Upwork listing [417] snippet; AAPC forum [420] user; Dominate Dental training [421] vendor). A DSO job ad for a 'Fee Schedule Administrator' auditing 'fee schedules against contracted reimbursement terms' shows the work exists as an in-house role at group level and not as a product for a three-chair practice (Greenhouse, PCDM [423]; MedsDental audit [419] vendor).

Table 7.4 Audit and consultant price anchors

Audit and consultant price anchor	Figure	Source
Open Dental custom queries; sheet design; training; on-site	$110/hr ($220 expedited); $55/hr; $80/hr; $4,325/day + $1,325/day	Open Dental fees [361] vendor
Dentrix consultant or trainer	No published rate; contractor trainers via MedSys; implementation $1,000-$5,000; MSP $1,200-$1,500/month	MedSys [418] snippet; CostBench [364] secondary
Offshore keystrokes	$4-$7/hr posting; $5-$8/hr verification; $600-$700/month VA	Upwork [417] snippet; OnlineJobs.ph [22]
Billing audit	MedsDental "Practice Audit Solution", no price; coder forum $500-$700 small practice, $500-$15,000 audits	MedsDental [419] vendor; AAPC [424] user
UK training day; UK consultant hour	£600-£1,500/day; £75-£200/hr (marketing); SOE e-learning free	Dominate Dental [421] vendor; SOE training [425] vendor
General consulting benchmarks	IT $100-$300/hr; marketing $75-$200/hr; most consultants $100-$250/hr	Consulting Success [426] secondary

The audit angle: set-up is a paid line item at every vendor and the 'configured but not working' complaints are all failures a periodic check would have caught before a patient or an auditor did: a recall type that resets dates, an eligibility add-on that never writes back, a claims-aging total that disagrees with the analytics layer, an FP17 flagged 'Invalid' in a filter nobody opens, a reminder program paid for and unused. No vendor in scope documents a misconfiguration rate, publishes a utilisation statistic or sells independent validation after go-live; the only priced anchors are a vendor's hourly SQL rate, an offshore poster's $4 an hour and a UK trainer's day rate.

SECTION 8

Ten handoffs drop the same three things: the owner, the timing and the money

Each handoff in the patient and payment chain is owned by a different product, and none owns the step before or after it: the lead CRM stops at 'new patient created', the PMS stops at 'treatment planned', the analytics layer surfaces 'unscheduled' but relies on a human to call, and the comms layer only knows about appointments that already exist. The numbers practices trust (case acceptance, aging, recall) differ by product because each defines the denominator differently, so the gap is definitional before it is operational. Every handoff statistic is vendor or consultant data; no practitioner forum post on any handoff was retrievable.

Table 8.1 Twelve handoffs, part 1: what drops, and the numbers

Handoff	What drops	Numbers
Call to booked	The caller: 31 of 100 hang up; 13 of 100 walk away unbooked; 4.8 get a call back	21 of 100 book; new-patient conversion 25%; after-hours ~30-75% of misses
Website or Facebook lead to booked	The lead: form submissions never contacted; two-thirds tracked on paper (UK)	5-8% form-to-booking with manual follow-up vs 15-22% with sub-5-minute response (unsourced); 2-3 week gaps (UK)
Booked to seated	The appointment: no-shows and same-day cancellations	4% (HSO), 6.9% no-show and 12.9% cancellations (Planet DDS 2025), 9-14% academic; 23.4% to 19.2% broken appointments after automated confirmation (2003)
Seated to verified	The breakdown: difficult payers checked last; eligibility not written to the EHR	~2 hours a day; 12 vs 4 minutes; 15% fully manual
Seated to treatment accepted	The case: "I'll think about it"; cost, insurance confusion, no follow-up	42-46% average vs 75-83% top 10%; treatment-dollar 39% vs 58%; perio-to-prophy 24% vs 70%; follow-ups raise acceptance "10% or more"; "60-70% of practices do no systematic follow-up" (unsourced)
Accepted to scheduled	The plan: accepted but never booked; nobody owns the list	"$1 to $1.5 million" unscheduled per practice; "practices that follow up within 48 hours recover 20-35%" (vendor)
Completed to claim paid	The claim: denied for admin reasons; aging reports disagree; small denials written off	8-15% denied; 54% paid after ~3 cycles; 5% appealed; AR 32-60 days; insurance "about 50% of income"
Completed to recall booked	The next visit: recall list "the easiest work to move to tomorrow"; duplicate continuing-care types reset dates	Reappointment 42-81% average vs 94% top; patients leaving with an appointment return at 80-90% vs 35-45% without
Lapsed to reactivated	The dormant patient: 800-2,000 per practice; four hours per list by phone	Reactivation recovers 10-25%; "40% of patients who do not make follow-up appointments never come back" (unsourced)
UK enquiry to new-patient exam	The enquiry: email inbox, paper diary, 2-3 week gap; NHS status unclear; 13.5% of practices unreachable	Fewer than half check leads daily; 47% offer private to NHS enquirers
UK exemption to clean FP17 and UDA credit	The claim: evidence box, band mismatch, two-month window, urgent split invisible on Compass	115,463 PCNs (UC) a year, ~30% withdrawn; comment codes; £900m clawback
UK private estimate to signed consent and deposit	The estimate: 22% paid more than quoted; 21% prices unclear; no UK data on estimate-to-consent conversion	GDC 1.6 written plan and estimate; deposit 50-100% at one practice

Table 8.2 Twelve handoffs, part 2: products that claim the step, and sources

Handoff	Products that claim it	Source
Call to booked	Weave, Adit, Arini, Viva, Annie, Peerlogic, Patient Prism (call intelligence and AI answering)	Patient Prism [4] vendor; Peerlogic [81] vendor
Website or Facebook lead to booked	DenGro, Viva, HubSpot set-ups, VAs at $3 per booked lead	Viva [86] vendor; Dentistry.co.uk, DenGro [84] vendor
Booked to seated	Weave, Solutionreach, RevenueWell, Lighthouse, Open Dental eConfirmations ($25-$40), NexHealth waitlist	Henry Schein One [5] vendor; Ainora [82] vendor; SchedulingKit [413] secondary
Seated to verified	Zuub, Pearl Precheck, Curve Eligibility+, NexHealth Verification, Weave, eAssist, VAs	Dentaltown, 2017 [160] vendor; NexHealth help [323] vendor
Seated to treatment accepted	DI follow-up tasks, Adit lists, Planet DDS treatment-plan analytics, consultants ("call the next morning")	Henry Schein One, 2024 [6] vendor; Clerri [91] vendor; DentistryIQ, Levin [181]

Handoff	Products that claim it	Source
Accepted to scheduled	DI "Unscheduled Patients" board; VA ads "Contact patients with pending treatment plans or unscheduled visits"	Henry Schein One [6] vendor; OnlineJobs.ph 1491618 [125]
Completed to claim paid	Vyne "denial support", billing companies at 4-10%, DI claims-aging, Dentrix Insurance Aging	DentistryIQ [8] snippet; Dental Claim Support [427] vendor
Completed to recall booked	Open Dental Web Sched Recall ($75), NexHealth one-click recalls, Hello Patient, comms layers	Hello Patient [180] vendor; Dentrix Enterprise [412] vendor; Clerri [95] vendor
Lapsed to reactivated	Adit "AI Recare" (coming soon), reactivation letters and scripts, VAs	Clerri [95] vendor; ACT Dental [96] vendor; Enterprise DNA [428] vendor
UK enquiry to new-patient exam	DenGro (£199-£249), practice websites' "request a callback", RoboReception	Dentistry.co.uk, DenGro [84] vendor; Healthwatch East Sussex [83]
UK exemption to clean FP17 and UDA credit	EXACT and Dentally EDI with "Invalid" flags; UDA Pulse guide; nothing reconciles to Compass or eDEN	NHSBSA FOI-02776 [347]; Dentally help [356] vendor; GDPUK [12]
UK private estimate to signed consent and deposit	PMS treatment-plan printouts; no product found	GDPUK, Which? [71]; SpaDental [147]

Definitional mismatch example: Dental Intelligence's claims-aging figure will not match Dentrix's Insurance Aging Report because Dentrix recalculates estimates each run and includes inactive patients while DI snapshots at claim creation; both are 'valid' and no reconciliation method is given [392].

SECTION 9

Dentrix against Open Dental and Curve; Weave against NexHealth and Solutionreach; Dentally against Exact: what users said, and when

The dated quotes below are what the reader can put in front of a practice owner; most come from search-engine summaries of review sites that blocked direct reading, so exact dates are given only where the summary carried them.

Table 9.1 Platforms: rating, dated user words, source

Platform	Rating	Dated user words	Source
Dentrix	Capterra 4.3 (376)	"every month customer service says they will close my account" (former owner, 2025-26); "constant updates have been a headache"; "Do they only have one person installing Dentrix updates? I am really beginning to regret working with Dentrix" (Paul T., 2026); "Dentrix's training model makes scaling nearly impossible" (Dr Josh Berd, 2026); support "$100 to $300 per month", "software updates breaking existing functionality, billing issues and unexpected charges" (CostBench, Jun 2026)	Capterra [370] snippet; Dentaltown/RevUp, 4 Jun 2026 [366] vendor; CostBench [363] secondary
Dentrix Ascend	Capterra 4.1 (235)	"Power Reporting in Dentrix Ascend feels unnecessarily clumsy and frustrating to use... the best solution seems to be Jarvis... but it comes with an additional cost"; "xrays features did not work alot of the time"; "confusing ledger, user-unfriendly insurance setup, inability to edit procedure codes"; fixes take "a day or two"	Capterra [374] snippet
Eaglesoft	Software Advice 4.1	"a little glitchy at times" (2026); "Really developing and fixing Eaglesoft stopped 15 years ago. Changes today are mainly cosmetic" (James K., CIO, 2026); "Eaglesoft today still looks like Eaglesoft back in 2015" (Duc T.); r/Dentistry (Jul 2024) preferred Open Dental, one said Curve "felt like a step back from Eaglesoft"	Software Advice [375] snippet; Dentaltown/RevUp [366] vendor; r/Dentistry [429] snippet
Open Dental	Capterra 4.6 (88)	"billing and accounting are very hard" (1 star, Mar 2025); support "gave inaccurate advice on two issues" and another rep "gave a different answer"; a Jan 2026 2-star lists only affordability as a pro; "Only thing that sucks is the allocated/unallocated/hidden payments in the ledger" (Reddit via vendor)	Capterra [376] snippet; Dentaltown/RevUp [366] vendor
Curve	Capterra 4.4 (285, 13 Mar 2026)	"The transition with billing and accounts receivable were difficult. Getting the insurance billing mastered took some time which caused a lag in insurance reimbursement. Prepare financially before making the switch!"; "The import of images from old chart software to curve is tedious. Charting is incomplete. There's no denture drawing."	Capterra [377] snippet
Denticon	Capterra 4.5 (118)	"expensive with frequent cost increases, poor xray integration and worthless support"; a 2026 support request "over six months without resolution"	Capterra [373] snippet
Sensei Cloud	GetApp 3.7 (31)	"the price almost doubled within our first month once add-ons for essential features were included" (May 2025, 1 star); support "abysmal", issues "take days, if not weeks"	Capterra [430] snippet
Weave	Trustpilot 4.6 (956; 9% 1-star)	"no way to cancel on the website; chat refers to the customer service number; callers are... left on hold"; promised $1,600 credit never issued, number "ported incorrectly" (Jan 2026); "gave 60 days' notice, though only 30 were required, and the company kept charging"; "kept separating things that we needed but keep bumping up the price on us"	Trustpilot [383] user, 2026; Capterra [384] snippet
NexHealth	Capterra 4.7 (31)	With Curve "didn't integrate as nicely... had to work hard to set up the booking workflow"; "way too expensive"; "hard to get in touch with tech support once we were an established customer"; 1-star on rigid billing terms and a refused refund	Capterra [386] snippet
RevenueWell	Capterra 4.4 (83)	"Very hard to get away from contarct"; set-up "took over three months" with the wrong Facebook page; SEO/Google Ads buyer "told to wait for results that never came" (2025); "Reminders are not sent out automatically as promised"	Capterra [387] snippet
Solutionreach	Capterra 3.8 (236); support 3.5	Five-day outage "without a callback"; "worst customer service" in years as an office manager; dedicated rep replaced by a slow technician after 90 days; auto-renewal; duplicate or late reminders	Capterra [388] snippet; ComplaintsBoard [431] user
Dental Intelligence	Capterra 4.6 (53)	"the numbers were always inaccurate and DI wouldn't/couldn't fix it" (G2); "a little too much for our small practice"; start-up fee, no refunds, auto-renewal by default	G2 [391] snippet; Capterra [390] snippet

Platform	Rating	Dated user words	Source
Zuub (verification)	Capterra 4.7 (38, Mar 2026)	Saves time and human error on benefit checks; lacks search by insurer, name or date of birth; "sometimes patients don't have their sub ID"	Capterra [321] snippet
AI receptionists (Arini, Annie, Viva, Peerlogic)	No independent reviews	"the latest grift"; "confidently made things up when it didn't know the answer"; "The longer the conversation goes on, the worse it gets" (Dentaltown, 7 Mar 2025, vendor-authored); "Patients hung up on robot voice" (Reddit via vendor, Mar 2026, unverified)	Dentaltown [115] vendor; Swiss Monkey [116] vendor
Dentally (UK)	Capterra UK 3.6 (18); Trustpilot 0	"clunkier than older softwares especially for use within the Scottish NHS SDR" (17 Feb 2026); GP17 DMF "manually count teeth present, filled, and extracted for every check-up claim"; "phone support is a paid add-on"; product director: "Charting isn't where we want it to be" (25 Feb 2026), "Dentally isn't where we want it to be for Scottish practices" (15 Apr 2026)	Dentally community [357] user, 2026; Capterra UK [432]
Exact / Software of Excellence (UK)	Capterra 4.1 (17); value 3.3; Trustpilot 0	"When some adjustments were needed, the communication with customer support was difficult and the charges quite high" (1 star); 2016 practice-manager testimonial praises support	Capterra [403] snippet; Dentistry.co.uk, 2016 [433]
Compass (NHSBSA)	n/a	"You cannot look at Compass and know where you are anymore"; "unknowingly driving a car with a faulty speedometer"; "This is not a minor reporting inconvenience. It is a fundamental contract management failure" (LDC motion, Jul 2026); 2016 rollout: login failures, inability to add dentists to contracts	GDPUK, 13 Jul 2026 [12]; Dentistry.co.uk, 2016 [434]

Seven gaps that recur regardless of vendor or country

- Reporting that cannot be exported or trusted without a paid analytics layer (Dentrix, Ascend, Fuse, Sensei, Denticon, the DI-versus-Dentrix aging mismatch).
- Unscheduled-treatment follow-up left to a human list (46% acceptance; DI, Adit and Planet DDS boards; '60-70% no systematic follow-up').
- Recall lists unworked and corrupted by duplicate continuing-care types.
- Insurance eligibility and claims aging handled outside the PMS at hours a day (Weave, NexHealth and Curve add-ons that do not write back to the ledger).
- Contract lock-in and opaque pricing with add-on creep (Weave, Solutionreach, RevenueWell, NexHealth, DI, Sensei, Eaglesoft subscription, Dentrix API fees).
- Migration loss of images and in-flight claims with non-refundable conversion fees (Curve, Open Dental, Sensei's CSV-only export).
- UK-specific: NHS claim validation and UDA-to-Compass reconciliation left to the practice while leads are handled on paper (Dentally community, GDPUK, DenGro). The consultant-shaped work in all seven is reconciliation, not configuration: aging report against analytics, Compass against the PMS, lead CRM against the PMS patient list, recall types against continuing-care lists (Dentaltown/RevUp [366] vendor; Dentally community [357] user, 2026; GDPUK, 13 Jul 2026 [12]).

SECTION 10

Thirteen service gaps, ranked by problem signal and vendor weakness

The ranking scores each gap on how strong and recent the problem signal is (regulator findings, enforcement, dated user complaints, job-ad and freelance demand, search phrases) and how weak the vendor coverage of the exact sub-problem is (whether a named product claims it and whether any independent source confirms it). A third column marks fit with a self-hosted or private-data angle, strongest where the work touches patient, claim or NHS records that the law keeps under a BAA, UK GDPR and the DSPT; a fourth marks whether the natural buyer is a dental marketing agency rather than a practice. Staffing shortage, the problem owners name first, is not listed because no software or data gap describes it. Crowded problems that are not gaps follow the thirteen in grey. The report describes existing products factually and does not design or recommend solutions.

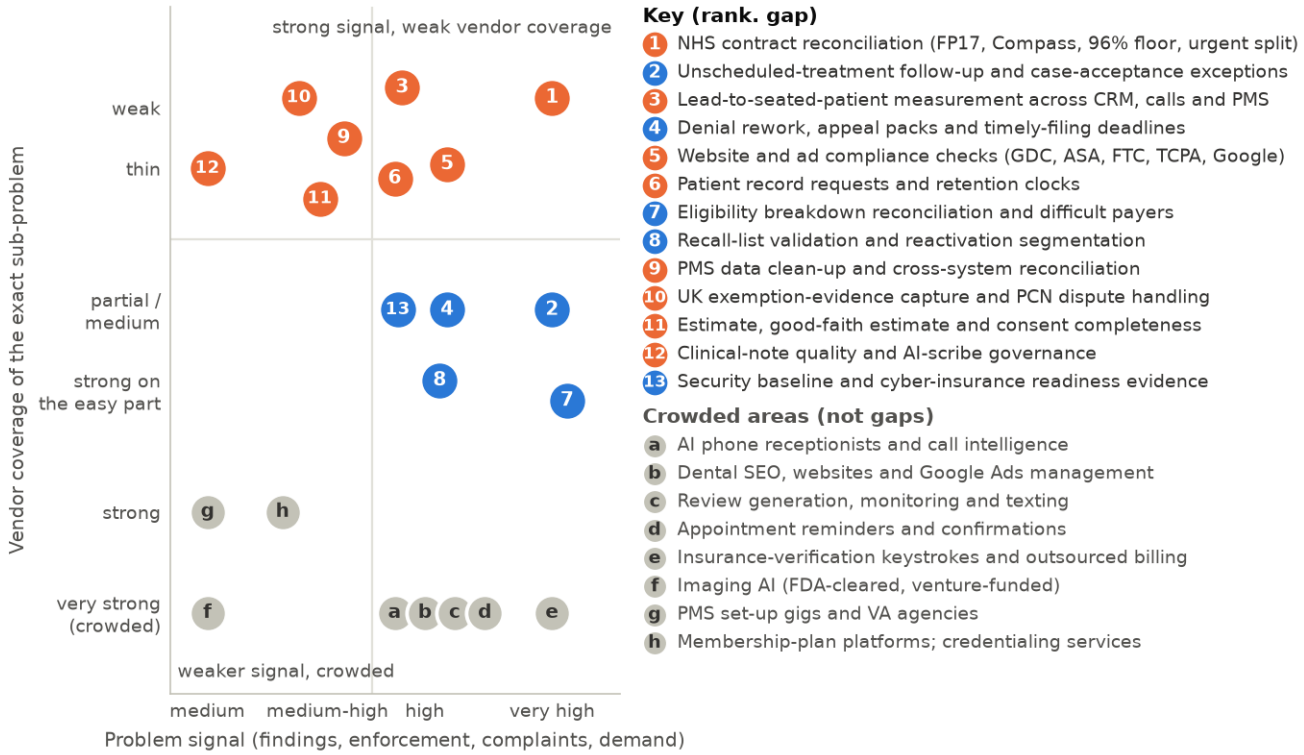
Table 10.1 The ranking, with the eight crowded areas in grey at the end

#	Gap	Problem signal	Vendor coverage of the exact sub-problem	Private data fit	Agency sale	Country	Automatable
1	NHS contract reconciliation: FP17 status against Compass, two-month window, 96% floor, 2026 urgent-care split, performer splits	Very high: £900m clawback, "faulty speedometer", LDC motion Jul 2026, late claims lose UDAs, Dentally "Invalid" flags fixed by hand, "compass cut off dates" autocomplete	Weak: PMS sends and flags; UDA Pulse is a guide; Compass cannot show the £60 element; eDEN closed to associates; no reconciliation product found	Strongest (NHS claim and patient data; DSPT)	No	UK	Fully for detection and exception lists; declaration, band and dispute human
2	Unscheduled-treatment follow-up and case-acceptance exception tracking	Very high: 42-46% acceptance, "$1-1.5m" unscheduled, VA ads "Contact patients with pending treatment plans", 34.8% plan AI notes but no follow-up tool	Partial: DI, Adit and Planet DDS surface lists; execution human; "AI Recare" coming soon; denominators differ by product	Strong (treatment plans, PHI)	No	US; UK estimate-to-consent unmeasured	Partly: list, owner, timing and reasons automatable; the clinical conversation human
3	Lead-to-seated-patient measurement across agency CRM, call tracking and the PMS	High: "no idea" of ROI, agency says "front desk is the problem", 37-38% calls unanswered, every dataset stops at cost per lead, "cost per registered patient" named as the missing metric	Weak: call-tracking and AI phone vendors measure their own step; no product joins lead source to seated patient and treatment value	Medium (lead and PMS data; BAA needed)	Strongest	Both	Fully for the join and the report; attribution judgement human
4	Denial rework, appeal-pack assembly and timely-filing deadlines	High: 8-22% denials, ~80% see more, 72.88% administrative, 54% paid after 3 cycles, 5% appealed, "denied three times", "Sit on hold", MetLife recoupment thread	Partial: billing companies at 4-10% of collections; Vyne "denial support"; no small-practice triage tool confirmed	Strong (EOBs, notes, radio graphs)	No	US	Partly: triage, pack assembly, deadline tracking automatable; narrative and signature human
5	Website and ad compliance checks (GDC items, ASA/CAP, FTC review rule, TCPA wording, Google health-ads policy, HIPAA in replies)	High: 1.8% of audited sites compliant; no "last updated" on any of five; "Dr" inconsistent; FTC warning letters Dec 2025; review-reply fines; GDC "registrant is responsible"	Thin: GDC checklist PDF; agencies do not advertise checking; no product found	Low (public pages)	Strongest	Both	Fully for the check; publication sign-off human and liable
6	Patient record requests (right of access, SAR) and retention clocks	High: six OCR dental cases on access, $5,000-$140,000; Shen Smiles Oct 2026; SAR one month; £10/£50 fees still advertised; 11-year retention	Thin: compliance vendors sell policies; PMS exports; no request-tracking product for a small practice found	Strongest (records)	No	Both	Partly: logging, deadline, assembly automatable; release decision and redaction human

#	Gap	Problem signal	Vendor coverage of the exact sub-problem	Private data fit	Agency sale	Country	Automatable
7	Eligibility breakdown reconciliation and difficult-payer exception handling	Very high: $2.1bn, 2 hours a day, 12 vs 4 minutes, "soul-crushing", 32.6% plan AI, 50+ proposals per Upwork post	Strong on the check (Zuub, Pearl, Curve, NexHealth, Weave, eAssist, VAs) but reviews confirm data not written back, no search by insurer, bare yes/no responses	Strong (PHI, payer portals)	No	US	Partly: scraping and compare automatable; interpretation and patient conversation human
8	Recall-list validation and reactivation segmentation	High: reappointment 42-81%, recall 60-70%, continuing-care types reset dates, "easiest work to move to tomorrow", 24-month NHS recall direction	Strong on reminders (every comms layer) but weak on list validation; reactivation sold as letters, scripts and VAs	Strong (patient lists)	Partial (reactivation campaigns)	Both	Fully for list validation and segmentation; the call human or vendor
9	PMS data clean-up and cross-system reconciliation, including software-stack creep	Medium-high: Dentrix Enterprise checklist, DI "numbers always inaccurate", 20-25 tools, "paying for it four times", conversion losses	Unpriced: vendors give the checklist; Open Dental $110/hr; offshore $4-$7/hr; no audit product	Strong (full database access)	No	Both	Fully for detection; decisions on deletion and merging human
10	UK exemption-evidence capture and PCN dispute handling	Medium-high: 115,463 PCNs a year (UC alone), ~30% withdrawn, £7.6m checking contract, enquiry letters return as calls	Weak: PMS captures the tick; no exception checker; RTEC for dental unconfirmed	Strong (NHS and benefit data)	No	UK	Partly: flag exemptions with "evidence seen: No" and expired certificates; the declaration and liability stay with the patient
11	Estimate, good-faith estimate and consent-record completeness	Medium-high: 22% paid more than quoted; 21% prices unclear; DCS +111%; GFE within 3 days; GDC 1.6	Thin: PMS prints plans; no check that a dated estimate and consent note exist per course	Strong (clinical records)	No	Both	Fully for the completeness check; the consent conversation human (Montgomery)
12	Clinical-note quality and AI-scribe governance (mandatory items, note volume, DPIA)	Medium, new: CDO "incredibly scary" (Oct 2026), MHRA device line (Aug 2026), DPIA and DCB0160 expected, GDC rules pending	Thin: Kiroku, Dentally AI notes, HSO Voice Notes generate; nobody sells the review or the DPIA	Strongest (clinical records; UK GDPR)	No	UK lead; US	Partly: checks for BPE, recall interval, consent and history entries automatable; clinical content human
13	Security baseline and Security Rule or cyber-insurance readiness evidence	High: 48% out-of-date OS, 25% shared logins, 17% failing backups, Westend, MFA required to bind cover, proposed rule with no size carve-out	Medium: MSPs at $1,200-$1,500/month, compliance vendors; thin for cheap evidence-based readiness at 3 chairs	Strong (inventory of ePHI systems)	No	US lead; UK DSPT	Fully for scanning and inventory; remediation and attestation human
crowded	AI phone receptionists and call intelligence	High and stable	Very strong: Weave, Adit, Arini, Viva, Annie, Peerlogic, Patient Prism, RoboReception; no public prices or independent reviews	Low	Low	Both	Automated, quality disputed
crowded	Dental SEO, websites and Google Ads management	High	Very strong: dozens of agencies, Fiverr from $10, white-label wholesale	Low	n/a (the agencies are the sellers)	Both	Commodity
crowded	Review generation, monitoring and texting	High	Very strong: My Social Practice, Swell, Birdeye, Podium, Weave; $5-$500 Upwork review gigs	Low	Low	Both	Commodity; FTC and GDC limits
crowded	Appointment reminders and confirmations	High	Very strong: every comms layer; Open Dental $25-$40	Low	Low	Both	Automated

#	Gap	Problem signal	Vendor coverage of the exact sub-problem	Private data fit	Agency sale	Country	Automatable
crowded	Insurance-verification keystrokes and outsourced billing	Very high	Very strong: eAssist, Philippine VAs at $5-$8/hr, Upwork BOB gigs, billing firms at 4-10%	Low	Low	US	Commodity labour
crowded	Imaging AI	Medium	Very strong: Overjet, Pearl, VideaHealth, Diagnocat, FDA-cleared, $133m-$155m raised	Low	Low	Both	Regulated device
crowded	PMS set-up gigs and VA agencies	Medium	Strong: Fiverr set-up gigs, Stealth Agents, VA Masters	Low	Low	US	Commodity
crowded	Membership-plan platforms; credentialing services	Medium	Strong: "dental membership plan software" and "dental credentialing services" autocomplete; Denplan, Practice Plan	Low	Low	Both	Service market

Gap map: the thirteen gaps placed by problem signal against vendor weakness, using the report's own ranking words; eight crowded areas in grey



Positions follow the wording of the ranking table in section 10 (very high / high / medium-high / medium signal; weak / thin / partial / strong on part / very strong coverage), not measured scores. Warm = coverage weak or thin; blue = partial or strong on the easy part; grey letters = the crowded areas the report says are not gaps.

The thirteen gap cards

Every card has the same eight fields in the same order. The three badges summarise the ranking table: problem signal (the report's own grade, warm when very high or high), vendor coverage (warm when weak, thin or unpriced; 'strong on the easy part' where vendors cover the check but not the reconciliation), and the report's own first word on automation (fully or partly).

1 NHS contract reconciliation: FP17 status against Compass, the two-month window, the 96% floor and the 2026 urgent-care split

UK

Problem signal: very high | **Vendor coverage:** weak | **Automatable:** fully

Problem	An English NHS practice must land every FP17 within two months of completion, correct rejected ones inside the same window, hit 96% of its annual UDA target or lose the whole shortfall, deliver 8.2% of contract value as urgent care from April 2026, and split the result across performers for pay; Compass shows only the £15 fixed element of the £75 urgent credit, so "a contract can hit its total on Compass and still miss the target", associates cannot see eDEN, and the year-end SD86c arrives in July with two months to dispute it.
Who has it	UK (England) NHS and mixed practices of one to five dentists; owners who inherited a UDA value (£15 to over £40, average £36.22); associates whose pay depends on the split; Wales under its own reform; Scotland on GP17 with hand-counted DMF in Dentally.
Evidence	About £900m clawed back over two years (£479m 2024, £457m 2025) (Dentistry.co.uk, 5 Mar 2026 [11] secondary); 32% of London NHS practices clawed back in 2022/23 (London.gov.uk [59]); late claims lose UDAs and corrected claims must be inside two months (NHSBSA KA-01820 [139]); "You cannot look at Compass and know where you are anymore", LDC motion calling it "a fundamental contract management failure" (GDPUK, 13 Jul 2026 [12]); rejected claims flagged "Invalid" and fixed by hand (Dentally help [356] vendor); BDA: "Clawback against arbitrary targets remains a major source of frustration" (BDA DDRB [14]).
Their words	"chasing UDA targets", "run out of UDAs", "clawback", "rebased", "handed back", "Universally Detested Acronym", "unknowingly driving a car with a faulty speedometer", "duplicate work and nobody's sharing the information"; autocomplete "compass nhs dental login", "nhs dental compass cut off dates", "nhs dental compass schedule", "nhs dental contract uplift 2026/27", "nhs dental claims dates"; job ads "manage claims and monitor compliance with claim requirements", "requirements of service delivery and remuneration under the NHS".
How they handle it today	The PMS (EXACT, Dentally, DentalPlus, R4) sends FP17s by EDI and shows acknowledgement or "Invalid"; the manager filters the NHS claims report, checks Compass "Activity Search Detail", reads the monthly schedule, DCS129 and DCS127, keeps a spreadsheet of UDAs delivered by performer, reads a commercial guide (UDA Pulse), and asks the LAT for leeway at year end; NHSBSA will not help with EDI errors.
Vendors already covering it	EXACT and Dentally: transmission and rejection flags only vendor; Dentally's product director: NHS England and Wales changes "consuming the roadmap", Scottish auto-fees and DMF "known gaps" user, 2026; UDA Pulse: a guide, not a tool secondary; NHSBSA Compass and eDEN: cannot show the £60 element; eDEN closed to associates press; no product found that reconciles PMS claims against Compass, the 96% floor and the urgent-care share.
Automatable?	Fully for the detection layer: a scheduled export of courses, FP17 status, submission dates, bands, performers and Compass schedules into a store the practice controls, rules for the two-month clock, comment codes, 96% and 102% lines, the 8.2% urgent share and the 30 September checkpoint, and an exception list by performer. Not automatable: the dentist's declaration on each form, the clinical band, the patient's signed PR form, the SD86c dispute and the decision to hand back.
What the reader would need to learn	The FP17 clinical data set and band-matching rules (V11.4 and V15.1), comment and error codes, the 2026 urgent-care, complex-care and QI mechanics, Compass report formats (DCS127, DCS129, SD86c, period codes), eDEN access rules, how EXACT and Dentally export claims (Dentally has a Zapier page; EXACT does not publish an API), UK GDPR and the DSPT for NHS data, the Wales and Scotland variants, and the fact that the 2026/27 NHS England guidance itself could not be read.

2 Unscheduled-treatment follow-up and case-acceptance exception tracking

US; UK estimate-to-consent unmeasured

Problem signal: very high	**Vendor coverage:** partial	**Automatable:** partly

Problem	More than half of diagnosed treatment is never accepted or never scheduled, the follow-up call "the next morning" or "within three days" belongs to no one, and the three products that count it define the denominator differently; the money marketing was meant to feed is lost after the patient is already in the chair.
Who has it	US general practices of one to five dentists on PPO and fee-for-service (the "$1-1.5 million" figure is per average practice); UK private and mixed practices selling implants, Invisalign and hygiene plans, where the estimate-to-consent conversion is unmeasured.
Evidence	Case acceptance 42% (2026) and 46% (2024) against 75-83% for the top 10%; adult fluoride 33% vs 91%; perio-to-prophy 24% vs 70%; "average practice leaves $1 to $1.5 million in annual revenue unscheduled"; follow-ups raise acceptance "10% or more" (Henry Schein One, 20 Aug 2024 [6] vendor; Henry Schein One, 22 Apr 2026 [5] vendor); treatment-dollar acceptance 39% vs 58% (Webtonic on DI 2024 [89] vendor, second-hand); "60-70% of practices do no systematic follow-up" (Clerri [91] vendor, unsourced); VA ad: "Contact patients with pending treatment plans or unscheduled visits" (OnlineJobs.ph 1491618 [125]); "Patients will NOT pay for teeth if they can't buy gas and groceries" (ADA HPI Q1 2026 [3] user, 2026).
Their words	"unscheduled treatment", "case acceptance", "treatment plan follow-up", "I'll think about it", "Dental Treatment Coordinator (Case Acceptance Focused)", "Private Patient Concierge and Treatment Coordinator"; "Print a report listing patients with incomplete treatment diagnosed over the past 12 months"; "Appoint someone to be responsible for follow up"; UK: "treatment coordinator dental salary uk per hour", "pressured to agree to particular treatments", "paid more than they had been quoted".
How they handle it today	A printed PMS report of incomplete treatment; a treatment coordinator or the front desk calling between inbound calls; Dental Intelligence's "Unscheduled Patients" board and follow-up tasks; Philippine VAs at $400-$700 a month; consultants' scripts; in the UK, a written estimate under GDC 1.6 and a deposit at booking, with no tracking of what happened next.
Vendors already covering it	Dental Intelligence (board, follow-up tasks, "Follow Up results"; one reviewer "the numbers were always inaccurate") vendor/user; Adit (follow-up lists; "AI Recare" coming soon) vendor; Planet DDS Denticon (treatment-plan analytics metric only) vendor; Practice by Numbers (bundle, unpriced) vendor; HSO's Jarvis (the benchmark source, $350 a user) vendor; consultants (Sandy Pardue, Levin, Dental A Team) selling the process; VAs selling the calls. No product confirmed to own the loop from plan presented to appointment booked with reason codes.
Automatable?	Partly: extracting planned-but-unscheduled procedures by date, value and provider from the PMS, assigning an owner and a due date, logging the reason given (cost, insurance, timing, fear) and reconciling the count against the analytics layer's figure is automatable; the clinical conversation, the financial arrangement and any AI-written patient message about clinical information (California AB 3030 disclaimer; HIPAA BAA) stay human or supervised.
What the reader would need to learn	PMS data models for treatment plans and procedure status (Open Dental's tables and $110/hour queries; Dentrix API at $1,750-$5,000 plus per-site fees; Dentally's Zapier triggers), CDT codes and plan values, how DI and Jarvis define acceptance, TCPA consent for follow-up texts, HIPAA business-associate terms, and the UK GDC estimate and consent rules.

3 Lead-to-seated-patient measurement across the agency CRM, call tracking and the PMS

Both

Problem signal: high | **Vendor coverage:** weak | **Automatable:** fully

Problem	Practices pay $1,000-$2,500 (£1,500-£8,000) a month and cannot see what came back; agencies report cost per lead and blame the front desk; every dataset stops at the call or the form, nobody joins lead source to seated patient and treatment value, and the two sides argue from numbers neither side measured independently.
Who has it	Private and mixed practices buying SEO, Google Ads, Meta ads or lead-gen in both countries; the dental marketing agencies serving them (US retainers $1,000-$2,500; UK £1,500-£8,000+; margins about 13-15% net), who lose the client when ROI cannot be shown.
Evidence	Principal spending £18,000 a year with "no idea" of ROI, "You need to give it more time" (Dentistry.co.uk, 29 Dec 2021 [17]); 37% of 55,000+ client calls unanswered, "your marketing isn't broken, your front desk is" (Dentaltown, 5 Aug 2026 [18] vendor); 31 of 100 callers hang up, 4.8 called back (Patient Prism, 2 Jul 2026 [4] vendor); cost per tracked lead $50.60 general to $212.63 Invisalign, "NOT booked patients" (Delmain, Jun 2026 [200] vendor); leads "turned up not even knowing the cost or what they wanted" (Trustpilot, Dentology [114] user); "cost per registered patient" is what matters (LinkedIn [235]); no source reports cost per seated patient in either country.
Their words	"cost per lead", "cost per new patient", "speed to lead", "lead flow", "new patient specials", "conversion tracking for calls and forms", "Google Business Profile", "call tracking", "no idea" of ROI, "give it more time", "generic menu-style packages"; agency job posts "SEO & PPC Specialist for Dental Practice Growth", "HubSpot Specialist – Long-Term Partner" needing "HubSpot and tracking foundations before launch"; UK "dental marketing agency uk", "seo cost per month uk", "dental practice google reviews".
How they handle it today	The agency reports from Google Ads, call tracking (CallRail, Patient Prism, Weave Call Intelligence) and its CRM; the practice counts new patients in the PMS; nobody matches the two; owners fire the agency or the receptionist; agencies subcontract tracking set-up on Upwork before a client launch.
Vendors already covering it	Patient Prism, Peerlogic, Weave, Convirza, VoiceStack: call outcomes for their own step vendor; DenGro (UK, £199-£249): lead pipeline to "new patient created" in EXACT vendor; Viva: AI follow-up with unsourced conversion claims vendor; HubSpot set-ups by agencies; Dental Intelligence: new-patient and production boards, not lead source. No product documented that reconciles agency-side leads with PMS-side seated patients and treatment accepted, and "cost per seated patient" appears in no vendor dataset.
Automatable?	Fully for the join and the report: matching lead records (source, campaign, time, phone or email) to PMS new-patient records, appointments kept and treatment accepted, with consent and a BAA because the PMS side is PHI; the attribution judgement (which touch "caused" the patient) and the decision to cut a channel stay human.
What the reader would need to learn	Call-tracking and ad-platform exports (Google Ads, Meta, CallRail, Patient Prism), HubSpot or DenGro data models, PMS new-patient and appointment tables, identity matching under HIPAA minimum-necessary and UK GDPR, TCPA and PECR rules on follow-up, Google's health-ads policy, and how agencies price (management fee $400-$700 a month at the $1,500-$5,000 tiers; white-label at $300-$1,500) so the work fits their margin.

4 Denial rework, appeal-pack assembly and timely-filing deadlines

US

Problem signal: high | **Vendor coverage:** partial | **Automatable:** partly

Problem	Roughly one claim in six comes back on first pass, mostly for administrative reasons the practice could have caught, the rework needs a pack (claim, EOB with reason, notes, radiograph, narrative, letter) inside a 90-180 day window, each round needs a longer narrative, small denials cost more to chase than they return, and only about 5% are ever appealed.
Who has it	US PPO-participating and Medicaid practices of one to five dentists, especially periodontal and crown-heavy ones; solo practices with 40-60 days in AR; practices that outsource billing at 4-10% of collections and still field the patient's call when the insurer pays less than the estimate.
Evidence	4% benchmark vs 8-15% typical (DentistryIQ [8] snippet); 22.5% of Delta in-network claims denied in 2024 (Dental Billing Podcast [329] secondary); nearly 80% report more denials (Becker's on Zentist [330] snippet); 72.88% administrative (DOAJ record [331]); 54% paid after three cycles of 45-60 days; ~5% appealed, ~50% reversed (dentalbilling.com [332] snippet); "denied three times, and she didn't know why", "over an hour and a half" on the phone (Yahoo/Daily Dot, Feb 2026 [103] user, 2026); "Claim denied," "Resubmit," "Appeal," "Sit on hold" (Dentaltown, 13 Aug 2026 [104] vendor).
Their words	"dental claim denied due to frequency", "dental claim denial codes", "dental claim denial code n640", "delta dental claim denied reddit", "what to do when dental insurance denied claim", "post-payment audit", "recoupment", "narrative", "attachments", "bundling", "downcoding", "coordination of benefits", "timely filing", "persistent follow-up on denied and aged claims", "monitoring claim status and resolving rejections and denials".
How they handle it today	The biller works the aging report "line-by-line", calling the same insurer several times a day at about 20 minutes a call; a simple denial takes 30 minutes to rework; the dentist writes a narrative when asked; small amounts are written off; outsourced billing firms at 5-7% of collections; Upwork billers advertising "persistent follow-up".
Vendors already covering it	Vyne Trellis ("denial support", flat fee, no price) vendor; DentalXChange, EDS (claims and attachments) vendor; eAssist, MedsDental, Dental Claim Support, Helpware (human billing at 4-10%) vendor; Curve and Henry Schein One blogs selling clean-claim features vendor; AAE warns general AI "lack[s] knowledge of carrier claims histories, state rules, and policy exclusions". No small-practice tool confirmed that triages denials by reason code, assembles the pack from the chart and tracks the appeal deadline per payer.
Automatable?	Partly: classifying denial reasons from ERA codes, pulling the matching EOB, notes, images and prior claims, drafting the cover letter and tracking the 90-180 day clock are automatable inside a BAA; the clinical narrative, the dentist's marking of the radiograph and the signature stay human, and the reviewer "decides only from what was submitted".
What the reader would need to learn	CDT coding and the January update, X12 837/835/277 and claim-adjustment reason codes, payer appeal rules and timely-filing limits, clearinghouse exports (DentalXChange, Vyne, EDS; Open Dental ERA tables), HIPAA minimum-necessary and BAAs, Medicaid-specific rules (Medi-Cal attachments, NPI), and how billing firms price so the service fits beside them.

5 Website and ad compliance checks against GDC, ASA/CAP, FTC, TCPA and Google rules

Both

Problem signal: high | **Vendor coverage:** thin | **Automatable:** fully

Problem	The pages that bring in private patients (Invisalign, implants, whitening, before-and-after galleries, "Dr" bylines, review widgets, specials, texting opt-ins) are the pages regulators check, and nobody checks them: UK sites omit GDC numbers, the complaints route and the "date last updated"; US sites run review incentives, pixel-tracked booking forms and specialty wording; agencies write the pages and the registrant "is responsible for checking that it is correct".
Who has it	UK private and mixed practices and the UK agencies that build their sites (Dental Focus "over 5,000 dentists", Dental Design, Dominate Dental); US practices and agencies running review campaigns, Google Ads and SMS; any practice replying to reviews.
Evidence	Only 1.8% of audited practice websites fully compliant with GDC guidance (PubMed, BDJ 2021 [19]); no "last updated" date and no GDC number on the home page of any of five sites read (Lund Dental [184]; Claregate Dental [187]); "Dr" and "Mr" used for the same clinicians (WhatClinic, Orchard Dental [435]); FTC rule effective 21 Oct 2024, penalties up to ~$53,088, warning letters 22 Dec 2025 (Crowell [241]); a $500 Upwork post asking for "genuine 5-star" reviews (Upwork [259] snippet); review-reply fines of $10,000-$50,000 (HIPAA Journal [249]); Aspen Dental $18.7m pixel settlement (DentistryIQ [239]); ASA braces enforcement notice and 2022 facilitator ruling (ASA [254]).
Their words	"gdc advertising guidance", "gdc social media guidance", "gdc ethical advertising", "cqc dental inspection checklist", "dental practice complaints procedure", "Specialist", "special interest in", "the treatment advertised may not be appropriate for every patient"; US "dental reviews by patients examples", "new patient dental specials", "no down no interest", "most major insurance plans"; agency "AI Visibility Services", "dental website design with SEO".
How they handle it today	The agency builds and the practice signs off without a checklist; the GDC publishes a one-page advertising checklist PDF; dento-legal advisers answer calls after the complaint; compliance consultancies run CQC mock inspections that do not cover the website; nobody reads the "last updated" field.
Vendors already covering it	GDC advertising checklist PDF official; CAP Copy Advice (free, non-binding) official; compliance consultancies (RightPath4, Lamia Murray) for CQC, not marketing vendor; US HIPAA vendors (Abyde) for policies, not pages vendor; no agency in the sample advertises a compliance check, and no product was found that audits a practice website, social feed and ad copy against the GDC, CAP, FTC, TCPA and Google health-ads rules.
Automatable?	Fully for the check: crawling the site and ad copy for the mandatory GDC items, GDC-number validity, title and specialist-list use, before-and-after disclosures, review-incentive wording, TCPA consent language, pixel presence on booking pages and health-category remarketing; the publication sign-off and the legal liability stay with the registrant or the practice.
What the reader would need to learn	The GDC advertising and social-media guidance and the specialist lists, CAP Code sections 3 and 12 and the dental and whitening advice, the FTC rule text, TCPA consent tiers, Google's personalized-advertising health policy, HIPAA tracking guidance on authenticated pages, state specialty-advertising rules (Texas 2021), and how agencies price so the check sells as a line in their retainer.

6 Patient record requests (right of access, subject access) and retention clocks

Both

Problem signal: high | **Vendor coverage:** thin | **Automatable:** partly

Problem	The most common trigger of federal action against a small US dental practice is a record request it did not answer in time or charged for; in the UK a subject access request must be answered within one month, free, with records kept at least eleven years; the requests arrive by phone, letter, portal and email, the records sit in the PMS, imaging software, paper charts and scanned folders, and no one logs the clock.
Who has it	Every US practice (covered entity) and every UK practice (controller); practices that have changed software and hold images in a proprietary format; practices with paper charts; practices after a breach, when "records could not be provided due to a hacking incident".
Evidence	Six OCR dental right-of-access cases from $5,000 to $80,000 in 2022 and Gums Dental $70,000 in 2024 (HIPAA Journal [249]); Shen Smiles $140,000 announced 8 Oct 2026, records "in plastic boxes, the floor, and a private bathroom" (Paubox [266] vendor); Westend Dental investigation started with an unmet records request (HIPAA Journal [267]); UK SAR one month, free, clock starts at the request; practice site still advertising £10/£50 fees (MDDUS [296]; Dentistry.co.uk, 2018 [297]); GP surgery reprimanded for sending 23 years of records when five were asked for (Freevacy, 3 Feb 2026 [294]); 11-year minimum retention (NHSBSA KA-01913 [295]).
Their words	"right of access", "copy of my records", "records request", "copying fee", "notice of privacy practices", "subject access request", "SAR", "retention", "11 years", "HC5", "records transfer when changing dentist"; OCR's "files regularly missing when patients arrived".
How they handle it today	The office manager prints or exports from the PMS, burns images, charges a fee or refuses, and answers when time allows; UK practices copy records on request and sometimes charge; after a software change, images are "in proprietary formats" and conversions cost $850-$900; indemnifiers advise on the phone.
Vendors already covering it	PMS export functions (Open Dental, Dentrix, Dentally) vendor; HIPAA compliance vendors (Abyde) selling policies and training vendor; indemnifiers (MDDUS, Dental Protection, DDU) with advice lines; records-management codes. No request-logging, deadline-tracking or minimum-necessary assembly product for a small practice was found.
Automatable?	Partly: logging each request with its date and channel, computing the 30-day (US) or one-month (UK) deadline, assembling the record set from PMS, imaging and document stores, and flagging retention dates are automatable; deciding what to release, redacting third-party data, handling insurer requests (the Staines case) and any refusal with reasons remain human.
What the reader would need to learn	HIPAA right-of-access rules and fee limits, UK GDPR Article 15 and ICO guidance, dental retention schedules by state and nation, PMS and imaging export formats (DICOM, proprietary), DSPT expectations, and minimum-necessary handling.

7 Eligibility breakdown reconciliation and difficult-payer exception handling

US

Problem signal: very high | **Vendor coverage:** strong on the easy part | **Automatable:** partly

Problem	The eligibility check is automated for easy payers and manual for hard ones; the automated answer is often a bare yes/no that a person must interpret; the breakdown produced by an add-on lives outside the PMS ledger and is re-keyed; and the payers that force a phone call are the ones checked last, so the miss surfaces as a denial or a surprise balance.
Who has it	US PPO and Medicaid practices with 20-30 appointments a day and one or two front-desk staff; practices on Weave, NexHealth, Curve or Zuub add-ons that do not write back to the chart; Medicaid, HMO, dual-coverage and out-of-network cases.
Evidence	$2.1bn spend, 12 vs 4 minutes, 82% electronic adoption, 15% fully manual (ADA News [7]; Deepgram [317] secondary); carriers "frequently" send "nothing more than a single yes or no response" (Open Dental [173]); NexHealth: insurance details "not written to the EHR" (NexHealth help [323] vendor); Zuub cannot search by insurer, name or date of birth (Capterra [321] snippet); "Eligibility is almost always going to be last" (Dentaltown, 2017 [160] vendor); 32.6% of dentists plan AI for verification (ADA HPI, Jul 2026 [45]); 50+ proposals on a $5-$20/hr Upwork post (Upwork [131] snippet).
Their words	"breakdown of benefits", "BOB", "eligibility, deductibles, annual maximums, frequencies, waiting periods, and limitations", "exact copay", "error-free", "before appointments", "on hold with the payer while the schedule backs up", "soul-crushing", "manual nightmare"; autocomplete "dental insurance verification companies", "dental insurance verification form pdf", "dental insurance verification services in florida".
How they handle it today	Phone and portal checks one to three days ahead, a printed or typed breakdown in the chart, an add-on PDF in the document centre, a Philippine VA at $5-$8 an hour or eAssist at $250-$650 a month, and a shrug for the hard payers.
Vendors already covering it	Zuub, Pearl Precheck, Curve Eligibility+, NexHealth Verification, Weave Insurance Verification, Overjet, Verrific, Eaglesoft Insurance Suite (the check) vendor; eAssist, Outsource Strategies, Dental Claim Support, VAs (the labour) vendor. Crowded on the check; thin on reconciling the breakdown to the PMS coverage tables, on tracking which payers were skipped, and on the exception list for the hard cases.
Automatable?	Partly: comparing add-on breakdowns to PMS coverage tables, flagging unchecked or stale verifications per appointment, and listing the payers that need a call are automatable; interpreting percentages and network status, the call itself for 30-40% of plans, and what the patient is told and charged stay human.
What the reader would need to learn	X12 270/271 content and limits, payer portal structures (Availity, Skygen, Delta), PMS insurance-plan and coverage tables (Open Dental), the add-ons' export formats, HIPAA BAAs, Medicaid state rules, and the price points ($2.50-$3 a check; $250-$650 a month) the service competes with.

8 Recall-list validation and reactivation segmentation

Both

Problem signal: high | **Vendor coverage:** strong on the easy part | **Automatable:** fully

Problem	The recall list is corrupted before it is worked: duplicate continuing-care types reset dates and drop patients, inactive patients stay active, and the list itself is "the easiest work to move to tomorrow" because no patient is waiting on the line; reactivation of 800-2,000 dormant patients is sold as letters, scripts and a four-hour phone session.
Who has it	US practices with hygiene reappointment below 85% and 1,200-1,500 active patients per provider; UK practices moving healthy adults to 24-month NHS recall and selling private hygiene and plans between visits; any practice that has migrated software.
Evidence	Reappointment "as low as 42%", 63%, 77-81% vs 94% top (DentistryIQ [92]; Webtonic [89] vendor); continuing-care types "cause recall dates to reset incorrectly and patients to be missed on lists" (Dentrix Enterprise [412] vendor); recall outreach "becomes the easiest work to move to tomorrow" (Hello Patient [180] vendor); reactivation recovers 10-25%; 80-90% return with an appointment vs 35-45% without (Clerri [95] vendor); UK 24-month recall direction and £3,400 recall QI payment (The Probe, 1 Oct 2026 [10] secondary); VA ads "Make recall and reminder calls for upcoming or overdue appointments" (OnlineJobs.ph 1491618 [125]).
Their words	"recall", "recare", "reactivation", "hygiene reappointment", "continuing care", "dormant patients", "dental recall script", "dental recall text message sample", "dental reactivation letter", "dental recall guidelines uk", "dental recall letter example", "NICE recall interval", "24-month recall".
How they handle it today	Comms-layer reminders at the interval the PMS holds; a printed overdue list worked by phone when time allows; postcards and letters; VAs; a reactivation campaign once a year; UK practices sending recall letters and switching lapsed NHS patients to private hygiene offers.
Vendors already covering it	Weave, Solutionreach, RevenueWell, Lighthouse, NexHealth, Open Dental Web Sched Recall ($75) and eReminders, Adit, Hello Patient (reminders and recall messaging) vendor; Clerri, Ainora and consultants (reactivation benchmarks and scripts) vendor; VAs (the calls). Strong on sending; no product found that validates the list (duplicate types, inactive flags, interval errors) or segments dormant patients by value and reason before the campaign.
Automatable?	Fully for validation and segmentation: reconciling continuing-care types, last-visit dates, interval settings and patient status, and ranking dormant patients by last treatment and plan status; the outreach is already automated by the comms layer or done by a person; UK recall intervals are clinical decisions under NICE.
What the reader would need to learn	PMS recall and continuing-care tables (Dentrix, Open Dental, Dentally), comms-layer sync behaviour, TCPA and PECR rules for reminders and marketing messages, NICE recall guidance and the 2026 NHS recall direction, and HIPAA or UK GDPR for patient lists.

9 PMS data clean-up and cross-system reconciliation, including software-stack creep

Both

Problem signal: medium-high | **Vendor coverage:** unpriced | **Automatable:** fully

Problem	A practice runs "20 to 25 connected software tools", pays for the same feature "in four places", holds duplicate insurance plans and employers, departed providers, un-inactivated patients, unsent claims and stale fee schedules, and reads two analytics numbers for the same thing that cannot be reconciled; the vendor gives away the clean-up checklist and nobody sells the clean-up or the reconciliation.
Who has it	US practices on Dentrix, Eaglesoft, Open Dental or a cloud PMS with a comms layer and an analytics layer; practices that migrated and lost images or in-flight claims; UK practices on Dentally Starter without the portal or on R4 moving to Sensei.
Evidence	Dentrix Enterprise clean-up list (Dentrix Enterprise [412] vendor); "the numbers were always inaccurate and DI wouldn't/couldn't fix it" (G2 [391] snippet); DI claims-aging will not match Dentrix (DI help [392] vendor); $300/month unused reminder program, $450/month Dentrix texting after moving to Weave, "paying for it four times" (Thriving Dentist, 14 Oct 2026 [33] vendor); migration losses and non-refundable conversions (Capterra, Curve [377] snippet; Open Dental conversions [362] vendor); Sensei UK exit CSV-only and paid (G-Cloud 14 [379] vendor).
Their words	"duplicate insurance plans", "continuing care types", "fee schedules", "unbilled procedures", "claims not yet sent", "reports don't allow importing to Excel", "outta sight, outta mind", "paying for it four times", "data conversion", "proprietary format"; autocomplete "dentrix vs open dental", "open dental vs dentrix cost", "dentrix vs dentrix ascend cost", "dentally vs exact".
How they handle it today	A one-off clean-up by the office manager from the vendor's blog; a $110/hour Open Dental query; a $4-$7/hour offshore poster; an MSP at $1,200-$1,500 a month for IT but not data; an accountant's year-end question about software subscriptions; nothing at all until a migration forces it.
Vendors already covering it	Open Dental (queries, set-up sessions, training at published rates) vendor; Henry Schein One "Dentrix Coach" consultants (unpriced) vendor; MedSys contractor trainers; Fiverr set-up gigs; MedsDental "Practice Audit Solution" (unpriced) vendor; DI, Jarvis and PbN (dashboards that show but do not reconcile) vendor. No priced "database audit" or "stack audit" product was found; "Dentrix audit" otherwise means expert-witness work.
Automatable?	Fully for detection: duplicate detection, orphan and stale-record reports, unsent-claim and unbilled-procedure lists, subscription inventory against feature use, and a reconciliation of the aging report to the analytics layer's figure; merging, deleting and cancelling stay human and liable.
What the reader would need to learn	PMS schemas and export routes (Open Dental SQL; Dentrix API cost; Eaglesoft's bridge; Dentally Zapier; Sensei CSV terms), the vendors' own clean-up checklists, how DI and Jarvis compute their metrics, migration mechanics and fees, HIPAA and UK GDPR for full-database access, and the price anchors ($110/hr, $500-$700 small-practice analysis, £600-£1,500 day).

10 UK exemption-evidence capture and PCN dispute handling

UK

Problem signal: medium-high | **Vendor coverage:** weak | **Automatable:** partly

Problem	Reception must record an exemption claim, see and note the evidence with its expiry, tick "evidence seen" truthfully and keep the signed form for two years; NHSBSA sample-checks later and sends the patient a £100 penalty, about 30% of which are later withdrawn; the enquiry letters come back to the desk as calls and complaints, and the tax-credit exemption quietly stopped being valid in April 2025.
Who has it	UK (England) NHS and mixed practices; practices in deprived areas with high Universal Credit and HC2 populations; reception teams told by the DDU not to advise patients on entitlement.
Evidence	115,463 dental PCNs to UC claimants in a year; 96,489 in eleven months (FOI-02776 [347]; FOI-02518 [348]); ~30% of PCNs withdrawn after challenge; 47% of treatments claimed exempt; £212m lost (NAO 2019 [349]); £7.59m six-month checking contract (Find a Tender [350]); evidence-seen rules and PR-form retention (NHSBSA guide [135]; KA-01916 [136]); tax-credit exemption withdrawn for acceptances on or after 6 Apr 2025 (FP17 guidance V14 [344]); "shouldn't face £100 penalties by default just for ticking the wrong box" (GDPUK [352]).
Their words	"exemption", "evidence seen", "HC2", "HC3", "Universal Credit", "PCN", "penalty charge notice", "enquiry letter", "check before you tick", "nhs dentist exemption certificate", "nhs dental exemption checker", "am i exempt from nhs dental charges", "can i claim back nhs dental charges", "Nhs dental charges penalty notice" (patient forum).
How they handle it today	Posters, counter cards and booklets from NHSBSA; a paper PR form signed and filed; the receptionist asking for a card or letter; "No" ticked when nothing is shown; the patient told checks are routine; the HC5(D) form for the unsure; calls fielded when the letter arrives.
Vendors already covering it	PMS exemption fields and certified e-signature capture (EXACT, Dentally; Dentally cannot invalidate a claim once the exemption is linked) [vendor]; NHSBSA Real Time Exemption Checking (dental scope unconfirmed) [official]; NHSBSA DECS (the auditor, not the practice's tool) [official]. No practice-side tool found that flags exemption claims with "evidence seen: No", expired certificates or withdrawn categories before the claim goes.
Automatable?	Partly: flagging each NHS course where an exemption is claimed without evidence, where a noted certificate has expired, or where a withdrawn category (tax credit) is used, and logging PCN enquiries by patient, is automatable; the patient's declaration, the truthful tick, the liability and any advice on entitlement stay with the patient and the practice, and the signature must be captured by a certified system.
What the reader would need to learn	The exemption categories and evidence rules, the PR-form retention and e-signature standard, FP17 fields for exemption and charge collected, RTEC status for dentistry, the PCN process and the NAO's withdrawal findings, and UK GDPR for benefit data.

11 Estimate, good-faith estimate and consent-record completeness

Both

Problem signal: medium-high | **Vendor coverage:** thin | **Automatable:** fully

Problem	A UK private course must have a written plan and cost estimate before treatment and a fresh one if it changes; a US self-pay patient must get a good-faith estimate within three business days; consent must be discussed and recorded to the Montgomery standard; the Which? sample shows 22% paying more than quoted and 21% unsure of the price, and nobody checks that each course has a dated estimate, a consent note and an updated medical history before the chair.
Who has it	UK private and mixed practices facing the DCS (complaints +111%) and the GDC; US practices with uninsured patients (22% of Americans) under the No Surprises Act; implant, Invisalign and sedation cases on both sides.
Evidence	22% "paid more than they had been quoted", 21% prices unclear, 16% pressured (GDPUK, Which?, 6 Oct 2026 [71]); DCS cases 465 to 985 (Dentistry.co.uk, 20 Mar 2025 [69]); GDC 1.6 and 1.7 (GDC Standards [146]); a signature "does not indicate valid consent" (Dentistry.co.uk, Feb 2021 [149]); GFE within three business days, $400 dispute threshold (ADA News, Mar 2022 [171]); "a cleaning quote only became a $173 bill once an exam was added" (MedCity News, 2014 [436]); CQC requires a current medical history (Dental Update [148]).
Their words	"written treatment plan and cost estimate", "consent", "Montgomery", "material risks", "good faith estimate", "self-pay", "No Surprises Act", "paid more than they had been quoted", "prices were unclear before treatment began", "pressured to sign up to a dental payment plan", "trust is fragile"; autocomplete "private dentist hourly rate", "how much does private dentist cost", "dental practice complaints procedure".
How they handle it today	A PMS treatment-plan printout handed over or emailed; a consent form for sedation and GA and a note for the rest; a medical-history form at the first visit and a verbal "any changes?" after; a GFE template from the state association for self-pay patients; the complaint handled afterwards by the practice manager and the indemnifier.
Vendors already covering it	PMS plan and estimate printing (EXACT, Dentally, Dentrix, Open Dental sheets at $55/hour design) vendor; Dentally Portal and Open Dental web forms for medical history vendor; Michigan and Pennsylvania dental associations' GFE templates official; indemnifier consent guidance (MDDUS, Dental Protection) secondary. No product found that checks every course for a dated estimate, a consent entry and a current medical history before treatment begins.
Automatable?	Fully for the completeness check: matching each planned or started course to a dated estimate, a consent note and a medical-history update, and flagging price changes against the estimate given; the consent conversation, the clinical explanation and the estimate itself stay with the clinician.
What the reader would need to learn	GDC Standards 1.6, 1.7 and principle 3, Montgomery and the DCS process, the No Surprises Act GFE rules for dental "excepted benefits", PMS treatment-plan and document tables, and the privacy rules for clinical records in both countries.

12 Clinical-note quality and AI-scribe governance

UK lead; US

Problem signal: medium	**Vendor coverage:** thin	**Automatable:** partly

Problem	AI note-takers now produce "computer files that are the length of textbooks", clinicians spend "as much time writing their notes as they treated the patient" because a missing item could be "career-ending", the MHRA says summarising tools are likely medical devices, NHS England expects a DPIA, DCB0160 documentation and explicit consent, the GDC has no rule yet, and nobody checks whether the note contains what the FP17, the CQC and the indemnifier need.
Who has it	UK practices adopting Kiroku, Dentally AI notes (45-90 transcriptions per surgery per month) or Henry Schein One Voice Notes; US practices where charting and note-taking is the top planned AI use (34.8%); any practice facing a GDC concern (1,766 in 2025, 58% clinical, "examination" the top allegation).
Evidence	"The volume of AI notes is getting incredibly scary" (GDPUK, 16 Oct 2026 [109]); MHRA device line (Dentistry.co.uk, 2 Aug 2026 [287]); NHS England guidance v3 and DPIA template (NHS England [288] snippet; GPCE summary [289]); Dental Protection's INFORMED and RECORDS framework (Dental Protection [437]); GDC joint statement 17 Sep 2026 (Dentistry.co.uk [285]); BPE and NICE interval mandatory on adult FP17s (NHSBSA FP17 guidance [151]); CQC looks at records to corroborate (CQC mythbuster 8 [438] snippet); 34.8% of US dentists plan AI charting (ADA HPI [45]).
Their words	"AI notes", "ai scribe for dentists", "ai notes for dentists", "hipaa compliant ai note taker", "clinical records", "contemporaneous notes", "career-ending", "DPIA", "DCB0160", "medical device", "automation bias", "the role AI played and the clinician's decision".
How they handle it today	The clinician reads and edits the AI draft (or does not), dictates, or types at lunch and in the evening; the practice buys the scribe inside the PMS tier; the DPIA is a template if done at all; record audits are periodic and manual; indemnifiers publish frameworks.
Vendors already covering it	Kiroku ("save me an hour a day"; 5.0 from 12) vendor/user; Dentally AI clinical notes (Essentials and Pro, capped, not in NI or ROI) vendor; Henry Schein One Voice Notes and Dentrix Ascend Voice (Bola) vendor; Dental Protection framework secondary; NHS England template DPIA official. Generation is covered; no product sells the completeness and quality review of notes against FP17, GDC and CQC expectations, or the DPIA and hazard log for a small practice.
Automatable?	Partly: checking each note for mandatory items (BPE, recall interval, consent noted, medical history updated, radiograph justification), length and duplication, and maintaining the DPIA and hazard log are automatable; the clinical content, the clinician's review (MHRA), and anything that supports diagnosis are human or regulated.
What the reader would need to learn	NHS England's ambient-scribe guidance and IG template, DCB0129 and DCB0160, the MHRA device boundary, GDC record-keeping standards and College of General Dentistry guidance, FP17 mandatory fields, UK GDPR Article 35 DPIAs, HIPAA BAAs for US scribes, California AB 3030, and how Kiroku and Dentally store and export notes.

13 Security baseline and Security Rule or cyber-insurance readiness evidence

US lead; UK DSPT

Problem signal: high | **Vendor coverage:** medium | **Automatable:** fully

Problem	Half of small practices run out-of-support operating systems, a quarter share logins, one in six has no working backup, insurers will not bind cover without MFA, encryption and offline backups, the proposed Security Rule would make the same controls mandatory with no size exception, and the practice has no inventory of what holds ePHI or NHS data.
Who has it	US practices of one to five dentists with no IT staff; UK practices on NHS networks completing the DSPT each June; practices whose MSP account was the entry point (Absolute Dental) or whose backup vendor was hit (DDS Safe).
Evidence	48% out-of-date OS, 25% shared logins, 20% exposed remote access, 17% failing backups, 9% patient Wi-Fi (Dental IT, 30 Jul 2026 [311] vendor); Westend: plain-text passwords, no risk analysis, no BAA (HIPAA Journal [267]); proposed rule: MFA, encryption, inventory, six-monthly scans, annual pen test, "no size exceptions" (Georgia Dental Association, 26 May 2026 [272]); "most carriers will not even issue a policy unless a practice proves it has implemented" MFA, encryption and offline backups (Techvera [313] vendor); "a huge number of dental practices still aren't using Multi-factor Authentication" (Practice Plan, 27 Mar 2026 [312] vendor); DSPT required for NHS contracts (DSPT news 169 [143]).
Their words	"security risk analysis", "business associate agreement", "MFA", "encryption", "immutable backups", "asset inventory", "penetration test", "cyber insurance application", "DSPT Standards Met", "Password1!", "backed up files: 0", "one vulnerable computer puts all the others at risk".
How they handle it today	A local IT firm on a monthly contract ($1,200-$1,500 per office in one US figure) or a cousin; a consumer router; a cloud backup nobody checks; a HIPAA policy binder from a compliance vendor; the DSPT filled in by the practice manager from last year's answers; the insurance application attested from memory.
Vendors already covering it	MSPs (TST Support, Dental IT, Black Talon) vendor; compliance vendors (Abyde, Medcurity, D3RX) selling risk analyses and policies vendor; cyber insurers' control questionnaires; DSPT self-assessment official. Crowded for managed IT and policy binders; thin for cheap, evidence-based readiness reports that map a three-chair practice's inventory, MFA status, backup logs and vendor BAAs to the proposed rule, the DSPT and the insurer's questions.
Automatable?	Fully for inventory, configuration checks, backup-log reading and the gap report; remediation, the risk-analysis sign-off, the BAA negotiations and the insurance attestation stay human, and the proposed rule's final status is unconfirmed.
What the reader would need to learn	The current HIPAA Security Rule and the January 2025 proposal, OCR's risk-analysis expectations, the DSPT version 9 and CAF alignment, Cyber Essentials, insurer questionnaires, dental PMS and imaging network layouts, backup verification, and the vendor-BAA landscape for AI tools.

Private-data fit, the agency sale, and the crowded areas

- The self-hosted or private-data angle is strongest for gaps 1, 2, 4, 6, 7, 9, 12 and 13: each touches claim, patient, NHS or clinical records that the law keeps under a BAA, UK GDPR and the DSPT, that consumer AI tools cannot lawfully hold, and that staff today paste into free tools or key by hand.
- The agency sale is strongest for gaps 3 and 5 and partly 8: agencies lose clients over unproven ROI and write the pages that break the GDC, ASA and FTC rules, their margin leaves 'a few hundred dollars' a month per client for subcontracted work, and they already buy tracking and HubSpot set-up on Upwork before a dental client launches (Upwork, HighNote [466] snippet; Swydo on Promethean [467] secondary).
- The crowded areas that are not gaps: AI phone receptionists and call intelligence (no public prices, no independent reviews, 'the latest grift' in practitioner circles); dental SEO, websites and Google Ads management; review generation and texting tools; appointment reminders and confirmations; insurance-verification keystrokes and outsourced billing at $5-$8 an hour or 4-10% of collections; imaging AI (FDA-cleared, venture-funded); PMS set-up gigs and VA agencies; membership-plan platforms and credentialing services.

SECTION 11

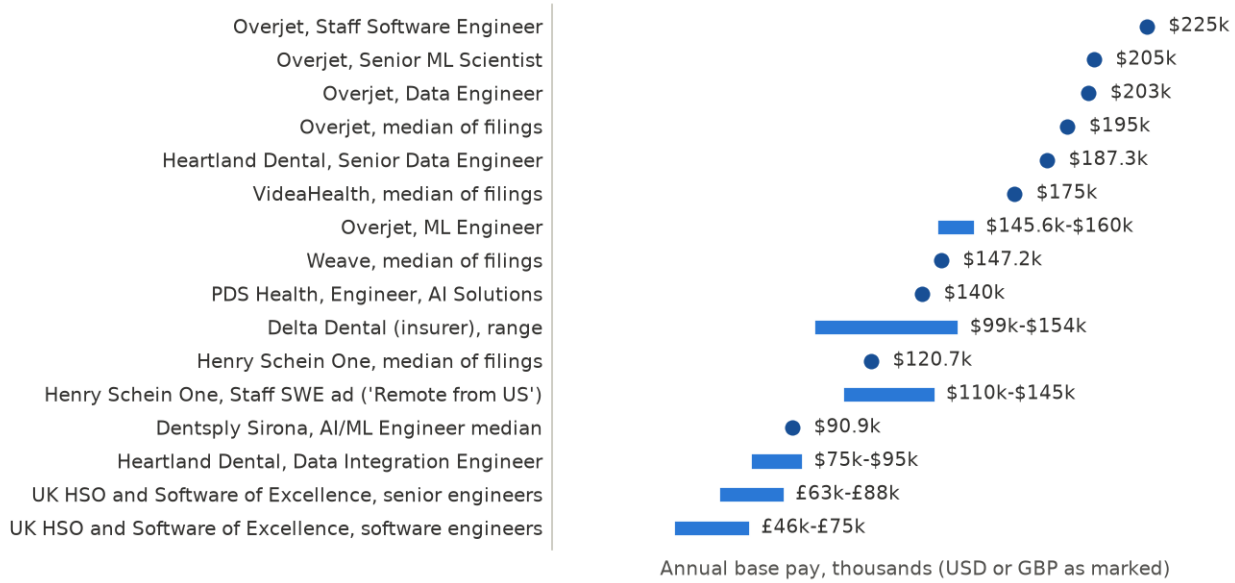
Employed path versus freelance path: a dozen sponsored seats at $195,000 against a $700 voice-agent gig

The employed path pays well and is small: a handful of R&D seats per vendor in named hubs, H-1B medians of $175,000-$195,000, and ads that ask for mainstream depth rather than dentistry. The freelance path is open to anyone and priced against offshore labour: a $600-$700-a-month Philippine VA who already lists Open Dental, posting gigs at $4-$7 an hour, and the only mid-four-figure budgets in AI-receptionist builds and PMS integration.

The employed path: pay by employer

- US Department of Labor H-1B filings put dental-AI engineering at **$145,000-$240,000 base** (Overjet median $195,000: Senior ML Scientist $205,000, Data Engineer $203,000, Staff Software Engineer $225,000; VideaHealth median $175,000), horizontal dental SaaS at **$110,000-$170,000** (Weave median $147,183; Henry Schein One median $120,700 and a Staff Software Engineer ad at $110,000-$145,000 'Remote from US'), DSO internal IT at **$75,000-$190,000** (PDS Health 'Engineer, AI Solutions' $140,000; Heartland Senior Data Engineer $187,257), insurers around $99,000-$154,000 and device makers lower (Dentsply Sirona AI/ML Engineer median $90,854) (Ellis, Overjet [20]; Ellis, VideaHealth [439]; Ellis, Weave [440]; Ellis, Henry Schein One [441]; Ellis, PDS [442]; Ellis, Heartland [443]; Ellis, Dentsply [444]). UK dental-software pay is **£46,000-£75,000 for software engineers and £63,000-£88,000 for seniors** at Henry Schein One and Software of Excellence, roughly a third to a half of US dental-AI pay (Glassdoor UK, HSO [21] snippet; Glassdoor UK, SOE [445] snippet).
- The seats are few: Overjet's careers page on 16 October 2026 listed **31 open roles of which three were R&D engineering**, all in San Mateo or Lahore; VideaHealth and Dental Intelligence listed no roles; DentalPost's 1,600-plus jobs have no technology category (Overjet careers [446]; Videa careers [447]; DentalPost [448]). The ads ask for mainstream depth, not dentistry: Overjet's Staff ML Engineer wanted 10+ years in Python, Go, C++ or Java, PyTorch, computer vision or LLMs, 6+ years on GCP or AWS, Kubernetes and Terraform, with no degree stated; dental knowledge is hired separately as 'Dental Consultant' clinician roles; FHIR, HL7, RAG, vector databases and PMS APIs appeared in no engineering ad found (General Catalyst board, Overjet [449]; Built In, Pearl [450] snippet; Welcome to the Jungle, HSO [451]).
- Remote means remote inside the US or a named hub: Overjet is hybrid two days a week in San Mateo, New York, Salt Lake City or Boston with Lahore as a geo-hub; Weave opened a second HQ in Noida; NexHealth has 'remote Nexers across the US, South America, and Europe'; sponsorship is small (Overjet 42 H-1B filings 2020-2026, 4-13 a year; VideaHealth 12; Henry Schein One 9; Weave 18; PDS 59) and no ad stated a sponsorship policy (Ellis, Overjet H-1B [452]; Weave careers [453]; NexHealth careers [454]).
- The 2025-2026 moves favour incumbents embedding others' AI over new ML teams: VideaHealth $40m Series B (Jan 2025), an Overjet Series C-II of $24.56m (one aggregator), KKR's $250m into Henry Schein, Henry Schein One's AWS partnership and embedded Voice Notes, forms and eligibility in Dentrix (Nov 2025), Open Dental adding $149 AI image analysis; against that, Dentsply Sirona's 2-4% workforce cut, Align's Q3 2025 restructuring, nearly 200 DSO transactions in H1 2026, and no 2025-2026 venture money found for Zuub, NexHealth, Dental Intelligence or Curve (Business Wire, VideaHealth [455]; CB Insights, Overjet [456] snippet; Henry Schein, KKR [457]; Henry Schein One, 25 Nov 2025 [458]; Dentsply 10-Q [459]; Align 10-Q [460]; Becker's, DSO M&A [461] snippet).

Employed pay: US dental-AI engineering at $145,000-$240,000 (Overjet median $195,000, VideaHealth $175,000), horizontal dental SaaS at $110,000-$170,000, DSO IT at $75,000-$190,000; UK dental software £46,000-£88,000

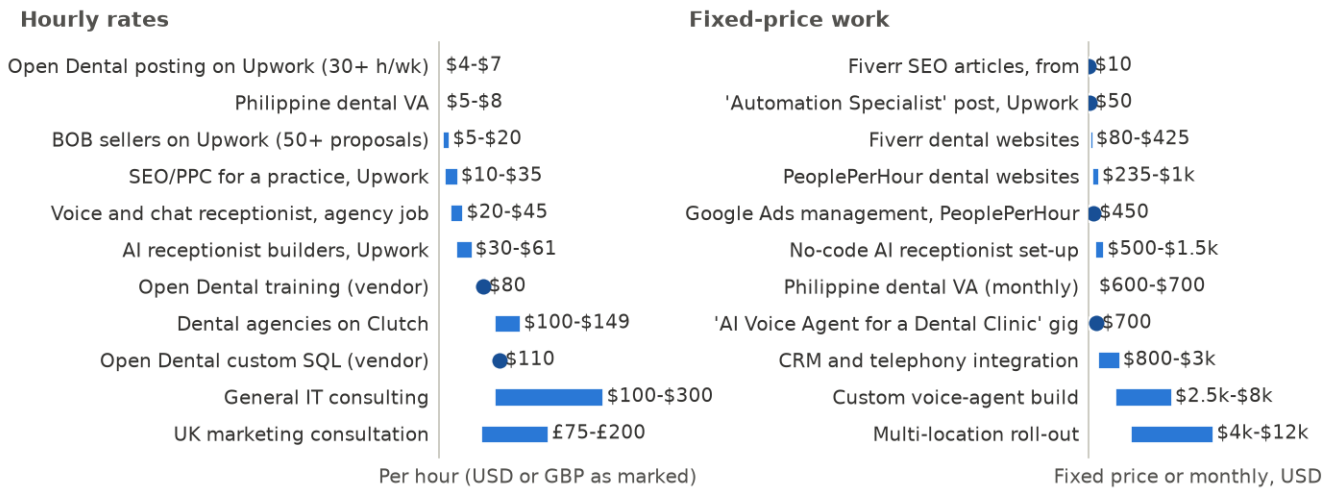


Sources: US Department of Labor H-1B filings via Ellis for Overjet [20], VideaHealth [439], Weave [440], Henry Schein One [441], PDS Health [442], Heartland [443], Dentsply Sirona [444]; Glassdoor UK for Henry Schein One [21] and Software of Excellence [445] (snippets, self-reported). Overjet listed 31 open roles on 16 Oct 2026, three in R&D engineering, all San Mateo or Lahore.

The freelance path is priced against offshore labour

- The buyer-side anchors are the Philippine dental VA at **$600-$700 a month or $5-$8 an hour**, already listing Open Dental, Flex, insurance verification and claims; the AI-receptionist subscription at $300-$900 per location per month; the agency retainer at $1,000-$2,500 a month with a management allocation of $400-$700; billing firms at 4-10% of collections; and the $401-$500 a month that 66% of practices budget for workflow software (OnlineJobs.ph 1335685 [22]; Remotive [462] snippet; Sagnik Bhattacharya [397] secondary; Becker's on Software Advice [411] snippet).
- Visible freelance demand for generic 'dental automation' is thin and cheap (one 'Automation Specialist for Dental Clinics' post at $50 fixed; posting at $4-$7 an hour), but the AI-receptionist build market has defined budgets: builders at **$30-$61 an hour**, a $20-$45 an hour agency job for 'Medical/Dental Virtual Receptionist (Voice & Chat)', no-code set-up $500-$1,500, **custom voice-agent build $2,500-$8,000**, CRM and telephony integration $800-$3,000, multi-location roll-out $4,000-$12,000, and a productised 'AI Voice Agent For A Dental Clinic' gig at $700 with three-day delivery (Upwork, automation post [463] snippet; Upwork AI receptionist builders [23] snippet; Upwork dental voice agent gig [464] snippet).
- Marketing gigs are commoditised (Fiverr dental websites $80-$425, SEO articles from $10, category pages showing roughly 300-3,100 listings; PeoplePerHour dental websites $235-$1,000, Google Ads management $450) and Upwork insurance posts draw 50-plus proposals at $5-$20 an hour (Fiverr [465] snippet; PeoplePerHour [132]; Upwork [131] snippet). Consulting rates that exist are Open Dental's $110 an hour for custom SQL and $4,325 a day on site, UK marketing consultations at £75-£200 an hour and a trainer day at £600-£1,500, and general IT consulting at $100-$300 an hour; **no Dentrix, Exact or Dentally independent consultant rate is published anywhere** (Open Dental fees [361] vendor; Dominate Dental [16] vendor; Consulting Success [426] secondary).
- Practices buy subscription-first and vendor-bundled (PMS add-ons, an AI receptionist, a billing service, a VA), so what the market visibly buys from a freelancer is implementation or integration of those subscriptions, or a replacement for a VA or billing-service cost line, not bespoke AI engineering; dentists name poor PMS integration (22%) as the second reason they abandon AI tools and better integration (22%) as the second reason they adopt (GDPUK, 1 Jun 2026 [281]).

Freelance market: posting and verification labour at $4-$8 an hour, AI-receptionist builders at $30-$61, and the only mid-four-figure budgets in custom voice-agent builds ($2,500-$8,000) and integration ($800-$3,000)



Sources: Upwork AI receptionist builders [23] (snippet); Upwork dental voice agent gig [464] (snippet); Upwork automation post [463] (snippet); Upwork Open Dental posting [417] (snippet); Upwork BOB job [131] (snippet); Upwork SEO/PPC post [234] (snippet); OnlineJobs.ph 1335685 [22]; Fiverr [233] (snippet); PeoplePerHour [132]; Open Dental fees [361] (vendor); Clutch [214]; Consulting Success [426] (secondary); Dominate Dental [16] (vendor).

Table 11.1 Employed path against freelance path, dimension by dimension

Comparison	Employed (dental AI and software vendors, DSOs)	Freelance and consulting
Income range	US $145k-$240k base at AI vendors; $110k-$170k dental SaaS; $75k-$190k DSO IT; $50k-$95k implementation and marketing-automation roles; UK £46k-£88k	$30-$61/hr AI-receptionist builds; $2,500-$8,000 custom build; $800-$3,000 integration; $700 productised gig; $110/hr PMS SQL (vendor rate); $100-$149/hr agency rates; floor $4-$20/hr against $5-$8/hr VAs
Time to first revenue	Set by hiring cycles, 6-10+ years' experience bars, hub location and 4-13 sponsorships a year per vendor; not quantified in any source	Days: $500-$700 jobs and three-day gigs exist; ticket sizes small until a $2,500-$8,000 build or a retainer lands
Competition	Scarce seats (3 R&D openings at Overjet; none listed at VideaHealth or DI) against the generic AI/ML labour market	Offshore VAs listing the same PMS skills; 300-3,100 Fiverr gigs; 50+ proposals per Upwork post; 250-999-person agencies at $100-$149/hr; billing firms at 4-10%
Clinical background or HIPAA needed	No clinical background; dental expertise hired as clinician roles; HIPAA not stated in ads (sampling limit)	No clinical background; buyers name the PMS and the insurance workflow; HIPAA and UK GDPR not stated in posts but unavoidable (BAAs, DSPT, fines)
Remote and visa reality	"Remote from US"; hubs in San Mateo, Boston, Lehi, Irvine, Lahore, Noida, Cairo, Manila; H-1B used at low volume; no sponsorship text	Buyers explicitly target LATAM, Europe and the Philippines; US business hours (CST/PST) and "clear, neutral accent" asked for voice roles
Skills on both sides	Python and SQL; API and PMS integration (Open Dental, Dentrix Ascend, Eaglesoft, Flex); Salesforce and HubSpot automation; voice and conversational AI; insurance-verification automation; data pipelines	Same core; plus Make, Zapier, Twilio, GBP and call-tracking set-up, and NHS claiming knowledge for the UK
Skills on one side only	Imaging computer vision and PyTorch model training (Overjet, VideaHealth); Kubernetes and Terraform platform work	Marketplace selling, agency-margin pricing, PMS-specific keystroke knowledge
2025-2026 moves	VideaHealth $40m; Overjet $24.56m (aggregator); KKR $250m into Henry Schein; HSO + AWS; Dentsply and Align cuts; back-office offshoring; ~200 DSO deals in H1 2026	No venture money into the automation layer; AI-receptionist category crowded; ADA finds 32.6% plan AI verification and 34.8% AI notes, i.e. integration work rising

The practical reading in the report's own words: an employed US dental-AI engineer earns in a month what the visible freelance market pays for one custom voice-agent build; the UK employed figure equals a year or two of a single agency retainer; the seats are a handful per vendor in named hubs with low sponsorship volumes; the freelance floor is set by a $700-a-month VA who already knows Open Dental; and the only budgets above that floor found in the open market are integration and reconciliation work, which is also the work the practices, the agencies and the regulators have all, in different words, said is missing. Neither path needs a clinical background; both need PMS integration, Python and SQL, insurance or NHS claiming workflow knowledge, and HIPAA or UK GDPR handling, which the ads do not state but the fines and the vendor terms make unavoidable.

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SECTION 12

What could not be verified

The researchers were blocked by many primary hosts (reddit.com on every mirror; Dentaltown's message board and GDPUK's forum beyond titles; Capterra, G2, Software Advice, GetApp and TrustRadius review bodies; Fiverr entirely; Upwork job, hire and catalog pages; Dental Economics; Becker's; DentistryIQ in part; RDH Magazine; hhs.gov and federalregister.gov; the ICO enforcement list; several NHSBSA guidance PDFs; NHS England's ambient-scribe, quality-and-payment and Recovery Plan pages; CQC mythbusters in part; the GDC's news pages; the Commons Library briefing; Indeed, NHS Jobs, BDJ Jobs and Reed ad bodies; most practice websites in the sweep), so a large share of quotes come from search summaries and secondary coverage, and the web-search budget ran out part-way through every workstream. The items below are the ones a reader should treat as unconfirmed before repeating them to a client.

Table 12.1 Items that rest on snippets, secondary reports, vendor claims or conflicting sources

Item	Status
Missed-call rates (31-38%), lead-response times (26 hours), recall rates (60-70%), reactivation recovery (10-25%), "60-70% do no systematic follow-up", "40% never come back"	All vendor or consultant figures with different denominators; several unsourced; no regulator or academic benchmark exists in either country
First-submission denial rates of 15-22%	Vendor blogs and a podcast's reading of Delta transparency files; the defensible range is 4% benchmark vs 8-15% typical, itself from a billing-vendor article seen only in a snippet
CAQH 12/7/4 minutes per check and 82%/15% adoption	Secondary write-ups of the CAQH Index; the PDF was not fetched
HIPAA Security Rule final status (May 2026, July 2027 or already final)	Conflicting vendor and trade reports; hhs.gov and the Federal Register blocked reading
OpenAI BAA eligibility for ChatGPT Enterprise	Two secondary sources conflict; help.openai.com returned 403
Reddit quotes on AI receptionists ("Patients hung up on robot voice", "24/7 coverage saved $2k/month")	Quoted by a vendor without links; reddit.com unreachable
Heartland Dental lawsuit over AI call recording	Single vendor blog; no court record found
Eaglesoft move to subscription-only; Dentrix API fees ($1,750-$5,000 plus per-site)	Competitor and aggregator claims and a Reddit user report; no Patterson or Henry Schein One statement
Dentally prices (Starter £125 vs £134 vs £195; Capterra £50/user) and Dentrix, Eaglesoft, Curve, Denticon, Weave tier, NexHealth, DI, Jarvis, PbN and every AI-receptionist price	Vendor pages are quote-only or ambiguous; aggregator estimates conflict
NHS England 2026/27 contract-management guidance; FP17 guidance V15.1; ProvClin guidance V5.0; FP17O UOA values; whether Real Time Exemption Checking is live for dentistry	PDFs returned 404 or redirect loops; rules quoted from the Oct 2022 V11.4 PDF, a High Court summary, a commercial guide and an accountancy-firm summary; the £70/£5 vs £75/£15 urgent figures were reconciled from secondary sources
National count of NHS contract hand-backs and practice closures by year; share of FP17s rejected or late; UK DNA rate; UK cost per seated patient; UK minutes per claim or exemption check	Not published in any source reached; only local FOIs and the 2017-2021 surveys exist
"68% of UK practices dissatisfied with agency leads"; "average practice owner spends $2,500 a month on an agency"; "$10-15 back for every $1"	Agency blog, YouTube sales claim and agency testimonial with no primary source
Dominate Dental's UK agency comparison and price list	Page header read "Hacked by Panataran" at the time of reading
Named-agency review counts and text (Wonderist, Roadside, Golden Proportions, Dental Focus, Dental Design, PBHS, Smile Savvy, Great Dental Websites)	Conflicting directory listings; no Trustpilot or Google review text reached for any UK agency
Fiverr gig prices, review counts and buyer requests; Upwork proposal counts and most budgets	Bot wall and 403 on every page; snippets only
Dental Reviewed AI survey (32%), Dental Economics-Levin 43% AI use, Software Advice $401-$500 budget, NexHealth 26% online booking, Weave 55% front-desk survey	Vendor or trade surveys with undisclosed or small samples; several seen only in snippets

Item	Status
Delmain cost-per-lead rows for dentures, cosmetic and Invisalign	The agency itself labels them "thin and directional" (2-13 campaigns)
Becker's reappointment and retention table (77% vs 94%; 57% vs 99%)	No source study or year identified
Membership-plan adoption (38% of practices; 8-12% of uninsured enrolled)	Vendor attribution to a Dental Economics study not reached
ADA HPI AI survey sample (552) and Catalyst Index sample and definitions	Not stated by the publishers; Becker's figure seen in a snippet
Overjet Series C-II $24.56m; investor-blog claims of Morpheus, Lightspeed and Telstra rounds	Single aggregator; the investor blog contradicts itself
Henry Schein 2026 layoffs and offshoring	Anonymous thelayoff.com posts
Iowa board fine for review replies (date, amount); California AB 3030 coverage of dental offices; TCPA rules specific to dental reminder texts; FCC 2025 revocation rule text	Pages returned 403 or 404; no dental-specific primary source
ICO monetary penalties or reprimands against UK dental practices; UK dental breach outcomes (BDA 2020)	ICO enforcement page is dynamic and could not be read; none found
Dental PCN totals beyond Universal Credit and JSA claimants; DSPT "Standards Met" counts for dentistry; CQC count of dental locations	Not retrieved

Conclusion

What the research changes

The problem a small dental practice will pay to remove is not dentistry and not even 'the phone'. It is the chain of handoffs that each product owns one link of and nobody reconciles: the lead the agency delivered against the patient the PMS seated, the treatment the dentist planned against the appointment nobody booked, the breakdown the add-on produced against the ledger the biller works, the FP17 the software sent against the UDA target Compass can no longer show, the record request the patient made against the thirty-day clock.

In 2025-2026 that chain came under outside pressure in both countries at once: the NHS reforms of April 2026 and a reporting system that cannot show the new target; the FTC review rule, the proposed Security Rule and OCR's October 2026 dental settlement; the MHRA's device line and the Chief Dental Officer's warning about AI note volume; insurers that will not bind cover without MFA and backups; and a marketing market in which neither the practice nor the agency can prove what the money bought. The strongest language is therefore reconciliation and evidence, not answering or generating.

Where the vendors already sit, and what the law reserves to a person

AI receptionists, SEO, review tools, reminders, offshore verification labour and imaging AI are where the vendors, the gigs and the venture money already sit, and their users complain about contracts, prices, accents and upselling, not absence.

The steps the law reserves to a person (the dentist's FP17 declaration and the clinical band, the patient's signed exemption form, Montgomery-standard consent, the clinician's narrative and signature on an appeal, the review of any AI transcript, the release decision on a record request, the BAA that consumer ChatGPT does not offer) are boundaries, and every one of the thirteen gaps stops at one of them.

Two vocabularies, two markets for the same skills

The vocabulary is split by country: 'clawback', 'UDAs', 'FP17', 'FTA', 'exemption', 'CQC', 'GDC' and 'Denplan' for the UK; 'no-show', 'breakdown of benefits', 'claim denied', 'PPO', 'office manager', 'HIPAA' and 'membership plan' for the US. The same service will need two sets of words, and sections 2 and 10 give them verbatim.

On money, an employed US dental-AI engineer earns in a month what the visible freelance market pays for one custom voice-agent build, the UK employed figure equals a year or two of a single agency retainer, the seats are a handful per vendor in named hubs with low sponsorship volumes, and the freelance floor is set by a $700-a-month VA who already knows Open Dental; the only budgets above that floor found in the open market are integration and reconciliation work, which is also the work the practices, the agencies and the regulators have all, in different words, said is missing.

This document reports problems, workflows, numbers, vocabulary and gaps only. It contains no solutions and no recommendations. The Sources appendix lists every URL the research cited; section 12 lists what could not be verified.